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**Proposed Programme Budget for the
financial period 2010–2011**

Eastern Mediterranean Region

Contents

Section 1. Regional overview	1
Section 2. Regional programme statements by strategic objective	7
Strategic Objective 1: Reduce the health, social and economic burden of communicable diseases.....	7
Strategic Objective 2: Combat HIV/AIDS, malaria and tuberculosis	22
Strategic Objective 3: To prevent and reduce disease, disability and premature death from chronic noncommunicable conditions, mental disorders, violence and injuries and visual impairment.....	32
Strategic Objective 4: Reduce morbidity and mortality and improve health during key stages of life, including pregnancy, childbirth, the neonatal period, childhood and adolescence, and improve sexual and reproductive health and promote active and healthy ageing for all individuals	36
Strategic Objective 5: Reduce the health consequences of emergencies, disasters, crises and conflicts, and minimize their social and economic impact	41
Strategic Objective 6: Promote health and development, prevent and reduce risk factors for health conditions associated with tobacco, alcohol, drugs and other psychoactive substances, unhealthy diets, physical inactivity and unsafe sex	44
Strategic Objective 7: Address the underlying social and economic determinants of health through policies and programmes that mainstream equity and integrate pro-poor, gender-responsive, and human rights-based approaches	49
Strategic Objective 8: Promote a healthier environment, intensify primary prevention and influence public policies in all sectors so as to address the root causes of environmental threats to health	54
Strategic Objective 9: Improve nutrition, food safety and food security, throughout the life-course and in support of public health and sustainable development.....	58
Strategic Objective 10: Improve health services through better governance, financing, staffing and management, informed by reliable and accessible evidence and research.....	61
Strategic Objective 11: Ensure improved access, quality and use of medical products and technologies	74
Strategic Objective 12: Provide leadership, strengthen governance and foster partnership and collaboration with countries, the United Nations system, and other stakeholders in order to fulfil the mandate of WHO in advancing the global health agenda as set out in the 11th General Programme of Work.....	77
Strategic Objectives 13: Develop and sustain WHO as a flexible, learning Organization, enabling it to carry out its mandate more efficiently and effectively	80

Section 1. Regional overview

Introduction

The 53rd Regional Committee for the Eastern Mediterranean held in Isfahan in September 2006, discussed and reviewed the draft Programme Budget for 2008–2009 which represented the first of three programme budgets prepared to operationalize the Mid-term strategic plan 2008–2013. The draft Programme Budget 2010–2011 is the second Programme Budget submitted to the Regional Committee for review and comments prior to the submission of the amended Mid-term strategic plan 2008–2013 and global Programme Budget 2010–2011 to the Executive Board in January 2009 and to the Health Assembly in May 2009. It covers the biennium 2010–2011. It will be the sixth biennial budget that follows the Organization-wide results-based approach adopted by WHO in 2000.

The proposed regional Programme Budget 2010–2011 has been prepared in consultation with regional facilitators and teams for the 13 Strategic Objectives, utilizing input received from all WHO Representatives and other staff in the Region. In its preparation, the global health agenda included in the 11th General Programme of Work and the broad strategic framework in the Medium-term strategic plan 2008–2013 have been taken into consideration. Similarly, the Country Cooperation Strategies which have been developed jointly between WHO and all countries in the Region have provided a consolidated vision of the priorities of Member States and hence contributed to a proposed programme budget that is well oriented to countries.

Challenges faced and lessons learnt

Difficult political environments persist, particularly for countries in complex emergencies. Weak capacity for policy analysis and formulation, strategic planning, economic analysis of health systems and long-term scenarios for human resources development, management and distribution remain major challenges. Health care delivery systems in the Region have widely differing characteristics and performance. Most countries in the Region have “mixed” systems of health care with a varying degree of mix of providers—public, private and governmental—although usually with one sector dominating. Inequities in health care financing put an increasing burden on households. The role of governments in health financing is decreasing, and historically there has been limited interest among donors to support health systems and service development projects.

However, the interest of Member States in health system development and strengthening is growing and this is reflected in WHO collaborative programmes covering the various health system functions and building blocks. Support from extrabudgetary sources is increasing and new windows of opportunity to further strengthen health systems are being offered through partnership with the GAVI Alliance and the Global Fund to Fight AIDS, Tuberculosis and Malaria. The studies carried out in some countries on social determinants of health have triggered the interest of policy-makers and highlighted the role of social determinants in strengthening health systems.

The direct correlation between poverty and ill health is now well understood. Social and economic determinants affect health outcomes and this explains the need for ensuring effective partnership between different sectors to achieve sustainable development. Success in addressing the underlying social and economic determinants of health requires that communities be empowered to become actively involved in development.

Evidence for effectiveness of interventions for health protection and promotion, including environmental health risks and health outcomes, should be further developed. Maternal, infant and under-five mortality and morbidity are unacceptably high in some countries as a result of shortages of human and financial resources, poor utilization of existing services and the implications of sanctions, wars and conflicts on adequate investment in health services, among other things. Family planning programmes are weak in some countries.

While modern and emerging environmental health problems, such as chemical exposure, are increasing, the Region is still struggling with problems such as water contamination and indoor air

pollution. The arid geography of the Region poses challenges such as water shortage and dust storms and climate change is expected to increase these. Environmental and occupational health capacity needs to be developed and further efforts are needed to develop national environmental health preparedness plans for emergencies

High levels of overweight and obesity, combined with pockets of under-nutrition and micronutrient deficiencies, create a complex nutritional picture for the Region. Building national capacities for noncommunicable disease prevention and care requires strong political commitment and intersectoral collaboration. In many countries, environmental health functions, nutrition policies and other risk factors contributing to the burden of noncommunicable diseases do not fall under the responsibilities of ministries of health. Collaboration should be further strengthened between ministries of health and other partner ministries for health protection and promotion.

Avoidable blindness and visual impairment remain among the major public health problems in the Region. Available data indicate that around 6 million are blind and 22 million visually impaired (which is respectively higher than other WHO Regions); 80% of blindness is avoidable. Unfortunately most of the blind population lives in low-income countries with large populations. Poverty and blindness are interrelated. Simple interventions are available and can easily reduce the burden of blindness. The major causes of blindness are cataract, refractive errors, glaucoma, diabetic retinopathy, and trachoma and childhood blindness. More efforts and support are necessary to reduce the blind population.

The Eastern Mediterranean Region is at risk for epidemic-prone diseases because it is a crossroads for international trade, travel, tourism, a centre for religious mass gatherings, and is continuously exposed to a changing expatriate workforce. Therefore, collaboration with global partnerships has become essential for communicable disease control, and the Regional Office continued its extensive support to countries in this regard. By end of 2007, the Global Fund to Fight AIDS, Tuberculosis and Malaria had approved a total of US\$ 534 million to support 14 countries. The Regional Office also facilitated partnership development for other communicable diseases, such as schistosomiasis and dracunculiasis. The importance of health system strengthening for scaling up communicable disease control has been recognized, with the health system strengthening window of the GAVI Alliance providing valuable support in this regard.

Rigorous implementation of the International Health Regulations (2005), which came into force in June 2007, as well as scaling up of preparedness for human pandemic influenza are still priorities in the Region. Avian influenza remained a major public health concern. Between March 2006 and September 2008, Egypt reported a total of 50 cases of highly pathogenic avian influenza type A/H5N1, with 22 related deaths. Pakistan reported three confirmed human cases of avian influenza, with one related death in December 2007. The threat posed by avian influenza and the potential for human pandemic influenza was prominent in planning activities in the past years, and will continue to be so. It is vital that the revised International Health Regulations (IHR 2005) are adhered to in this and other areas of disease control to ensure global health security.

Low case detection remains a serious problem in tuberculosis control in the Region. By the end of 2007, the case detection rate had increased from 44% to 52%, with the situation particularly improved in countries with a high burden of tuberculosis. The global targets are to detect 70% of the existing cases of tuberculosis and successfully treat 85% of them.

Malaria epidemiology in the Region is a mosaic of situations ranging from areas that are malaria-free to others that are hyper-endemic. As a result, different types of strategies and different kinds of intervention are needed. Three countries in the Region (Islamic Republic of Iran, Iraq and Saudi Arabia) have set malaria elimination as a target. The main strategy is strengthening and upgrading the malaria programme with particular emphasis on malaria surveillance and vector control. For the 13 countries in the Region that are free from malaria or have residual foci, the main strategy is prevention of malaria reintroduction through strengthening surveillance for early detection of imported cases and vector monitoring.

The estimated number of people living with HIV in the Region by end 2007 was 530 000, with HIV prevalence estimated at between 0.1% and 0.3 % of the adult population aged 15–49 years. There are concentrated epidemics among injecting drug users in four countries, with another country at high risk of such an epidemic. Overall, countries succeeded in providing antiretroviral therapy to 79% of people living with HIV known to the health system. However, this relates to only 6% of the estimated number in need of antiretroviral therapy. Activities in countries focused primarily on responding to growing technical guidance and support needs, mobilizing resources, building capacities and expanding partnerships at national and regional levels.

It is a vision of the Regional Office that “No child will die from a vaccine-preventable disease in the Region”. The only way to achieve this is to make sure that every child in the Region receives a safe vaccine for each childhood vaccine-preventable disease, and that new and improved vaccines of regional importance are added to the immunization schedule as soon as they become available. Materializing this vision means that obstacles to increasing access to routine immunization services will need to be overcome, that a mechanism to ensure vaccine availability and quality in the Region is established, and that barriers against introduction of new and under-used vaccines are overcome. Diarrhoeal diseases and acute respiratory infections continue to be the major killers of children under 5 years in the Region. Several effective new vaccines against this group of illnesses are currently available or are in the pipeline, including *Haemophilus influenzae B* (Hib), pneumococcal and meningococcal conjugate vaccines as well as rotavirus vaccine.

The top priority for the regional polio eradication program is to interrupt poliovirus transmission in the two remaining endemic countries (Pakistan and Afghanistan) as soon as possible. The main challenges facing in these two countries are related to lack of access due to the security situation in certain areas and high population mobility within and between the two countries. As the goal of poliomyelitis eradication nears, countries of the Region must remain vigilant against the threat of importation of wild poliovirus, which can result in large-scale epidemics such as those that occurred in Yemen and Somalia in 2005. Therefore, the priority for polio free-countries is to sustain this status through continued sensitive AFP surveillance and maintaining high population immunity with OPV vaccination. Securing and managing funds and staff to maintain high levels of performance is another ongoing challenge for the polio eradication programme.

The leading role of WHO in collaborative activities with other United Nations agencies, nongovernmental organizations and potential partners at regional and country levels is the key for accelerating the achievement of targets set in the global health agenda. Linkages between different initiatives and programmes related to health and development should prevent duplication of efforts and lead to synergy in these approaches. Evidence-based information and success stories are important tools to gain further support from policy-makers. The active involvement and collaboration of ministries of finance and planning is a key factor for attaining programme sustainability and expansion. Strengthening WHO presence at country level remains a major priority. WHO presence in countries will be further supported through utilization of the country cooperation strategy documents, evaluation of the process and outcome of the strategies, and enhancement of country office human resource capabilities.

Implementing the programme budget

The regional programme budget for 2010–2011 will be implemented through operational plans prepared by countries and the Regional Office. The office-specific expected results at country and regional levels are country focused. While meeting the specific needs of countries, the results are derived from and support the achievement of the regional expected results, which accordingly contribute to achievement of the Organization-wide expected results in the global Proposed Programme Budget document (see Figure 1). Strategic directions provide greater transparency and accountability. Indicators with baselines and targets are provided for each regional expected result. The ongoing process of identifying gaps between the resource requirements for achieving results and available resources will be supported and monitored. To the extent possible, adjustments will be made

across strategic objectives and resources will be shifted to close gaps and ensure that available resources are allocated in the right place at the right time.

Overall level of the budget

The global Proposed Programme Budget 2010–2011 has been prepared to cover the three segments of budget: WHO programmes; partnership and collaborative arrangements; and outbreaks and crisis response. An amount of US\$ 4937 million was planned to cover the first two segments, with US\$ 3833 million for WHO programmes and US\$ 1104 million for partnerships and collaborative arrangements. It is very difficult to predict the budget needs for response to crises; therefore the budget for this purpose has not yet been planned.

The budgetary allocation proposed for the Eastern Mediterranean Region for 2010–2011 is US\$ 433 million for WHO programmes, representing approximately 11% of the global budget for this segment, and US\$ 52.4 million for partnership and collaborative arrangement, representing approximately 5% of the global budget for this purpose. This amounts to a total of US\$ 485.4 million for the Eastern Mediterranean Region, representing 9.8% of the total budget for 2010–2011. Out of this, US\$ 95.5 million is proposed to be financed from the regular budget and US\$ 390 million from voluntary contributions.

The budget split between regional/country offices and headquarters will be 70% and 30%, respectively, of the total budget proposed for 2010–2011. Implementation of this integrated budget, comprising all sources of funds rather than only regular budget will require more work with partners and donors in aligning voluntary contributions with the programme budget to meet the targets set in the medium-term strategic plan, and to ensure that the availability of resources is equitable across the Organization.

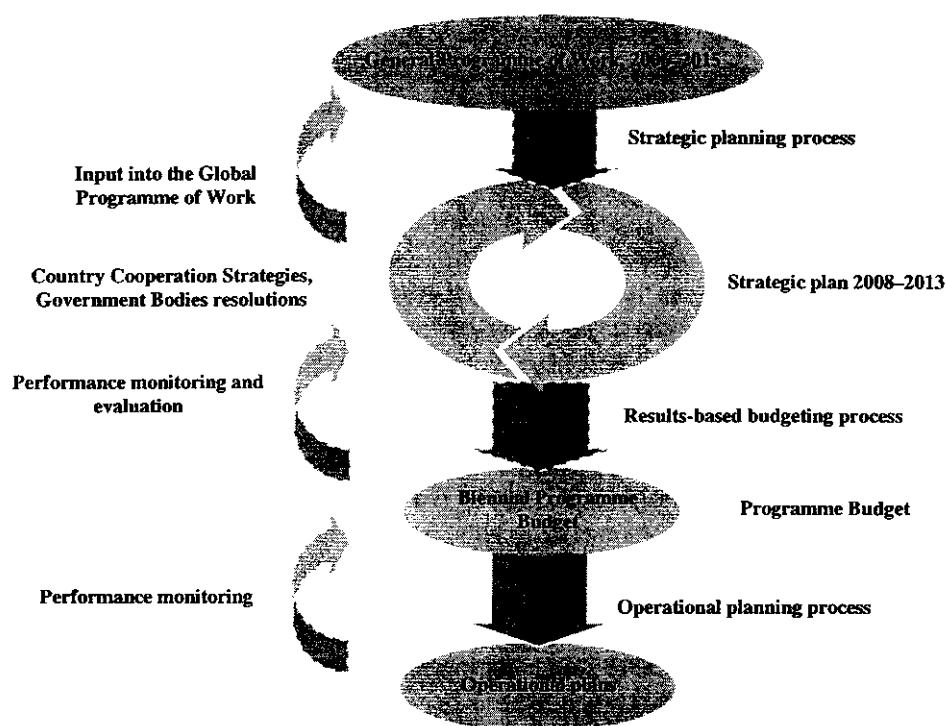


Figure 1. Relationship between the 11th General Programme of Work, Medium-Term Strategic Plan and biennial Programme Budget

Table 1 shows distribution of resources in 2006–2007, 2008–2009 and 2010–2011 respectively. The proposed distribution of resources from all sources of funds for 2010–2011 by each Strategic Objective is shown in Table 2. Funds include only two components: WHO base programmes and partnership and collaborative arrangements. The proposed allocation of resources to countries in 2010–2011 is shown in Table 3.

Table 1. Distribution of resources for 2006–2007 and 2008–2009, and proposed distribution for 2010–2011

Biennium		Regular budget (US\$ 000)			Voluntary contributions (US\$ 000)			Total all sources of funds (US\$ 000)		
		RO/ICP	Country	Subtotal	RO/ICP	Country	Subtotal	RO/ICP	Country	Total
2006–2007	Funds	34 934	52 522	87 456	67 625	226 765	294 390	102 559	279 287	381 846
	%	39.9	60.1	100	23	77	100	27	73	100
2008–2009	Funds	39 092	52 478	91 570	97 724	384 029	481 753	136 816	436 507	573 323
	%	42.7	57.3	100	20.3	79.7	100	23.9	76.1	100
2010–2011 (proposed)	Funds	39 560	52 440	92 000	98 350	295 050	393 400	137 910	347 490	485 400*
	%	43	57	100	25	75	100	28.4	71.6	100

RO Regional Office

ICP Inter-country programme

* excluding outbreak and crisis response cost

Table 2. Proposed distribution of funds from all sources for the Eastern Mediterranean Region, 2010–2011

Strategic Objective		WHO Programmes (US\$ 000)	Partnership and collaborative arrangements (US\$ 000)	Total from all sources of funds (US\$ 000)
1	Reduce the health, social and economic burden of communicable diseases	103 300	52 400	155 700
2	Combat HIV/AIDS, malaria and tuberculosis	52 000		52 000
3	Prevent and reduce disease, disability and premature death from chronic noncommunicable conditions, mental disorders, violence and injuries	19 000		19 000
4	Reduce morbidity and mortality and improve health during key stages of life, including pregnancy, childbirth, the neonatal period, childhood and adolescence, and improve sexual and reproductive health and promote active and healthy ageing for all individuals	35 050		35 050
5	Reduce the health consequences of emergencies, disasters, crises and conflicts, and minimize their social and economic impact	8 000		8 000
6	Promote health and sustainable development, prevent and reduce risk factors for health conditions associated with use of tobacco, alcohol, drugs and other psychoactive substance use, unhealthy diets, physical inactivity and unsafe sex	21 000		21 000
7	Address the underlying social and economic determinants of health through policies and programmes that strengthen health equity and integrate pro-poor, gender-responsive, and human rights-based approaches	13 000		13 000
8	Promote a healthier environment, intensify primary prevention and influence public policies in all sectors so as to address the root causes of environmental threats to health	19 000		19 000
9	Improve nutrition, food safety and food security, throughout the life-course, and in support of public health and sustainable development	9 000		9 000
10	Improve health services through better governance, financing, staffing and management, informed by reliable and accessible evidence and research	71 790		71 790
11	Ensure improved access, quality and use of medical products and technologies	17 200		17 200
12	Provide leadership, strengthen governance and foster partnership and collaboration in engagement with countries, to fulfil the mandate of WHO in advancing the global health agenda as articulated in the 11th General Programme of Work	28 140		28 140
13	Develop and sustain WHO as a flexible, learning Organization, enabling it to carry out its mandate more efficiently and effectively	36 520		36 520
Total		433 000	52 400	485 400

Table 3. Proposed budget allocation to countries for 2010–2011 pending WHA approval and availability of funds

Country	Budget allocation for 2010–2011			
	Assessed contributions (US\$)	Voluntary contributions for WHO Programmes (US\$)	Voluntary contributions expected from partnerships and collaborative arrangements (US\$)	Total (US\$)
Afghanistan	12 096 078	44 834 375		56 930 453
Bahrain	131 818	488 585		620 403
Djibouti	1 285 673	4 765 375		6 051 048
Egypt	7 339 313	27 203 323		34 542 636
Iran, Islamic Republic of	4 285 508	15 884 330		20 169 838
Iraq	7 716 915	28 602 915		36 319 830
Jordan	2 039 823	7 560 647		9 600 469
Kuwait	255 621	947 464		1 203 084
Lebanon	1 029 468	3 815 745		4 845 213
Libyan Arab Jamahiriya	1 256 319	4 656 573		5 912 892
Morocco	4 733 710	17 545 600		22 279 310
Oman	244 670	906 875		1 151 545
Pakistan	18 022 898	66 802 263		84 825 160
Palestine	1 646 488	6 102 745		7 749 234
Qatar	138 939	514 979		653 918
Saudi Arabia	753 937	2 794 484		3 548 421
Somalia	6 882 148	25 508 831		32 390 979
Sudan	8 723 677	32 334 498		41 058 174
Syrian Arab Republic	3 754 424	13 915 856		17 670 280
Tunisia	2 723 781	10 095 753		12 819 534
United Arab Emirates	316 041	1 171 414		1 487 456
Yemen	6 622 751	24 547 370		31 170 121
Total	92 000 000	341 000 000		433 000 000

Section 2. Regional programme statements by strategic objective

Strategic Objective 1: Reduce the health, social and economic burden of communicable diseases

Regional issues and challenges

a) Communicable disease surveillance, forecasting and response

The implementation of the International Health Regulations (2005) is greatly dependent on the implementation of national, regional and global surveillance and containment strategies that rapidly detect, verify and respond promptly and effectively to unexpected outbreaks and epidemics at local, national, regional and international levels. Countries of the Eastern Mediterranean Region agreed voluntarily to implement the newly revised International Health Regulations (2005) during the Fifty-ninth World Health Assembly in 2006. However, the countries in the Region need to improve their capacity to respond to outbreaks in a coordinated and multisectoral way.

Many countries in the Region have dynamic daily interaction with other countries all over the world as a result of tourism and movements of expatriate workers and religious visitors. Many countries in the Region face the following challenges.

- Inadequate epidemiological and laboratory capacities to early detect and respond to public health emergencies of international concern
 - Shortage in trained epidemiologists
 - Rapid turnover of national focal points
 - Migration of trained health personnel to other countries
 - Inadequately equipped national public health laboratories. Although some countries have established excellent laboratories, most countries lack public health laboratories that deal with many emerging diseases, especially viral diseases.
 - Lack of appropriate training programmes in communicable diseases surveillance and response
- Inadequate acquisition, maintenance and utilization of epidemiological and laboratory databases to identify trends of disease occurrence, and early detect emerging and re-emerging communicable diseases
 - Limited use of maps
 - Limited data on the increasing resistance of micro-organisms to antimicrobial drugs.
- Inadequate transparency of Member States in reporting public health emergencies of international concern. Some countries do not fully report communicable disease outbreaks out of concern for potential economic losses that may be associated with reduction of flow of tourists.
- Inadequate communication system for surveillance of communicable diseases
- Inadequate resources to respond to public health emergencies of international concern. In many countries, support for surveillance and forecasting activities is, generally, greatly reduced between outbreaks.
- Political instability in many countries due to man-made and natural disasters which result in relative disruption of health services, including the communicable diseases surveillance system, and promote the spread of infectious diseases.
- Inadequate involvement of the private medical and health services in many countries in surveillance and response to emerging public health emergencies of international concern. This has led to further underreporting of communicable diseases and delay in detection of outbreaks
- Inadequate integration of surveillance activities with other relevant disciplines; e.g. the veterinary sector.
- Need to revise some national public health laws to be consistent with the International Health Regulations (2005).

b) Neglected tropical diseases and zoonoses

Sudan continues to be the only country in the Region endemic for dracunculiasis. The absolute majority of cases are reported from southern states, while northern states report only a few imported cases. The principal issue for the eradication of dracunculiasis in Sudan has been the maintenance of the disease surveillance system in the previously endemic areas in the northern states, establishment of surveillance systems in previously inaccessible areas in the southern states and achievement of high coverage of new cases with containment. The continued insecurity in some areas in southern states and the weakness of surveillance and supervision will be the main challenges for the national dracunculiasis eradication programme in Sudan.

The elimination of leprosy as a public health problem was achieved at the national level in all Member States by 2000. By the end of 2005, the majority of countries further reduced the prevalence of leprosy to less than 2 per 10 000 at the district level. With the reduction of the burden of leprosy, countries are facing the problem of maintaining results already achieved in the elimination of leprosy through the integration of leprosy diagnosis, treatment and referral services within available public health systems. The main challenges for the achievement of leprosy elimination at subnational level are: the security situation in some countries; presence of high stigma; and continued backlog of unidentified cases. Prevention and rehabilitation of disabilities among leprosy cases is also a challenge.

Lymphatic filariasis is endemic in Egypt, Sudan and Yemen. Mapping of endemic areas has been completed in Egypt and Yemen, and all populations at risk in these two countries have been covered by mass drug administration (MDA). Sudan is continuing mapping of endemic areas. The main challenges for the national programmes in Egypt and Yemen are verification of lymphatic filariasis elimination in communities covered with MDA and maintenance of surveillance systems in former endemic foci. The main issues for Sudan are achieving full coverage of endemic areas with mapping activities, preparation of the national plan for MDA, integration of activities within available public health system and securing community participation.

Schistosomiasis and intestinal parasitic infections continue to remain important public health problems in Sudan and Yemen, and partly in Somalia. Long-standing integrated schistosomiasis control and deworming programmes need to be sustained in countries with low endemicity (Jordan, Libyan Arab Jamahiriya, Oman) and in countries that recently achieved control targets (Afghanistan, Egypt, Morocco and Saudi Arabia). For 2010–2011, the major challenge will remain how to sustain the programmes for the long term and prevent the demobilization that occurs when elimination targets are met and that can lead to the re-emergence of transmission. Lessons must be drawn from the Blue Nile project in Sudan, where the prevalence of schistosomiasis is now among the highest in the world despite the fact that disease was almost eliminated several years ago. Another challenge is to ensure local and sustainable sources of funds to cover regular and uninterrupted mass treatment campaigns in primary schools at the most peripheral level in areas that are insecure or difficult to access in Somalia, Sudan and Yemen.

Different forms of leishmaniasis entities occur in the Region, each requiring a specific control strategy for prevention. Anthroponotic visceral leishmaniasis (AVL) due to *Leishmania donovani* (archibaldi) regularly causes severe outbreaks in Sudan with thousands of deaths. Anthroponotic cutaneous leishmaniasis (ACL) caused by *L. tropica* is a major problem in the Region, particularly in the Syrian Arab Republic (more than 20 000 cases in 2003) and in some urban foci in Afghanistan. Zoonotic cutaneous leishmaniasis (ZCL) outbreaks due to *L. major* continue to appear periodically in desert zones (in Egypt, Islamic Republic of Iran, Iraq, Jordan, Libyan Arab Jamahiriya, Morocco, Pakistan, Syrian Arab Republic and Tunisia). Most countries except Afghanistan, Somalia and Sudan have satisfactory surveillance/case management systems. The major challenges regarding these diseases are the absence of evidence-based validated strategies for prevention or interruption of transmission and support for the high cost of diagnostic and treatment tools. In 2010–2011, the focus will be on adequate operational support to continue to respond to leishmaniasis (AVL and ACL) outbreaks (provision of drugs, diagnostic tools, wider coverage in southern Sudan) and on the development of new approaches for prevention and control through operational research.

Trypanosomiasis control targets were achieved in all foci of southern Sudan. The focus will continue to be on consolidating the achievements obtained, implementing an integrated and sensitive surveillance system and expanding vector control activities in limited areas. The major challenge is to strengthen local capacity to take over implementation and management of the programme from nongovernmental organizations.

Brucellosis, hydatidosis and rabies remain the major zoonotic diseases in the Region, and have a considerable economic impact on veterinary public health. All these zoonoses require specific actions or programmes focused on animals in order to be prevented or eliminated. This is not happening in the majority of countries of the Region because of the absence of multisectoral approaches or consensus on control between the veterinary and other health sectors. Most zoonosis control programmes are limited in surveillance and management of human cases. Rabies continues to constitute a major threat, especially in Afghanistan, Pakistan, Yemen and parts of the Islamic Republic of Iran and Morocco. Considerable funds are spent by most countries of the Region in post-exposure treatment of humans to reduce mortality. It is planned in 2010–2011 to continue progress towards targets set by the Regional Committee in 2003 in its resolution on control of zoonotic diseases (EM/RC50/R.11). In particular, plans are to coordinate with the Mediterranean Zoonosis Control Programme (MZCP) and other regional and international organizations, such as the OIE and FAO, in order to enhance multisectoral technical support to countries for control activities and to coordinate with the MZCP in the Region to expand implementation of a regional control programme for brucellosis and strengthen capacity-building in zoonosis control.

c) Vector control

Vector-borne diseases are responsible for almost 20% of the estimated global burden of infectious diseases. Eleven per cent (11%) of the total estimated burden of vector-borne diseases affects the countries of the Region, where only 8% of the world's population lives. Such diseases include: malaria, leishmaniasis, dengue and dengue haemorrhagic fever, Rift Valley fever, Japanese encephalitis and Crimean–Congo haemorrhagic fever, onchocerciasis, African trypanosomiasis and lymphatic filariasis. The Region has also recently witnessed a significant re-emergence of vector-borne diseases. Some of these diseases are showing an increasing trend to spread to areas where they did not occur before, to expand their transmission season and to intensify their transmission in places where they are already present. Factors responsible for this re-emergence include ecological and climatic changes and human activities such as development projects, civil strife, urbanization and population movement.

Following the malaria eradication era, vector control, one of the key strategic approaches for vector-borne disease control, lost its perceived importance in many countries. This was reflected in reduction of resources allocated to vector control, with a subsequent loss in national capacities for vector control. Currently, the implementation of vector control programmes is not coordinated among the different relevant sectors, resulting in loss of prevention and control opportunities and duplication and wastage of meagre resources. Implementation of vector control continues to rely on a single tool, the use of insecticides, and no new insecticides have been produced over the past 20 years. This calls for judicious use of the few effective insecticides (mainly pyrethroids) currently remaining.

d) Vaccine-preventable diseases and immunization

The ultimate goal for 2015 is that “no child will die from a vaccine-preventable disease in our Region”. The way to achieve this goal is to make sure that every child receives a safe vaccine for each vaccine-preventable disease, and that new and improved vaccines of regional importance are added to the vaccination schedule as soon as they become available. National routine immunization programmes, in collaboration with WHO and various partners, have been working towards achieving important regional goals by 2010, including measles elimination and DPT3 coverage greater than 90% at national level, with at least 80% coverage in all districts, as well as reducing under-five mortality due to vaccine-preventable diseases. Accordingly, the strategies of WHO and other Expanded Programme on Immunization (EPI) partners need to be articulated and coordinated in a way that all

constraints are addressed and that approaches recommended in the Global Immunization Vision and Strategy (GIVS) are adequately implemented.

Increasing access to routine immunization will constitute the main priority. Despite remarkable achievements in increasing coverage in 2007, particularly in Yemen and Sudan, approximately 2.2 million infants in the Region still did not receive basic vaccines. More than 95% of these infants live in Afghanistan, Pakistan and Somalia, where progress towards universal access to routine immunization has been slow despite technical and financial support from WHO, UNICEF, GAVI and other partners. The slow progress in these countries is primarily related to weak managerial capacity, lack of ownership and accountability, and lack of security (Afghanistan, Somalia, southern Sudan).

In parallel, several countries have been unable to establish appropriate surveillance systems to monitor EPI activities and performance and the impact on disease burden. Efforts to document disease burden relating to some major vaccines that are or will be soon on the market, such as rotavirus and *Streptococcus pneumoniae* vaccines, is also a high priority. Surveillance for vaccine-preventable diseases is weak, fragmented and of poor quality in several countries, contrasting most of the time with a strong acute flaccid paralysis (AFP) surveillance system. Moreover, in some other countries, surveillance for vaccine-preventable disease is more or less working, but data generated are rarely used for action due to lack of communication and coordination between the surveillance unit and the national EPI unit.

Regarding new vaccine introduction, 6 countries have not introduced *Haemophilus influenzae* b (Hib) vaccine yet, including (as of 15 June 2008) 2 countries eligible for support from the GAVI Alliance (Afghanistan, for which the GAVI Alliance has approved support for Hib vaccine introduction in 2009, and Somalia) and 4 low-middle income countries (Tunisia, Egypt, Islamic Republic of Iran and Iraq) which, despite the well documented high Hib disease burden, have so far been unable to introduce the vaccine for financial reasons. Accordingly, and in order to solve this issue and assist all countries, including the low-middle income ones, to introduce new vaccines, efforts are in process to raise the awareness of global and regional partners as well as to upgrade the national decision-making capacity and to strengthen vaccine procurement and regulation systems, including establishing sub-regional or regional pooled vaccine procurement systems. In order to strengthen the decision-making process, and prepare for rotavirus and pneumococcal vaccines introduction, the Regional Office has launched three surveillance networks and has been working in collaboration with several partners such as the Hib initiative, Rota-ADIP and Pneumo-ADIP. These networks need to be strengthened in terms of quality and efficiency and expanded to more countries.

Regarding measles elimination, with the recent successful campaigns conducted in priority countries, the estimated measles mortality decreased from 97 000 in 1999 to only 23 000 in 2006 (76.3% reduction), which means that the Region is on the track to reach the 2010 measles mortality reduction goal (90% reduction in comparison with 1999 level). However, the regional goal is measles elimination by 2010, and stronger and more sustainable efforts have to be deployed in the Region, particularly in countries where measles transmission is still ongoing (measles incidence rate higher than 1 per 100 000 population). Moreover, achievements made through supplementary immunization activities conducted in countries with low routine immunization coverage (Afghanistan, Djibouti, Pakistan, Somalia, Sudan and Yemen) need to be sustained, either through stronger routine immunization or through frequent follow-up campaigns. This should be decided based on regular monitoring of susceptibility to measles. Unfortunately, measles surveillance is still weak in several countries, even among those that have implemented nationwide vaccination campaigns.

Successes in measles elimination, and the use of rubella vaccine (MMR) for routine immunization in several countries, have resulted in an important increase in rubella cases and outbreaks among older age groups and have increased the risk of congenital rubella syndrome (CRS) in some countries. Accordingly, the Regional Office has developed a regional strategy to eliminate CRS in countries using rubella vaccine by 2010, and has been supporting countries to develop and implement appropriate immunization strategies and surveillance activities. So far, only a few countries are

implementing those strategies. For 2010–2011, stress should be put on expanding CRS elimination activities to the remaining countries.

e) Operational research in tropical diseases

Communicable diseases are one of the greatest barriers to the achievement of the global health agenda. Excluding HIV/AIDS, tuberculosis and malaria, communicable diseases account for 20% of deaths in all age groups, 50% of child deaths and 33% of deaths in the least developed countries. Without a reduction of disease burden, the achievement of other health-related goals, as well as of goals related to education, gender equality, poverty reduction and economic growth, will be put in jeopardy. Thus combating the burden of communicable diseases is a key component of the WHO strategies to achieve the Millennium Development Goals. Research is needed to evaluate new vaccines, devise innovative interventions to increase vaccine coverage, and estimate disease burden in order to guide policy about vaccine introduction. Research is also needed to investigate cost-effective approaches for adequate preventive or therapeutic care for neglected tropical diseases; devise and evaluate new interventions for their early detection/diagnosis and successful treatment; and determine the predictors of morbidity and mortality in the community and health systems and devise suitable interventions to reduce their burden. A culturally appropriate and pro-poor research agenda should always take into consideration the cost-effectiveness of the different tools and interventions and their adaptability to remote areas with limited facilities and hard-to-reach populations.

A common challenge facing neglected tropical diseases is the lack of effective vaccines and diagnostic tools. Research to evaluate new technology for prevention and early diagnosis of these diseases is therefore essential to enhance disease control. New drugs and simplified regimens could also be evaluated in the Region.

Epidemics and emerging diseases can place sudden and intense demands on health systems. The predictors of epidemics could be studied in order to allow better preparedness and response to these epidemics. The epidemiology of the emerging diseases should be elucidated to assist in the development of vaccines and drugs. New tools and strategies to accelerate the interruption of wild poliovirus could be also devised to enhance complete interruption of polio transmission.

Facing the above challenges in research would depend on the research capacity of researchers in the Region and their ability to produce and disseminate valid and reliable results. Therefore, research capacity-strengthening is crucial to the achievement of targets for communicable disease control. Special emphasis should also be given to providing technical support in scientific writing in order to validate the results produced.

Monitoring of the translation of the research results into practice by the national control programmes should be enhanced. The impact of the research results on disease control should also be evaluated.

f) Polio eradication programme

The goal for the polio eradication programme is to interrupt poliovirus circulation and achieve polio-free status for all countries, leading to certification of the Region as polio-free. Only two of the 22 countries have not interrupted endemic transmission of poliomyelitis (Pakistan and Afghanistan). These two countries have witnessed a reduction in the total number of cases, as well as a geographic and genetic restriction of the poliovirus circulation. This is primarily due to the numerous high quality supplementary immunization activities conducted in the past two years.

Countries which have experienced outbreaks due to importation after being polio-free for several years have also succeeded in stopping these outbreaks (Sudan, Yemen and Somalia). Strong AFP surveillance systems are established in the countries to ensure timely identification of wild poliovirus cases, whether related to endemic transmission or importation. National and sub-national immunization days are conducted in several other countries at high risk for importation and with weak routine immunization coverage.

A critical step in the end-game of polio eradication is certification. All countries have submitted reports to the Regional Certification Committee. The national certification documents from 18

countries were accepted. The national certification document for Yemen is pending review. Somalia is submitting a provisional document since the transmission was stopped in March 2007 and will submit in April 2009 the national certification document, while Afghanistan and Pakistan are submitting yearly progress reports. Additionally, laboratory containment of wild poliovirus and potential infectious materials has also started with 17 countries completing phase one of the containment activities and 15 submitting quality assessment reports. The Regional Office continues to provide advice and support for countries on OPV cessation policies. Currently all countries either use OPV alone or in combination with IPV in their routine immunization schedule.

Strategic approaches

a) Communicable disease surveillance, forecasting and response

- Supporting implementation of the International Health Regulations (2005). The Regional Office will strive to strengthen national early warning, surveillance and response systems and epidemic preparedness through:
 - strengthening national epidemiological capacities by supporting different training activities, including field epidemiology training programmes
 - strengthening national public health laboratories
 - supporting establishment of regional reference laboratories
 - supporting cross-border joint surveillance activities
 - promoting transparency of countries in reporting public health emergencies of international concern.
- Strengthening regional capacity to respond to public health emergencies of international concern that could occur within the Region through:
 - supporting development of a regional web-based surveillance system
 - supporting a regional response capacity for containment of known epidemic-prone infections, emerging infectious risks and unexpected disease threats, including those that may be deliberately initiated, through global epidemic intelligence and global outbreak alert and response
 - participating actively in the Global Outbreak Alert and Response Network (GOARN).

b) Neglected tropical diseases and zoonoses

All global strategic approaches are relevant to the achievement of the regional objectives. In particular, countries will need to invest sufficient human, political and financial resources in ensuring high quality interventions for prevention, detection, treatment and control of communicable diseases among all at-risk populations. Specific approaches are related to the organization of surveillance and control activities in areas with complex emergencies, namely in southern Sudan.

c) Vector control

Prevention is a key global strategy for the control of vector-borne diseases. The Regional Office has continued to provide support to countries to ensure that intervention tools for the prevention of vector-borne diseases such as insecticide residual spraying, insecticide-treated bednets and larval control, are applied timely and appropriately. This is always done with equity, maximum coverage for high impact and sustainability of such interventions in mind.

Successful implementation of vector control for vector-borne diseases relies not only on effective intervention tools, but also on the availability of national capacity to deliver such tools. It was for this reason that the 52nd Session of the Regional Committee in 2005 endorsed the regional strategic framework on integrated vector management for the prevention of vector-borne diseases. This is an approach with the potential to sustain such capacities through collaboration among different sectors. In implementing the regional strategy for the prevention of vector-borne diseases, the following are the priority interventions:

- Development and incorporation of integrated vector management into national health policies
- Establishment/and or strengthening of collaboration among relevant sectors at national level

- Scaling up and implementation of effective vector control interventions tools for high disease impact
- Strengthening of national infrastructure and human resource capacity for integrated vector management
- Strengthening national capacity in entomological surveillance and in management of insecticides and resistance
- Validation and testing of appropriate new products and tools for vector control through operational research
- Strengthening national capacity to prepare and respond to major vector-borne disease epidemics.

d) Vaccine-preventable diseases and immunization

In supporting the efforts of Member States, the WHO Secretariat will focus on:

- Strengthening its leadership and collaboration with global health stakeholders, partnerships and civil society in working with Member States to articulate ethical and evidence-based policies, and to facilitate expanded, and reduced disparity in, community access to existing and new tools and strategies, including vaccines, that meet acceptable standards of quality, safety, efficacy and cost-effectiveness;
- Strengthening its capacity to fulfil its obligation to provide technical assistance, to build capacity, and to respond to Member States; this includes facilitating national and international resource mobilization and advocacy efforts;
- Providing Member States with tools, strategies, and technical assistance for the evaluation, assessment, and strengthening of monitoring and surveillance systems;
- Coordinating integrated surveillance activities at the regional levels for informing policy decisions and public health responses;
- Strengthening the capacity of Member States to undertake health research especially on the development of tools and strategies for the prevention, early detection, diagnosis, treatment, and control of communicable diseases.

e) Operational research in tropical diseases

- Generating in-depth information about the epidemiology of communicable diseases, and community and health system factors affecting effective preventive and therapeutic care;
- Evaluating new and improved tools, including drugs, vaccines and diagnostics, new/improved interventions, and public health policies devised for the prevention and control of communicable diseases;
- Strengthening capacity of researchers in research methods, data management and analysis and scientific writing;
- Forging a close link between researchers in academic institutions and national control programmes;
- Monitoring and evaluating the implementation of research results by the national control programmes for communicable diseases;
- Disseminating research results for their effective use in the prevention and control of communicable diseases.

f) Polio eradication programme

- Supporting countries to achieve certification of poliomyelitis eradication by ensuring high routine immunization coverage, strengthening and refining the supplementary immunization activities and ensuring certification standards of AFP surveillance
- Ensuring destruction and containment of poliovirus and potential infectious material.

*Regional assumptions and risks and option analysis***a) Communicable disease surveillance, forecasting and response**

Assumptions for achieving the strategic objective are that: necessary resources are made available in both Member States and the Regional Office; and that entry into force of the International Health Regulations (2005) will translate into a renewed commitment by all Member States to strengthen their national surveillance and response systems; demonstrate transparency; and invest in capacity-building

Risks that may adversely affect the achievement of strategic objectives are: emergence of competing priorities such as man-made and natural disasters (e.g. civil unrest, earthquakes, etc.); and emergence of pandemic influenza.

b) Neglected tropical diseases and zoonoses

The following assumptions are made for achievement of the strategic objectives: national surveillance and response systems will be further strengthened; and cooperation of all partners in elimination/eradication of diseases will continue.

The following risks may adversely affect the achievement of the strategic objectives:

- Insufficient financial support by donor agencies for national programmes. Global, regional and national fund-raising activities should be further strengthened to support the implementation of planned activities
- Weakness of public health surveillance and control systems. Promotion of integrated approaches and community-based interventions should be supported.

c) Vector control

The prevention of vector-borne diseases through integrated delivery of intervention tools depends to a great extent on political will and the strength of the national intersectoral coordination mechanisms. The endorsement of integrated vector management by Member States in the Region is evidence of their political commitment.

Assumptions

- It is assumed that countries in the Region will maintain and sustain the political commitment to implement integrated vector management through intersectoral coordination
- The capacity of a core team of national staff to plan and implement vector control intervention tools for vector-borne diseases will grow continuously
- New products and intervention tools for vector control will be made available through a global alliance between the public and private sectors. Currently the number of options available are limited and national efforts are threatened by the development and expansion of vector resistance

Risks

- Inability to sustain intersectoral coordination/mechanism at national level by the different sectors through either competition or lack of strong leadership by the Ministry of Health
- Continued emphasis on curative methods of vector-borne disease control at the expense of prevention
- Countries withdrawing vector control resources for example when they have achieved elimination of one of the vector-borne disease without taking into account the potential risk of re-introduction of the same disease or others that benefited from the same interventions
- Prevention and elimination of vector-borne diseases in low-income countries rely to a great extent on international support. Dwindling of such resources could negatively affect prevention of vector-borne diseases.

d) Vaccine-preventable diseases and immunization

The strategic objective can be achieved under the following assumptions:

- Member States invest human, political, and financial resources in ensuring and expanding equitable access to high quality and safe vaccines
- Member States establish and maintain effective coordination mechanisms with all partners and across all relevant sectors at the country level, and work with the WHO secretariat in extending these coordination mechanisms to the regional and international spheres.
- National involvement in research is increased, through achievement of the objectives for investment in health research and research capacity strengthening and towards integration of research into the mainstream of national programme activities and plans.

The main risks that may adversely affect the achievement of the strategic objective are:

- Increased pressure to divert resources away from vaccine-preventable diseases prevention and towards other aspects of health, and EPI not being recognized as a priority on health agendas, particularly in the least developed countries.
- Continuation of current insecurity, war and unrest in the Region.
- Failure to complete interruption of polio transmission by end 2007, resulting in additional supplementary immunization activities, and hence, additional costs.
- An influenza pandemic causing unprecedented morbidity and mortality, and grave economic harm.

e) Operational research in tropical diseases

This strategic objective can be achieved under the following assumptions:

- National involvement in research is increased, through achievement of the objectives for investment in health research. Research capacity-strengthening and integration of research into the mainstream of national programmes and plans will be critical to improved access to, and use of, research findings.
- Political commitment to allocate 2% of the national health budgets of the Region to research is ensured.
- Communicable diseases, excluding HIV, tuberculosis and malaria, are recognized as a priority for the national and international health agendas and resources for research are secured and mobilized.

The primary risk that may adversely affect the achievement of the strategic objective is the inadequacy of the private sector and unilateral efforts to secure the funds to bridge the gaps in investment in research, which were identified more than a decade ago: less than 10% of global health research resources are spent on health problems that affect 90% of the world's population.

f) Polio eradication programme

The assumptions of the regional poliomyelitis eradication program are as follows:

- Member States will:
 - continue to exhibit high political commitment to ensure the quality and prioritization of the polio eradication programme activities.
 - sustain sensitive AFP surveillance performance assessed through basic indicators at national and sub-national levels. High risk countries are expected to achieve higher standards of performance to be able to stop on-going transmission and address the ongoing risk of importation.
 - achieve high OPV3 coverage through routine immunization programmes and conduct vaccination campaigns to supplement any gaps to ensure high population immunity required to stop transmission and be prepared for any possible importation.
- Partners will supplement national resources as required to ensure the availability of the necessary funds to conduct the activities in the last steps towards eradication.

- WHO will continue to provide Member States with the necessary technical support related to polio eradication activities and advise on updates regarding OPV cessation policies.

The main risks that can impede the achievements of the goals of the programme are:

- Continued inaccessibility due to insecurity and active fighting in the remaining endemic countries. As well as the political instability impacting the performance of standard programme activities in polio-free countries.
- The spread of misconceptions and rumours related to vaccine safety leading to refusals in communities, hindering efforts to ensure all children are immunized with multiple OPV doses, especially in remaining endemic countries.
- Risk of importation from remaining endemic countries of the world to polio-free countries with low routine immunization coverage and AFP surveillance gaps.
- Unavailability of the needed resources to conduct required activities such as supplementary immunization activities thus increasing the risk of outbreaks due to importation.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
01.001	Policy and technical support provided to Member States in order to maximize equitable access of all people to vaccines of assured quality, including new immunization products and technologies, and to integrate other essential child health interventions with immunization.	01.001. EM01	Appropriate policy, tools, and technical support provided to all Member States to improve equitable and sustainable access to vaccines of assured quality and to expand immunization beyond infancy within the framework of regional and national disease reduction goals in the context of health systems strengthening and links with other essential health interventions	Proportion of countries with > 90% national coverage and > 80% DTP3 coverage in all districts	16	18
				Reduction in measles mortality compared to baseline in 1999	80%	90%
				Number of countries with routine vaccination programmes that are extended into the second year of life and entry into primary school	16	21
		01.001. EM02	Appropriate policy, tools, and technical support provided to all Member States to introduce new vaccines and technologies within the framework of regional and global disease reduction goals and GIVS	Number of countries introducing Hib vaccine	16	20
				Number of countries introducing rotavirus vaccine	0	6
				Number of countries introducing pneumococcal vaccine	3	10
		01.001. EM03	Effective collaboration with GAVI partners for the accelerated introduction of childhood vaccines in all countries	Number of GAVI eligible countries with second measles vaccine dose introduced	0	5
01.002	Effective coordination and support provided in order to achieve certification of poliomyelitis eradication, and destruction, or appropriate containment, of polioviruses, leading to a simultaneous cessation of oral poliomyelitis vaccination globally.	01.002. EM01	Effective coordination and provision of support to Member States to achieve certification of poliomyelitis eradication and destruction, or appropriate containment of polioviruses, leading to a simultaneous cessation of oral polio vaccination globally	Sustaining high population immunity (at least 90% of 6-59 months non-polio AFP cases with >3 OPV)	20	All countries
				Achieving and sustaining a highly sensitive AFP surveillance system: the 8 priority countries attain non-polio AFP rate > 2 per 100,000 < 15; other countries achieve a rate > 1 per 100 000 and all countries achieve > 80% stool adequacy	All countries	Indicator sustained in all countries
				Number of countries having completed Phase I of laboratory containment of wild poliovirus and potentially infectious material, and submitted report on quality	20	22

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
				of containment activity		
01.003	Effective coordination and support provided to Member States in order to provide access for all populations to interventions for the prevention, control, elimination and eradication of neglected tropical diseases, including zoonotic diseases.	01.003. EM01	Progress in elimination of dracunculiasis, leprosy and lymphatic filariasis will be achieved according to regional targets	Number of dracunculiasis cases	> 20 000	< 4000
				Number of countries having achieved elimination of leprosy at sub-national levels	19	22
		01.003. EM02	Control of schistosomiasis, soil-transmitted helminths, trypanosomiasis and leishmaniasis will be expanded and sustained	Coverage of at risk school-age children with regular deworming (including schistosomiasis) treatment	0 to less than 10%	50%
				Sustained trypanosomiasis prevalence below 3%	<3%	<3%
				Number of areas in Sudan not covered by leishmaniasis treatment centres	5	2
		01.003. EM03	Multisectoral programmes to control zoonoses adopted and implemented	Number of programmes jointly coordinated/implemented by Ministries of Health and Agriculture	3	6
		01.003. EM04	Vector-borne disease endemic countries are supported to develop, implement, monitor and evaluate integrated vector management (IVM) into national health policies	Number of vector-borne disease-endemic countries that have developed national IVM strategic plans	8	22
				Number of countries implementing IVM	3	15
				Number of countries with an intersectoral coordination mechanism at national level	10	15
				Number of vector-borne disease-endemic countries reporting at least 80% coverage of key interventions (ITNs/LLINs and IRS)	3	15
				Number of vector-borne disease-endemic countries with a functional vector control unit at the central level	3	15
				Number of vector-borne disease-endemic countries that have mapped the distribution of vectors and their resistance to the lowest administrative level	2	22
				Number of vector-borne disease-endemic countries implementing WHO guidelines on pesticides practices and management	0	22
		01.003. EM05	Endemic countries to scale up control/elimination/eradication programmes for neglected tropical diseases	% coverage of target population by chemotherapy in Sudan, Yemen and Somalia	0 to less than 10%	50%
01.004	Policy and technical support provided to Member States in order to enhance their capacity to carry out surveillance and monitoring of all communicable diseases of public health importance.	01.004. EM01	Support provided by WHO at all levels to Member States to develop, strengthen and maintain the capacity to detect, assess, notify, report and respond to events in accordance with IHR (2005)	Number of countries with surveillance systems and training for all communicable diseases of public health importance	14	18
				Number of countries receiving technical support from WHO to adapt generic surveillance and communicable disease monitoring tools or protocols to specific country situations	10	16

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
		01.004. EM02	Appropriate policy, tools, and technical support provided to all Member States to build/enhance capacity to monitor of the performance of the national immunization programme and its impact on disease burden	Number of countries with monitoring systems for district-level coverage data according to agreed-upon indicators	18	22
				Number of countries achieving > 80% of all surveillance indicators for measles	16	20
				Number of countries with functioning AEFI surveillance system	15	20
				Number of countries for which joint reporting forms are received on time at regional level in accordance with established timelines	17	21
		01.004. EM03	Appropriate policy, tools, and technical support provided to all Member States to enhance capacity to make evidence-based decisions for new vaccine introduction	Number of countries with ongoing surveillance for bacterial meningitis	18	22
				Number of countries with information documenting disease burden for rotavirus vaccine introduction	10	18
				Number of countries with information documenting disease burden for pneumococcal vaccine introduction	9	17
		01.004. EM04	Effective collaboration with GAVI partners for strengthening vaccine-preventable disease surveillance in countries	Number of GAVI eligible countries benefiting from GAVI support to strengthen their vaccine-preventable disease surveillance	0	5
	01.005 New knowledge, intervention tools and strategies that meet priority needs for the prevention and control of communicable diseases developed and validated, with scientists from developing countries increasingly taking the lead in this research.	01.005. EM01	Reduced opportunities for human infection with H5N1, and consultation on areas for research, soliciting proposals and support to successful proposals	Number of reports published on global research needs and priorities for a disease, or type of intervention.	10	25
		01.005. EM02	Support provided by WHO at all levels to Member States to develop capacities, including facilities and resources, at points of entry in accordance with IHR (2005)	Number of new and improved tools (e.g. medicines, vaccines or diagnostics or other interventions) or implementation strategies developed with significant WHO contribution which have been translated into public sector use in at least one developing country	2	7
				Number of peer reviewed publications arising from WHO supported research for which the first author's institution is from a developing country	12	14
		01.005. EM03	Vector-borne disease endemic countries are supported in validating and testing appropriate new products and tools for vector control	Number of vector-borne disease-endemic countries testing and validating new vector control intervention tools	1	8
				Number of vector-borne disease-endemic countries applying the results of new vector control intervention tools	0	8
		01.005. EM04	Technical and financial support provided to countries to conduct operational and applied research for new immunization strategies, vaccines and	Number of research studies and/or projects supported by WHO	3 per year	10 over 2 year period

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
			technologies			
		01.005. EM05	By the end of 2009, new knowledge, new improved tools, interventions and public health strategies for communicable diseases will be generated and/or validated; research capacity strengthened, results disseminated and translated into policy and practice of the national control programmes, and partnership established	Proportion of new, significant and relevant scientific advances in biomedical, socio-behavioural, economic and public health strategies Proportion of new/improved drugs, vaccines, diagnostic tests, validated and approved Number of countries using the generated knowledge in promoting community-based interventions	14% of the funded projects evaluated new/improved interventions; 30% evaluated new tools; 41% generated new epidemiological information and 14% evaluated new/improved public health strategies	At least 10% of the funded projects within this expected result will lead to halving of the morbidity/mortality of a communicable disease or result in its elimination as a public health problem. At least another 10% of the funded projects will be subjected to follow-up studies due to the importance of their results and their potential for effective prevention and control of communicable diseases
				Research results of national/international significance Number of organized workshops, training courses, follow-up visits, and recruited consultants	Research methodology workshops (2001 to 2005); Follow up visits to Sudan (2002-2005) and Yemen (2003); Workshop on Project planning and evaluation, Sudan 2005 Recruiting consultants for technical support	Researchers/institutions previously supported by the scheme able to compete in the international arena of grants
				Number of articles originating from SGS funded projects published in indexed journals Number of annual meetings organized in the Region to disseminate results; Number of annual reports from WHO research programmes communicated to ministries of health Publication of the final report summaries, and other research products; Number of visits to the SGS web site	Regular posting of research products and events on the SGS web site; Publication of the final report summaries and multicountry study in tuberculosis; Compilation of 90 articles originating from the SGS projects during 1992-2002; 33 articles originating from 38 final reports of the	Sustaining a biannual publication of final report summaries for 2 successive rounds; at least 10% annual increase in articles published in indexed journals; Publishing overviews of multicountry studies or studies tackling

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
					2000-2001 projects published in a special issue of the EMHJ	similar research topics
				Number of annual reports obtained from ministries of health in response to the follow-up activities Number of research projects that were the subject to future research Proportion of new/improved tools/interventions/public health strategies adopted for disease control	Collecting evidence about the implementation of research results by national control programmes in response to the first call for evidence G7 (to collect evidence about the implementation of research results by ministries of health)	EMRO is receiving feedback from ministries of health for at least 50% of projects implemented during the previous biennium 10% annual increase in the implementation rate of research results by control programmes of the Ministry of Health
				Number of partners; funds mobilized	TDR (US\$ 150 000) US\$ 15 000 (donations)	Partners providing technical and financial support
01.006	Support provided to Member States in order to achieve the minimum core capacities required by the International Health Regulations (2005) for the establishment and strengthening of alert and response systems for use in epidemics and other public health emergencies of international concern.	01.006. EM01	WHO alert and response infrastructure and operations strengthened and adapted at Regional and Country levels in accordance with IHR (2005)	Number of countries that have completed the assessment and developed a national action plan to meet core capacities for surveillance and response, or self-assessment of core capacities for surveillance and response, in line with their obligations IHR (2005)	10	18
				Number of countries whose national laboratory system is engaged in at least one internal and one external quality-control programme for epidemic-prone communicable diseases	14	18
01.007	Member States and the international community equipped to detect, assess, respond to and cope with major epidemic and pandemic-prone diseases (e.g. influenza, meningitis, yellow fever, haemorrhagic fevers, plague and smallpox) through the development and implementation of tools, methodologies, practices, networks and partnerships for prevention, detection, preparedness and intervention.	01.007. EM01	Support for preparedness plans for human pandemic influenza at regional and country levels	Number of countries having national preparedness plans and standard operating procedures in place for readiness and response for major epidemic-prone diseases (e.g. meningitis, yellow fever, pandemic influenza)	10	15
				Number of international coordination mechanisms for supply of essential vaccine, drugs and equipment for major intervention	4	6
				Number of countries with basic capacity in place for safe isolation of infectious cases and safe laboratory handling of dangerous pathogens	4	6
		01.007. EM02	Vector-borne disease endemic countries are equipped to respond to	Number of countries with capacity to respond to vector-borne disease epidemics	2	10

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
			major disease epidemics	Number of countries with contingent stocks (insecticides, spray pumps etc.) to rapidly respond to vector-borne disease epidemics	1	10
		01.007. EM03	Effective collaboration with partners for the accelerated introduction of vaccines against epidemic-prone diseases into countries	Percentage of countries affected by epidemic-prone diseases against which potent and safe vaccines are available, that have introduced these vaccines	0	50%
01.008	Regional and global capacity coordinated and made rapidly available to Member States for detection, verification, risk assessment and response to epidemics and other public health emergencies of international concern	01.008. EM01	WHO interprogrammatic coordination and commitment achieved at all levels to support the implementation of IHR (2005), and adequate resources mobilized at all levels for implementation of IHR (2005)	Number of locations with global event-management system in place to support coordination of risk assessment, communications and field operations for headquarters, regional and country offices	8	14
				Proportion of requests for support from Member States for which WHO mobilizes coordinated international support for disease investigation and containment efforts, characterization of events, and sustained containment of outbreaks	50%	70%
				Median time to verification of outbreaks of international importance, including laboratory confirmation of etiology	9 days	7 days
01.009	Effective response by Member States and the international community to emergencies situations due to epidemic and pandemic prone diseases	01.009. EM01	Partnership for outbreak and crisis response	Number of countries with integrated plan for response to outbreaks and crisis	6	11

Strategic Objective 2: Combat HIV/AIDS, malaria and tuberculosis

Regional issues and challenges

a) HIV/AIDS

In most countries of the Eastern Mediterranean Region, HIV prevalence is estimated to be low among the general population but concentrated among populations most at risk. Access to essential HIV prevention and care services is still very limited in the countries of the Region. Coverage of the estimated number of HIV infected people in need of antiretroviral therapy is with 6%, the lowest among the six regions of WHO. The main challenges for achieving universal access to HIV prevention, treatment and care in the health sector in the Region are:

- Inadequate information on populations at risk of HIV and their access to HIV prevention and care services
- Prevailing stigma and discrimination against groups most at risk of HIV transmission and people living with HIV/AIDS
- Inadequate leadership and coordination of national responses
- Weak health and social systems capacities (including technical and managerial capacities, lack of human resources, physical infrastructure)
- Weak involvement of civil society and private sector as partners
- Cultural and socioeconomic context that inhibits access to services (post-conflict, displacement, migration, illiteracy, poverty).

b) Malaria

Geographical diversity in the Eastern Mediterranean Region determines malaria variability in terms of endemicity, intensity of transmission and type of malaria. In the sub-Saharan countries of the Region and Saudi Arabia and Yemen, *P. falciparum* is the predominant malaria parasite species. In the other endemic countries, mainly Afghanistan, Islamic Republic of Iran and Pakistan, both *P. falciparum* and *P. vivax* are transmitted, in different proportions.

Nine countries have eliminated local malaria transmission: Bahrain, Jordan, Kuwait, Lebanon, Libyan Arab Jamahiriya, Palestine, Qatar, Tunisia and United Arab Emirates. The risk of malaria re-introduction in these countries still exists. In four countries, malaria transmission has either recently been interrupted (Morocco, Oman, Syrian Arab Republic) or it occurs in a very limited residual foci (Egypt). In three countries, namely the Islamic Republic of Iran, Iraq and Saudi Arabia, malaria is still endemic in certain localized areas, and the countries are either planning for elimination or are implementing a malaria elimination strategy. More than 95% of the malaria cases in the Region occur in six countries: Afghanistan, Djibouti, Pakistan, Somalia, Sudan and Yemen. Sudan alone accounts for almost 50% of the total regional burden.

Malaria endemic countries are facing several challenges, including: limited coverage and low quality of laboratory services for diagnosis, particularly in high burden countries, where only one quarter of the clinical malaria cases are laboratory confirmed; weak health information and malaria surveillance systems, which are unable to provide reliable data on the malaria burden; limited access to effective treatment and prevention measures; weak leadership and management skills at national and lower levels; lack of compliance of the private sector with national policies and guidelines; and weak community involvement and lack of community structures to deliver interventions to remote and inaccessible populations.

Countries aiming at malaria elimination are confronted with obstacles that include: limited expertise in malaria elimination and weak national capacity for implementing elimination interventions; lack of accurate and updated stratification maps of malaria transmission; lack of effective strategies for coordination of cross-border activities; and weak intersectoral coordination.

Countries of the Region also face an additional constraint: spreading resistance to antimalarial medicines and insecticides. The unstable political situation in certain areas due to war or civil unrest,

globalization and rapidly changing dynamics in environment, climate and migration also pose formidable challenges.

c) Tuberculosis

Activities related to the DOTS (directly observed treatment, short-course) strategy have been widely expanded, with regional DOTS coverage at 98%. All countries achieved the global target of 100% coverage except Afghanistan, Iraq, Somalia, Sudan, United Arab Emirates and Yemen. Afghanistan is still expanding the DOTS strategy. In Iraq, Somalia, Sudan and Yemen, areas that are not covered are those with security problems.

Treatment success rates are high, with the regional average at 83%. Nine countries have achieved the global target of 85% or higher treatment success rates and another eight countries have achieved rates between 80% and 84%. In five countries (Kuwait, Libyan Arab Jamahiriya, Qatar, Saudi Arabia and United Arab Emirates), treatment outcomes were affected by expatriate patients who left the country during the course of treatment.

The main issue is the very low case detection rate: although the rate has been increasing, the regional average is only 52%. Only eight countries (Bahrain, Jordan, Kuwait, Libyan Arab Jamahiriya, Morocco, Oman, Somalia and Tunisia) have achieved the global target of 70% case detection. In Afghanistan, Egypt, Islamic Republic of Iran, Lebanon, Pakistan and Qatar, case detection rates reached between 50% and 70%. In the remaining 8 countries, case detection was less than 50%. It ranged between 30% and 50% in Djibouti, Iraq, Saudi Arabia, Syrian Arab Republic, Sudan and Yemen, and was less than 30% in Palestine and the United Arab Emirates.

At present, only four countries (Bahrain, Oman, Somalia and Tunisia) have achieved all the global targets.

The first challenge in the Region is the expansion and enhancement of high-quality DOTS activities. Expansion of DOTS activities is still needed in a few countries, particularly in Afghanistan. More importantly, the quality of DOTS activities needs improvement in all countries. The quality of diagnosis, treatment, surveillance, logistics and monitoring activities is not always high. Involvement of all care providers is also a major challenge, as health care providers both in the public and private sectors are not fully involved in DOTS activities.

Addressing emerging challenges such as HIV/AIDS, multidrug resistance and complex emergency is important. Mobilizing and empowering communities to increase their knowledge and demand for good tuberculosis care is needed. Establishing wider partnerships at all levels is also critical, because challenges relating to tuberculosis are vast and relate to different issues in health and socioeconomic development.

d) Operational research, tropical diseases

Paragraph deleted

In order to enhance achievement of the control targets for HIV/AIDS, tuberculosis and malaria, research is needed to: investigate cost-effective approaches for promoting the universal access of preventive or therapeutic care; devise and evaluate new interventions for early detection/diagnosis and successful treatment; determine the predictors of morbidity and mortality in the community and health systems; and devise suitable interventions to reduce their burden. A culturally appropriate and pro-poor research agenda should always take into consideration the cost-effectiveness of the different tools and interventions and their adaptability to remote areas with limited facilities and to hard-to-reach populations including injecting drug users, sex workers and prisoners. The agenda should also address the needs of populations in conflict situations and humanitarian crises and ensure relevance of research to socio-cultural contexts.

Particular emphasis should be given to involvement of the community in disease control, health system research and effective collaboration with the private sector. Health system research can identify

gaps and barriers against universal access to prevention and care and inform policy towards making a change. Deficiencies may be related to the knowledge and skills of the health care workers in raising community awareness and delivering adequate quality of care according to the recent therapeutic guidelines. They may be also attributed to deficient equipment and supplies or to managerial deficiencies.

A common challenge facing the three diseases is the lack of effective vaccines and valid diagnostic tools. Research to evaluate new technology for prevention and early diagnosis of these diseases is therefore essential to enhance disease control. New medicines and/or simplified regimens could also be evaluated in the Region.

Another challenge facing control programmes in the Region for these three diseases is the lack of reliable figures about the true burden of these diseases in the community. Data reported to the public health systems are often underestimated, as reports have shown that the private sector contributes to the management of at least half of the cases. Research is needed to evaluate the true disease burden, and to devise suitable interventions to strengthen surveillance/information systems.

Facing these challenges in research would depend on the research capacity of the researchers in the Region and their ability to produce and publish valid and reliable results. Therefore, research capacity-strengthening is crucial to the achievement of control targets for these diseases. Special emphasis should also be given to providing technical support in scientific writing in order to validate the produced results.

Monitoring and evaluating the translation of the research results into practice of the national control programmes of HIV/AIDS, tuberculosis and malaria should be enhanced, and the impact of the research results on disease control should be also evaluated.

Strategic approaches

a) HIV/AIDS

Major impetus will be given to promoting the delivery and universal access of essential interventions for prevention, treatment, care and support to halt transmission and curtail morbidity and mortality from the three diseases. At the primary care level, these can be harmonized to maximize the effectiveness of a given patient encounter with the health system; and to optimize on the various entry points. Special emphasis will be placed on maximizing prevention; ensuring that the services are also tailored and delivered to the poor, vulnerable groups and hard-to-reach populations, including injecting drug users, sex workers and prisoners; addressing the needs of populations in conflict situations and humanitarian crises; ensuring relevance to sociocultural contexts; and encouraging use of evidence, norms and standards in policy and programme formulation.

Strengthening and supporting human resources and provider networks and enhancing public-private mix will be vital, including training and upgrading the skills of health professionals and community workers; widening the service provision networks and pool of providers; strengthening human resource management capacity; strengthening the engagement of nongovernmental and private sector institutions; strengthening referral systems; tapping the potential of community health workers, persons living with the diseases and family members; and promoting strategies to retain health human resources.

Facilitating the availability and promoting proper use of quality, safe and affordable medicines, diagnostics, insecticides and health commodities; expanding quality-assured laboratory networks; and ensuring well functioning public and private supply chains will also be crucial.

Monitoring, evaluation, and surveillance systems for decision-making, progress monitoring and accountability towards HIV, tuberculosis and malaria targets will be enhanced, as well as improving effectiveness and efficiency of information systems (generation and use of age and sex disaggregated data); strengthening epidemiological and behavioural surveillance; strengthening data collection and analysis capacity (including financial tracking); assessing impact of interventions and trends of the

three diseases in special population groups; refining indicators for key new interventions (such as the long-term impact of antiretroviral treatment for people with HIV/AIDS and resistance monitoring.

Efforts to ensure sustained political commitment, engagement of communities and affected persons, and more effective partnerships will also be critical. Advocacy for concerted efforts to combat the three diseases will be a major factor for success.

Enabling and promoting research, particularly in areas of safe and effective prevention technologies (such as vaccines and microbicides), medicines (including simplified regimens) and diagnostic tools, and operational research to determine effectiveness of service delivery approaches within the different contexts, will also be essential.

In supporting the efforts of Member States, WHO Secretariat will focus on:

- Developing global HIV/AIDS, tuberculosis and malaria policies, strategies and standards.
- Providing technical cooperation and coordination efforts to Member States for the implementation of policies, strategies and standards
- Facilitating availability and proper use of high quality medicines and commodities
- Measuring progress towards global and regional targets and assessing national programme and system performance, financing and impact
- Facilitating partnerships, advocacy and communications
- Supporting global, regional and subregional and intercountry initiatives aimed at prevention and control of HIV/AIDS, tuberculosis and malaria
- Assisting Member States as appropriate to develop and implement mechanisms for resource mobilization and utilization
- Fostering and supporting research and building research capacity in target countries.

The contribution of the Regional Office to combat HIV in the Region is laid out in the document *Strengthening health sector response to HIV/AIDS and sexually transmitted infections in the WHO Eastern Mediterranean Region*. The main strategies are to:

- promote political commitment, mobilize resources and strengthen collaboration with relevant partners within and outside the health sector and with civil society
- promote WHO-recommended policies and to provide strategic and technical guidance
- support countries upon request to meet their technical and operational needs
- strengthen the generation and dissemination of knowledge and strategic information on HIV/AIDS and STI.

b) Malaria

The regional strategic plan for Roll Back Malaria aims at: halving the malaria burden in the countries with high malaria problem; eliminating malaria in countries with low malaria burden; and preventing re-establishment of malaria transmission in malaria-free countries. The plan supports expansion of malaria-free areas through malaria elimination initiatives at subregional level such as in north Africa and the Arabian peninsula, and also malaria-free projects at the subnational level.

The key technical approaches to achieve the objectives are to:

- Promote and facilitate access of populations at risk to reliable diagnosis and effective treatment of malaria
- Promote and scale up application of effective preventive measures against malaria for populations at risk
- Support prevention and control of malaria in epidemic situations, complex emergency and urban settings
- Support countries to strengthen the capacity of the malaria control programme at national and district level in partnership with all relevant agencies
- Support expansion and maintenance of malaria free areas and malaria elimination, wherever feasible
- Strengthen malaria surveillance, monitoring and evaluation system and operational research.

c) Tuberculosis

The Strategic Approach is to implement the Stop TB Strategy. The Stop TB Strategy is based on the DOTS strategy and contains the following five major components: pursue high-quality DOTS expansion and enhancement; address HIV/TB, multidrug-resistant tuberculosis, complex emergency and other challenges; contribute to health system strengthening; engage all health care providers; empower people with tuberculosis and communities; and enable and promote operational research. Countries that have not yet achieved the global targets for tuberculosis control need to drastically improve tuberculosis control by adopting the new Stop TB Strategy. Countries should also complete a multi-year strategic plan to identify strategic directions, activities and anticipated financial need. The Global Plan to Stop TB 2006–2015 is the basis for the national plans. The estimated total cost to implement the Plan is US\$ 56 billion over 10 years; the regional component is US\$ 3 billion. Developing regional and national Stop TB partnerships is essential. In this respect, the Regional Office has facilitated the establishment of the Eastern Mediterranean Partnership to Stop Tuberculosis in order to bring sustainable technical and financial support from partners from both within and outside the Region.

In order to support the countries in implementing the Stop TB Strategy, the Regional Office will focus on:

- Developing and updating regional tuberculosis control policies and strategies.
- Providing technical cooperation and coordination efforts to countries for the implementation of policies and strategies
- Facilitating availability and proper use of high quality tuberculosis medicines and commodities
- Measuring progress towards global and regional targets and assessing national programme and system performance, financing and impact
- Facilitating partnerships, advocacy and communications, particularly through scaling up the Eastern Mediterranean Partnership to Stop TB and national partnerships
- Supporting global, regional and subregional and intercountry initiatives
- Assisting countries as appropriate to develop and implement mechanisms for resource mobilization and utilization
- Fostering and supporting operational research and building research capacity in target countries.

d) Operational research, tropical diseases

- Generating in-depth information about the epidemiology of HIV/AIDS, tuberculosis and malaria, and community and health system factors affecting early detection and effective care including the private sector.
- Evaluating new and improved tools, including medicines, vaccines and diagnostics, new/improved interventions and public health strategies devised for the prevention and control of HIV/AIDS, tuberculosis and malaria.
- Strengthening capacity of researchers in research methods, proposal development, data management and analysis and scientific writing.
- Forging a close link between researchers in academic institutions and national control programmes.
- Monitoring and evaluating the implementation of research results by the national control programmes of HIV/AIDS, tuberculosis and malaria;
- Disseminating research results for their effective use in the prevention and control of HIV/AIDS, tuberculosis and malaria.

Regional assumptions and risks and option analysis

a) General

Enabling HIV, tuberculosis and malaria control programmes to successfully scale up requires a consistent and strong national (all levels) capacity to develop evidence-based policies, analyse their effects, and adjust them as necessary. It also requires substantial increase in resources, reinforcing

health systems and building institutional capacity to solve operational constraints. This strategic objective would be achieved under the following assumptions:

- HIV/AIDS, tuberculosis and malaria will continue to be recognized as a priority on national and international health and development agendas
- Strengthening of national health systems will be accorded a higher profile, with the aim of attaining universal access to essential health services and care
- Partnership mechanisms and involvement of stakeholders will be strengthened with the aim of attaining the agreed targets at national and regional levels, and synergy and coordination among the increasing number of actors in HIV/AIDS, tuberculosis and malaria will become a reality
- Global partners including bilateral donors and multilateral agencies in particular the Global Fund to fight AIDS, Tuberculosis and Malaria, will continue, and increase, to finance AIDS, tuberculosis and malaria care as well as health systems strengthening
- Gender inequalities, discrimination and stigmatization currently fuelling the three diseases will be addressed as priority cross-cutting issues
- Security status will be improved in the countries with complex emergency.

The following risks have been identified that may adversely affect the achievement of the strategic objective:

- Difficulties in raising and sustaining the necessary resources, both for WHO and for Member States, as more and more competing priorities emerge
- Health gains achieved by WHO and Member States in HIV/AIDS, tuberculosis and malaria may not be sustained in the least developed countries if the political and financial commitment is not increased
- Difficulties in sustaining WHO leadership functions and interface within the array of actors, in the midst of growing number of actors and partnerships, increasing competition for resources and special coordination and harmonization challenges
- The security situation could become worse not only in countries with complex emergency, but also in countries that are stable at present.

b) HIV/AIDS

Assumptions

- HIV/AIDS and STI prevention, treatment and care services will become an integral component of essential health service delivery
- Governments and ministries of health will liaise with partners in civil society to provide access to HIV prevention and care services for hard-to-reach populations
- The cost of HIV treatment, including second-line antiretroviral therapy (ART), will decrease

Risks

- Stigma associated with HIV will continue to prevail in society and among health service providers
- Social and economic changes will increase behaviours that put populations at higher risk of HIV
- Improper use of ART and other factors may cause emergence of drug resistance and increase the need for more expensive ART regimens.

c) Malaria

Assumptions

- WHO technical support to priority countries is sustained by field staff
- Intercountry and cross-border coordination mechanisms are functional
- Research and development activities are supported at international and national levels for responding to the changing nature of malaria eco-epidemiology

Risks

- Complacency and loss of interest may ensue when the burden is decreased, leading to weakness in malaria programmes and exposing countries to the risk of reestablishment of local malaria transmission
- Irrational use of artemisinin-based combination therapies and insecticides will expedite the spread of resistance and result in loss of these effective tools

d) Tuberculosis**Assumptions**

- Epidemiological factors that can affect the tuberculosis situation will remain unchanged or will not get worse; this includes the level of HIV epidemiology and also the level of multidrug-resistant tuberculosis
- The newly started Eastern Mediterranean Partnership to Stop TB, together with national partnerships to stop TB, will increase awareness on tuberculosis and generate resources from within the Region

Risks

- For a variety of reasons, particularly political, countries of the Region may face difficulties in receiving financial support for tuberculosis care from donors
- Countries of the Region may not engage in the Eastern Mediterranean Partnership to Stop TB, and may not provide resources for tuberculosis control

e) Operational research, tropical diseases**Assumptions**

- A close link is established and sustained between researchers in academic institutions and national control programmes
- Countries allocate more human and financial resources to health research
- Research capacity is strengthened and awareness about evidence-based public health is increased among health policy-makers

Risks

- Inadequate commitment to and understanding of health research in the Region

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
02.001	Guidelines, policy, strategy and other tools developed for prevention of, and treatment and care for patients with, HIV/AIDS, tuberculosis and malaria, including innovative approaches for increasing coverage of the interventions among poor people, and hard to-reach and vulnerable populations.	02.001. EM01	Regional and national policy, strategy and technical guidance documents and implementation tools and plans developed for prevention, treatment and care for HIV and countries supported in policy development and strategic planning	Number of countries with costed national HIV strategic plans for the health sector	1	5
		02.001. EM02	The regional strategy for malaria control and elimination by 2020 adopted by all countries, and national policies, strategies and guidelines developed/updated accordingly	Number of endemic countries with updated national strategy for malaria control and elimination for 2010–2020	5	9
				Number of endemic countries with developed/ adopted policies and strategies for private sector involvement/compliance with national guidelines	3	6
				Number of targeted countries that have developed a community-based strategy for management of malaria	3	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
				Number of countries that have developed a national integrated vector management strategy	9	11
		02.001. EM03	Stop TB Strategy fully adopted by all countries as national policy for tuberculosis control, and multi-year national strategic plans and national tuberculosis control guidelines updated and implemented accordingly	Number of countries that have adopted and implemented the Stop TB strategy	22	22
				Number of countries that have updated, costed multi-year national strategic plans	6	15
02.002	Policy and technical support provided to countries towards expanded gender sensitive delivery of prevention, treatment and care interventions for HIV/AIDS, tuberculosis and malaria, including integrated training and service delivery; wider service-provider networks; and strengthened laboratory capacities and better linkages with other health services, such as those for sexual and reproductive health, maternal, newborn and child health, sexually transmitted infections, nutrition, drug dependence treatment services, respiratory care, neglected diseases and environmental health.	02.002. EM01	Policy and technical support provided to countries to strengthen health system infrastructure, human resources and technical capacity to enable expanded delivery of prevention, treatment and care interventions for HIV and STI	Number of countries with at least 80% antiretroviral therapy coverage rate (% of people living with HIV/AIDS in need of ART who receive ART)	1	4
		02.002. EM02	Endemic countries supported to strengthen capacity of malaria control programme at all levels	Number of targeted countries with WHO field staff	5	7
				Number of countries with functional national malaria training and research centres	4	8
				Number of conducted regional training courses and workshops (planning and management, malaria laboratory diagnosis, case management, epidemic control, vector control and monitoring and evaluation) in 2010–2011	0	6
		02.002. EM03	Policy and technical support provided to countries so that all countries have established high-quality tuberculosis care based on the Stop tuberculosis strategy and continue to achieve the global targets and reduce tuberculosis burden in line with the Target 8 of Millennium Development Goals.	Number of countries that have achieved the target for tuberculosis case detection (70%)	10	16
				Number of countries that have achieved the target for tuberculosis treatment success (85%)	12	18
02.003	Global guidance and technical support provided on policies and programmes in order to promote equitable access to essential medicines, diagnostic tools and health technologies of assured quality for the prevention and treatment of HIV/AIDS, tuberculosis and malaria, and their rational use by prescribers and consumers, and, in order to ensure uninterrupted supplies of diagnostics, safe blood and blood products, injections and other essential health technologies and commodities.	02.003. EM01	Countries supported through capacity building and technical assistance to ensure uninterrupted supplies of HIV/STI medicines and diagnostics	Number of targeted countries (8) that report uninterrupted supply of HIV medicines and diagnostics during 2010–2011	0	5
			Endemic countries providing universal access to reliable malaria lab diagnosis and effective, quality assured and safe anti-malarial treatment and effective preventive measures against malaria for populations at risk	Number of endemic countries able to provide malaria laboratory diagnosis for at least 80% of total cases	3	9
				Number of endemic countries able to provide high quality WHO-recommended malaria treatment to more than 80% of malaria cases	3	9

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
				Number of endemic countries reporting at least 80% coverage of effective vector control methods among populations at risk	3	9
		02.003. EM02	Effective drug management systems established in all countries and regular access to high quality TB medicine ensured either through national resources or with help of the Global Drug Facility (GDF) and other international partners	Number of countries that have ensured regular access to high quality tuberculosis medicine	22	22
				Number of countries that have received Global Drug Facility support	15	20
				Number of countries receiving support from the Green Light Committee	15	22
02.004	Global, regional and national systems for surveillance, evaluation and monitoring strengthened and expanded to keep track of progress towards targets and allocation of resources for HIV/AIDS, tuberculosis and malaria control and to determine the impact of control efforts and the evolution of drug resistance.	02.004. EM01	Capacities of country programmes strengthened to carry out HIV/STI surveillance according to WHO/UNAIDS recommendations and to monitor coverage of essential prevention and treatment services	Number of countries with surveillance systems that are able to provide information on HIV prevalence in low and high risk populations	5	9
		02.004. EM02	Effective malaria surveillance/monitoring and evaluation system established in endemic countries to measure malaria morbidity and mortality, coverage of key interventions	Number of endemic countries with strong routine surveillance and information systems that are able to provide reliable data on malaria incidence among populations at risk	3	9
				Number of countries that have conducted surveys reporting on coverage of key malaria interventions or parasite prevalence	4	6
				Number of endemic countries that have conducted monitoring of anti-malaria drug efficacy in 2010–2011	0	6
				Number of endemic countries conducted monitoring of insecticide resistance in 2010–2011	2	9
		02.004. EM03	Effective monitoring and evaluation systems for tuberculosis control including measurement of its epidemiological impact, through revised WHO recording and reporting system and computerized surveillance tools	Number of countries with high quality surveillance with ENRS or other computerized system	18	22
				Number of countries with drug resistance surveys	15	22
				Number of countries with updated/revised information on the epidemiological situation of tuberculosis	12	22
02.005	Political commitment sustained and mobilization of resources ensured through advocacy and nurturing of partnerships on HIV/AIDS, tuberculosis and malaria at country, regional and global levels; support provided to countries as appropriate to develop or strengthen and implement mechanisms for resource mobilization and utilization and increase the absorption capacity of available resources; and engagement of communities and affected persons increased to	02.005. EM01	Countries supported to achieve political commitment, to access increased financial resources allocated to HIV and STI programmes and to involve the private sector, civil society organizations and affected people	Number of low and lower middle income countries that are supported in submitting new grant proposals to access international financial resources during 2010–2011	0	6
		02.005. EM02	Effective partnership established with all relevant agencies and	Number of countries with GFATM support	7	7

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
	maximize the reach and performance of HIV/AIDS, tuberculosis and malaria control programmes.		resources mobilized at regional and national level	Number of regional activities for promoting advocacy effective partnership and resource mobilization in 2010–2011	0	5
				Number of activities supported to promote cross-border coordination mechanisms among neighbouring countries in the Region and other regions in 2010–2011	0	4
		02.005. EM03	Functional national partnerships for tuberculosis control established in all countries that contribute to sustainable financing to tuberculosis control in the countries; Eastern Mediterranean Partnership functional in mobilizing additional financial and technical support for country efforts	Number of countries with effective national partnership for tuberculosis control	10	15
				Number of countries with GFATM support	11	12
02.006	New knowledge, intervention tools and strategies developed and validated to meet priority needs for the prevention and control of HIV/AIDS, tuberculosis and malaria, with scientists from developing countries increasingly taking the lead in this research.	02.006. EM01	Countries supported to build capacity and to carry out basic and operational research on HIV/STI	Number of new research projects supported	0	12
		02.006. EM02	Operational research on different aspects of malaria supported and results used in planning and management of malaria control programme	Number of malaria operational research supported in each biennium by the Small Grants Scheme and other mechanisms	12	20
		02.006. EM03	Operational research activities incorporated in tuberculosis control in all countries as a core function of national tuberculosis programmes	Number of operational research projects, addressing key tuberculosis programme challenges, supported by the Small Grants Scheme and other mechanisms	14	20
		02.006. EM04	By the end of 2011, new knowledge, new improved tools, interventions and public health strategies for HIV, tuberculosis and malaria will be generated and/or validated; research capacity strengthened, results disseminated and translated into policy and practice of the national control programmes, and partnership established	Number of implemented new/improved interventions (including tools, strategies) that are developed through SGS-supported research activities	2	12
				Number of manuscripts published in peer-reviewed journals based on SGS-supported research activities	120	150

Strategic Objective 3: To prevent and reduce disease, disability and premature death from chronic noncommunicable conditions, mental disorders, violence and injuries and visual impairment.

Regional issues and challenges

Chronic noncommunicable conditions, mental disorders, violence and injuries are currently the major causes of death and disability in almost all countries. In recent years the regional committees, the Health Assembly and the United Nations General Assembly have given WHO an important set of mandates for tackling these issues.

These causes of death and disability are responsible for 75% of all deaths globally – a figure that is projected to increase over the next 10 years. Over the period 2006–2015, deaths from communicable conditions, maternal and perinatal conditions and nutritional deficiencies are expected to decrease by 3%. At the same time, deaths from chronic noncommunicable conditions are expected to increase by 17%, deaths from neuropsychiatric disorders by 14% and those caused by injuries by 12%. The major part of this increasing burden will be borne by low-income and middle-income countries, where these three causes of death are already responsible for at least 80% of all deaths.

A full range of interventions for chronic noncommunicable conditions, mental disorders, violence and injuries have been shown to be cost effective and affordable in all regions. For example, an outlay of US\$ 7 per capita covers the cost of a basic mental health package at primary health care level; US\$ 1 spent on smoke alarms produces a health-cost savings of US\$ 21; combination drug therapy for individuals at high risk of a cardiovascular event is estimated to avert 63 million disability-adjusted life years every year worldwide; and cataract surgery generates increased economic productivity that is equivalent during the first year to 1500% of the cost of the intervention.

Strategic approaches

Tackling chronic noncommunicable conditions, mental disorder, and violence and injuries will need to be made a priority for health and for development at both national and international levels. A comprehensive public health approach that includes the fostering of multisectoral collaboration and innovation is essential. Member States should develop strengthened and coordinated responses to chronic noncommunicable diseases, mental disorders and promotion of mental health, and violence and injuries, based on evidence and integrated action. Giving higher priority to primary prevention, ensuring community participation, and reorienting health systems to provide effective health care for chronic conditions are critical to successful outcomes in countries.

Regional assumptions and risks and option analysis

The following assumptions underlie achievement of this strategic objective:

- A high level of multisectoral cooperation is sustained between global and national stakeholders, and it is recognized that multisectoral action is a prerequisite for success
- Countries give priority to integrated prevention and management of the conditions, disorders and injuries concerned
- It is recognized that countries need to give priority to primary health care over tertiary care when allocating resources.

The risks that could prevent achievement of the strategic objective are:

- Combating the growing threat to health and development posed by chronic noncommunicable conditions, mental and behavioural disorders, violence and injuries continues to be omitted from the high-level development schedule, as set out in the Millennium Development Goals
- National programmes for the prevention of noncommunicable diseases, mental disorders, visual impairment, violence and injuries are unable to receive the requisite resources to implement the key policies and interventions.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
03.001	Advocacy and support provided to increase political, financial and technical commitment in countries to address chronic noncommunicable conditions, mental and behavioural disorders, violence and injuries and disabilities	03.001. EM01	Advocating for putting prevention and control of noncommunicable diseases, injuries, mental and behavioural disorders (including disabilities) and visual and hearing impairment high on national political and developmental agendas	Number of countries with increased budget for the units for prevention and control of noncommunicable disease in the Ministry of Health	8	14
				Number of countries with increased budget for the units for injury prevention and control at the national level based at the Ministry of Health	8	14
				Number of countries with increased budget for the mental health units	8	14
				Number of countries with a functional unit for prevention and control of blindness and visual impairment at the national level	8	14
		03.001. EM02	Increase in resource allocation for chronic noncommunicable diseases (including visual and hearing impairment), mental and behavioural disorders injuries and management of disabilities at national and sub-national levels	Number of countries with increasing allocation for chronic noncommunicable diseases, mental and behavioural disorders injuries and management of disabilities as well as visual and hearing impairment compared to the previous year's allocation	12	18
		03.001. EM03	Stronger capacity at regional and country offices for providing technical support to Member States at all levels	Availability of full-time staff for each area of work included in this strategic objective at regional level	7	14
				Availability of full-time/part time staff for most of the areas of work included in this strategic objective in selected country offices	7	14
		03.001. EM04	Guidance and support provided to countries for strengthening health services with regard to the management and treatment of tobacco dependence	Number of cessation programmes evaluated	6	12
03.002	Guidance and support provided to countries for the development and implementation of policies, strategies and regulations to address chronic noncommunicable conditions, mental and neurological disorders, violence, injuries and disabilities together with visual impairment, including blindness.	03.002. EM01	Advocacy for putting prevention and control of noncommunicable diseases, injuries mental and behavioural disorders (including disabilities) and visual and hearing impairment high on national political and developmental agendas	Number of targeted countries that have joined the Eastern Mediterranean Approach to Noncommunicable diseases network	22	sustained
				Number of countries implementing national plans for prevention and control of noncommunicable disease	22	sustained
				Number of countries with national plans implemented for prevention and control of blindness and visual impairment	22	sustained
		03.002. EM02	Technically support provided to countries for developing and implementing a national level multisectoral plan of action for injury and violence prevention and disability	Number of targeted countries implementing national policies and plans on injury and violence prevention and rehabilitation	13	18

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
		03.002. EM03	Technical support given to countries in formulating and implementing policies and plans based on community-based mental health interventions on mental health and substance abuse services	Number of countries receiving and utilizing guidance on policies, strategies and regulations on mental, behavioural, neurological and psychoactive substance use disorders	19	22
				Proportion of targeted countries that have developed comprehensive national plans for prevention of visual and hearing impairment and are implementing them	19	22
03.003	Improvements made in Member States' capacity to collect, analyse, disseminate and use data on the magnitude, causes and consequences of chronic noncommunicable conditions, mental and neurological disorders, violence, injuries and disabilities together with visual impairment, including blindness.	03.003. EM01	Support provided to countries for integrating surveillance of noncommunicable diseases (including blindness) in the primary health care information system	Number of countries with established surveillance system for noncommunicable diseases and blindness in country health information system	16	22
		03.003. EM02	Technical support provided to countries for integrating injury surveillance in existing health information system and developing a database on the magnitude of violence, unintentional injuries	Number of countries having a national injury surveillance system and the results are used in policy-making	11	18
				Number of countries publishing yearly injury reports	11	18
		03.003. EM03	Support provided to countries to incorporate the assessment of psychiatric patients and vulnerable groups in the health information system	Number of countries that have psychiatric epidemiological information about specific population groups	13	18
03.004	Improved evidence compiled by WHO on the cost-effectiveness of interventions to tackle chronic noncommunicable conditions, mental and neurological and substance use disorders, violence, injuries and disabilities together with visual impairment, including blindness	03.004. EM01	Countries supported to produce and develop cost-effectiveness of interventions addressing chronic noncommunicable conditions, mental and behavioral disorders, violence and injuries and disabilities	Evidence on the (cost)-effectiveness of a core package of interventions for chronic, noncommunicable conditions is summarized and a price tag is estimated for its implementation	9	12
				Evidence on (cost)-effectiveness of widely available interventions for management of depression, schizophrenia, epilepsy and substance use disorders developed and made available	9	12
		03.004. EM02	Countries supported to integrate prevention and control of noncommunicable diseases and injuries in the existing community based initiatives	Number of community based initiatives having components of noncommunicable diseases/injuries prevention and control	10	15
				Number of community based rehabilitation programmes available at the national and subnational levels	10	15
		03.004. EM03	Countries supported to scale up the prevention of blindness programmes according to the regional guidelines	Number of countries having national level prevention of blindness programmes	20	22

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
03.005	Guidance and support provided to Member States for the preparation and implementation of multisectoral, population-wide programmes to promote mental health and to prevent mental and behavioural disorders, violence and injuries, together with hearing and visual impairment, including blindness.	03.005. EM01	Capacity strengthened in countries for developing and implementing multisectoral population-based programmes for violence and injuries prevention	Guidelines for multisectoral interventions to prevent violence and unintentional injuries published and widely disseminated	6	10
		03.005. EM02	Capacity strengthened in countries for developing and implementing multisectoral population based programmes for prevention of hearing and visual impairment	Proportion of targeted countries implementing WHO-recommended strategies for population-wide prevention of hearing and visual impairment	8	14
		03.005. EM03	Technical support given to countries in formulating and implementing policies and plans based on community-based mental health interventions on mental health and substance abuse services	Countries having guidelines on prevention and management of depression, schizophrenia, epilepsy and substance use disorders developed and made available	8	12
03.006	Guidance and support provided to Member States to improve the ability of their health and social systems to prevent and manage chronic noncommunicable conditions, mental and behavioural disorders, violence, injuries and disabilities together with visual impairment, including blindness.	03.006. EM01	Guidelines on prevention and management of noncommunicable disease (including blindness) and care available at all levels.	Number of guidelines produced and disseminated for the prevention and control of most prevalent noncommunicable diseases	14	20
		03.006. EM02	Technical support provided to countries for strengthening pre-hospital and hospital trauma care system using WHO guidelines and protocols	Number of countries adapting and implementing WHO guidelines on pre-hospital and essential trauma care guidelines	8	14
		03.006. EM03	Strengthened health and social systems that respond better to the disabled people and people with mental health needs	Countries having national level disability (mental and physical) management and rehabilitation programmes	9	14
				Number of countries that strengthened rehabilitation services using the recommendations of the <i>World report on disability and rehabilitation</i>	9	14

Strategic Objective 4: Reduce morbidity and mortality and improve health during key stages of life, including pregnancy, childbirth, the neonatal period, childhood and adolescence, and improve sexual and reproductive health and promote active and healthy ageing for all individuals

Regional issues and challenges

This strategic objective is aimed at strengthening the core service components of primary health care and address an enormous burden of disease, intensifying action towards reaching key health-related MDGs (Goals 4 and 5) and other international commitments, such as universal access to reproductive health care. In some countries of the Region, the situation is worsening due to unfavourable conditions such as increased incidence of sexually transmitted infections among adolescents, the proportionate increase in neonatal mortality and the ever increasing population of elderly people with special health and social needs. Ongoing challenges such as neonatal mortality and newly emerging ones such as the increase in the older population, along with inadequate human and financial resources in several countries of the Region, require applying an integrated approach aims at scaling up towards universal access to effective interventions, paying attention to gaps in health and gender equity, and ensuring integration, synergy and coordination between programme areas, service delivery levels, civil society and private sector.

Political will to make a difference in these areas is flagging and resources are insufficient. Unfortunately, those who are most affected, poor women and children, have the least influence on decision-makers and are often missed by the health services they need. Some issues are politically and culturally sensitive and do not draw the attention that they should, given the burden they place on public health. Efforts to improve the quality of health care and to increase coverage are insufficient. Competing health priorities, vertical programme approaches and lack of coordination between the concerned health authorities and development partners result in lack of alignment of plans of action, programme fragmentation, missed opportunities and in inefficient use of the limited resources that are currently available. Lack of attention to gender inequality and gaps in health equity undermine ongoing efforts to decrease mortality and morbidity globally. This pattern can be changed through concerted action of all involved. As older people are becoming a larger and more visible proportion of the general population, better statistical information on demographic ageing and its causes, consequences and specific regional aspects is urgently needed to guide policies and programmes. Demographic changes will lead to changing demands on health systems in the countries of the Region. Health care systems will be expected to include care for older adults along with care for other groups. The main challenge in approaching population ageing issues is how to make ageing a positive experience. Longer life must be accompanied by continuing opportunities for health, participation and security, where older people can enjoy better quality of life.

Recognizing the need to accelerate reduction of morbidity and mortality and improve health during key stages of life, WHO has re-emphasized the continuum of care approach that covers the support that individuals, families and communities are able to provide to all levels of the health care system, including the appropriate actions of skilled health workers both in the community and in health facilities, and good quality services at both the primary health care level and in hospitals. Critically, the continuum of care approach requires a functioning referral system, with the necessary linkages between the different levels of health care, to ensure that complications, especially life-threatening emergencies, are well and quickly managed.

Technical knowledge and programme experience indicate that effective interventions exist for most of the health problems covered by this strategic objective, and that basic interventions are feasible and affordable even in resource-constrained settings. This implies the need for strengthening research, studies, surveillance and other data collection activities to enable evidence-based planning and implementation in accordance with the real health needs of the public. There is general agreement that what is required is action towards reaching universal access to and coverage with key interventions (WHA58.31). To this end, adopting a life-course approach which recognizes the influence of early life events and of inter-generational factors on future health outcomes will serve to bridge gaps and build

synergy between programme areas, and will also provide an effective support to ensure active and healthy ageing (WHA58.16).

Strategic approaches

This strategic objective will require a country-led planning and implementation process for scaling up towards universal access to and coverage with maternal, newborn, child, adolescent, sexual and reproductive health care, while addressing gender inequality and growing health inequities that fuel the high levels of mortality and morbidity.

Integration, synergy, and harmonization must be achieved at the programme and service delivery levels, and a continuum of care approach must be ensured that runs through the life course, and spans the home, the community and different levels of the health system. This needs to occur within the broader framework of health systems strengthening to ensure adequate and equitable financing and delivery of quality health-supportive services, with marginalized and underserved groups receiving priority attention. Of particular relevance to this strategic objective is the need to address the crisis in human resources for health.

Achievement of this strategic objective will also require the promotion of community-based interventions to increase demand for services and to support appropriate care in the home across the life course. The different roles and needs of women and men should be given due attention in order to achieve optimum health outcomes. The sexual and reproductive health of women and men outside of the reproductive process and beyond the reproductive age will also receive attention.

In addition, Member States will need to develop, implement and evaluate policies and programmes that promote healthy and active ageing and the highest attainable standard of health and well-being for their older citizens. Member States and partners must commit resources and prioritize national action through intensified advocacy and mobilization of all partners around one concrete plan at country level. In supporting the efforts of the Member States, the WHO Secretariat will focus on:

- providing technical guidance for the formulation and implementation of effective, evidence-based policies and interventions, aiming for universal access to care, with due attention to gender inequality and gaps in health equity;
- supporting countries to build their capacity for service delivery, with particular attention to the strengthening of human resources for health, and the provision and rational use of essential medicines, safe blood, health technologies and commodities;
- aligning the technical content of programmes and developing synergies between programme areas (including nutrition, HIV, tuberculosis and malaria), addressing the specific needs of female and male children, adolescents, adults and older individuals, while ensuring continuum of care from the home to the first-level health facility and referral facilities, and through the life stages;
- supporting the necessary research and development of technologies and interventions, and providing the necessary evidence on determinants and causes of, and on the effectiveness of programmes;
- supporting countries to monitor their health situation, by age and sex, and assess progress towards internationally agreed goals and targets relevant to this objective, and to monitor and evaluate programmes to ensure optimal coverage with effective services;
- working through partnerships to mobilize political leadership and resources for improving sexual and reproductive, maternal, newborn, child and adolescent health of both sexes and working towards healthy ageing.

Regional assumptions and risks and option analysis

This strategic objective is formulated under the following assumptions.

- Overall strengthening of health systems will occur, including the development and maintenance of a suitable infrastructure, a reliable supply of essential drugs and commodities, functional referral systems, and a competent and well-motivated health workforce.
- International and national actions for dealing with the crisis affecting human resources for health will be undertaken.
- Key processes such as improved harmonization of the work of UN agencies at country level and integration of health in national planning and implementation instruments, such as poverty reduction strategy papers and medium-term expenditure frameworks will be pursued.
- Potential for raising new resources for WHO's work in these areas will be realized. There is considerable political interest in making progress towards the MDGs, this will likely grow between now and 2015, along with support from global partnerships and initiatives, including the Partnership on Maternal, Newborn and Child Health.

The following risks have been identified that may adversely affect the achievement of the strategic objective: threats posed by the continued spread of the AIDS pandemic and setbacks in malaria control; and, in some countries, natural crises, political instability and food insecurity may lead to reversals in some indicators.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
04.001	Support provided to Member States to formulate a comprehensive policy, plan and strategy for scaling up towards universal access to effective interventions in collaboration with other programmes, paying attention to reducing gender inequality and health inequities, providing a continuum of care throughout the life course, integrating service delivery across different levels of the health system and strengthening coordination with civil society and the private sector.	04.001. EM01	Technical support to Member States to develop comprehensive policies, strategies and plans for scaling up towards universal access to effective interventions, paying attention to gaps in health and gender equity, and ensuring synergy and coordination between programme areas, service delivery levels, civil society and the private sector	Number of countries that have a strategy on making pregnancy safer	0	5
				Number of countries that have a strategy on integrated management of child health	0	5
				Number of countries that have a strategy on adolescent health and development	0	5
				Number of countries that have a policy on universal access to sexual and reproductive health	0	5
				Number of countries that have a policy on the promotion of active and healthy ageing	0	5
04.002	National research capacity strengthened as necessary and new evidence, products, technologies, interventions and delivery approaches of global and/or national relevance available to improve maternal, newborn, child and adolescent health, to promote active and healthy ageing, and to improve sexual and reproductive health.	04.002. EM01	National research capacity strengthened as necessary and new evidence, products, technologies, interventions and delivery approaches of global and/or national relevance available to improve maternal, newborn, child and adolescent health, to promote active and healthy ageing, and to improve sexual and reproductive health	Number of new research centers strengthened through comprehensive institutional development and support	2	5
				Number of completed studies on priority issues in relevant field of health	5	10

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
				Number of new or updated systematic reviews on best practices, policies and standards of care	0	5
04.003	Guidelines, approaches and tools for improving maternal care applied at the country level, including technical support provided to Member States for intensified action to ensure skilled care for every pregnant woman and every newborn, through childbirth and the postpartum and postnatal periods, particularly for poor and disadvantaged populations, and the monitoring of progress.	04.003. EM01	Technical support provided to countries to build national capacities for developing responsive policies and strategies and implementing and monitoring programmes for improving maternal health and achieving health related MDGs by ensuring skilled care for through pregnancy, childbirth and the postpartum period, particularly for poor and disadvantaged populations	Number of countries implementing strategies to ensure skilled care for every birth	5	10
				Number of countries adapting and utilizing the Integrated Management of Pregnancy and Childbirth (IMPAC) policy, technical and managerial norms and guidelines	5	10
				Number of countries that have adapted strategic directions for accelerating reduction of maternal mortality	5	10
04.004	Guidelines, approaches and tools for improving neonatal survival and health applied at country level, with technical support provided to Member States for intensified action towards universal coverage, effective interventions and monitoring of progress.	04.004. EM01	Technical support provided to countries to build national capacities for developing gender responsive policies and strategies and implementing and monitoring programmes for improving neonatal health and achieving health related MDGs by ensuring skilled care through pregnancy, childbirth and the neonatal period, particularly for poor and disadvantaged populations	Number of countries that have adapted and implemented the packages of IMPAC interventions to include the full newborn period	5	10
				Number of countries that have adapted and implemented the packages of IMCI interventions to include the full newborn period	5	10
04.005	Guidelines, approaches and tools for improving child health and development applied at the country level, with technical support provided to Member States for intensified action towards universal coverage of the population with effective interventions and for monitoring progress, taking into consideration international and human-rights norms and standards, notably those stipulated in the Convention on the Rights of the Child.	04.005. EM01	Technical support provided to countries to implement, monitor and evaluate policies, strategies and plans, and cost effective interventions to improve child health and development for achieving health-related MDGs	Number of countries that have adapted and implemented the full package of IMCI interventions	8	12
				Number of countries implementing strategies to increase coverage with child health and development interventions	8	12
04.006	Technical support provided to Member States for the implementation of evidence-based policies and strategies on adolescent health and development, and for the scaling up of a package of prevention, treatment and care interventions in accordance with established standards.	04.006. EM01	Technical support provided to countries to develop policies and strategies and plans, and implement, monitor and evaluate cost effective interventions to improve adolescent health and development in formal and informal settings (in and out schools) for achieving health-related MDGs	Number of countries with a functioning adolescent health and development programme	5	10
				Number of countries that have adapted strategic directions for promoting adolescent health and development	5	10
				Number of countries with functioning national networks of health promoting schools	6	15

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
04.007	Guidelines, approaches and tools made available, with provision of technical support to Member States for accelerated action towards implementing the strategy to accelerate progress towards the attainment of international development goals and targets related to reproductive health, with particular emphasis on ensuring equitable access to good-quality sexual and reproductive health services, particularly in areas of unmet need, and with respect for human rights as they relate to sexual and reproductive health.	04.007. EM01	Technical support provided to countries to build national capacities for developing gender responsive policies and strategies and implementing and monitoring programmes for improving sexual and reproductive health and achieving health related MDGs	Number of countries implementing the Global Reproductive Health Strategy	0	5
				Number of countries having reviewed their existing national laws, regulations and policies relating to sexual and reproductive health	0	5
04.008	Guidelines, approaches, tools, and technical assistance provided to Member States for increased advocacy for consideration of ageing as a public health issue, for the development and implementation of policies and programmes aiming at maintaining maximum functional capacity throughout the life course and for the training of health care providers in approaches that ensure healthy ageing.	04.008. EM01	Technical support provided to countries to build capacities for developing policies and strategies; implementing and monitoring plans and programmes for active, healthy ageing	Number of countries which will have implemented community-based policies focused on strengthening primary health care capacity to deal with ageing issues	4	8
				Number of countries which will have implemented multisectoral policies, strategies and plans of action reflecting WHO revised Regional Strategy for Active, Healthy Ageing and Old Age Care for 2006–2015	4	10

Strategic Objective 5: Reduce the health consequences of emergencies, disasters, crises and conflicts, and minimize their social and economic impact

Regional issues and challenges

This strategic objective is aimed at contributing to human security and quality of life by minimizing the health impact of all forms of humanitarian crises, natural or man-made, and by addressing the health needs of affected populations. Each year, one in five WHO Member States experiences a crisis that endangers the health of its people. In 2006–2007 alone, over 166 countries globally were affected by natural and man-made disasters resulting in over 68 890 deaths and more than US\$ 110 billion in related costs.

Similarly in the Eastern Mediterranean Region, war, internal conflict and natural disasters continue to increase the vulnerability of populations. In the past two decades, 15 of the 22 countries (85% of the population) have been involved in conflict situations. In addition to the clear and localized threats and vulnerabilities that impact the poorest populations, newly emerging threats include possible human pandemic influenza, sky rocketing food and oil prices, and climate change. If left unaddressed, these emerging global threats combined with existing vulnerabilities, will result in potentially catastrophic consequences for marginalized populations.

Emergencies place sudden and intense demands on health systems, exposing existing weaknesses in these systems and disrupting economic activity and development. In the Eastern Mediterranean Region, where many countries are undergoing health transition and have relatively weak health infrastructure, disasters have seriously challenged routine health services, with disruption continuing for months on end and have effaced years of developmental gains. Experience has repeatedly shown that recovering from the disastrous effects of major and complex emergencies and crises takes much longer than perceived by the international community, and that the impact of such crises on health services and on the health status of populations persists for years. These factors further compound early recovery and reconstruction efforts in the case of most countries. Thus, effective response to humanitarian crises in the Region will require collective action by the international community, strong national public health preparedness capacity and efficient regional and global coordination.

The call for and investment in national and local capacities for preparedness and risk reduction in health has been echoed by Member States and by civil society. WHO's aim is to strengthen the health system to be able to deliver services effectively when most needed: in times of disaster. The 2008–2009 world disaster reduction campaign under the theme “hospitals safe from disasters” provides an opportunity to bolster regional and national awareness and commitment for such measures in the health sector. Campaign activities target the health facility, its human workforce and functional resilience towards natural hazards and man-made threats.

Strategic approaches

As part of global humanitarian reform, WHO has been tasked with coordination of the health cluster in addition to sustaining programmes aimed at disaster preparedness and risk reduction, response and early recovery. In terms of response, improvement is needed in many areas: mass casualty management; water, sanitation and hygiene; nutrition; communicable and noncommunicable diseases; maternal and newborn health; mental health; pharmaceuticals; health technologies; health logistics; health information services; and management and recovery of health infrastructure.

Funding of health-related aspects of emergency preparedness and response is also a major concern. In this regard it is critical to ensure that needs analysis and project formulation are well connected to larger processes both within the United Nations system and within WHO. This requires the development of partnerships and coordination that can bring a greater flow of predictable funding, especially for chronic and complex emergencies.

A number of strategic approaches will be taken to support the efforts of countries of the Region.

- Actively supporting Member States to build their capacity in the field of emergency preparedness and response through multisectoral, multidisciplinary and all-hazard approaches

- Capitalizing on the momentum created by the campaign for hospitals safe from disasters as an entry point for more sustainable development for risk reduction and preparedness in health
- Establishing and maintaining national and international operational capacities for rapid response and for leading coordinated action of multiple stakeholders in public health emergencies, disasters, conflict and other crises
- Development of knowledge bases and competencies for preparing for and responding to emergencies based on an all-hazard public health approach
- Developing partnerships and coordination mechanisms with governments, civil society and networks of collaborating centres and other centres of excellence to ensure timely and effective interventions when needed
- Developing technical and operational capacity in support of countries in crises, particularly in conducting health assessments, coordinating health action, filling in gaps, providing guidance and monitoring the performance of humanitarian action on health and nutrition of affected populations
- Leveraging the vast array of skills across WHO in support of response to emergencies (i.e. mental health, nutrition, water and sanitation, food safety, medicines, violence and injury prevention, mass casualty management, communicable disease control, maternal and child health)
- Bridging the gap between relief and development through early recovery and transitional processes whereby health is positioned as a key sectoral and national priority.

Regional assumptions and risks and option analysis

Assumptions

- Greater investment is made and called for by countries to address risk reduction and preparedness of the health sector to better respond to and recover from emergencies
- The World Health Assembly and Regional Committee for the Eastern Mediterranean continue to assert and endorse emergency preparedness and humanitarian action as an integral part of WHO/EMRO's mandated work
- Resources are made available by the donor community to address the gaps within humanitarian assistance and risk reduction programmes

Risks

- The misconception that work in emergency preparedness and response is an additional responsibility on top of the regular normative and developmental work of the Organization
- Insufficient development of mechanisms, readiness and competencies across WHO for effective and expeditious work in emergency situations
- Insufficient funding of the core functions necessary to conduct work in emergency preparedness and response and for WHO to fulfil the mandate of leader of the Interagency Standing Committee health cluster

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
05.001	Norms and standards developed, capacity built and technical support provided to Member States for the development and strengthening of national emergency preparedness plans and programmes.	05.001. EM01	Support provided to countries to reduce the risks of death, disease and disability resulting from emergencies, and to protect health sector development achievements through improved emergency preparedness and risk management at country and community levels.	Percentage of High Risk Member States (HRMS) having disaster management policies and plans	20%	50%
				Percentage of High Risk Member States (HRMS) having functional EPR programmes at the national level in place, in order to reduce risk and build capacity	20%	60%
05.002	Norms and standards developed, capacity built and technical support provided to Member States for a timely response to disasters associated with natural hazards and to conflict related crises.	05.002. EM01	Support provided to countries in implementing appropriate measures to protect health, by effectively responding emergencies (including man-made or natural disasters), through interventions based on best health practices in all public health priority areas ¹	Percentage of total accessible, affected population reached/served for health care provision, in an emergency response	50%	100%
				Percentage of affected MS having an operational and adequately staffed Health Cluster /s within 24-72 hours of onset of a new major crisis	50%	100%
05.003	Norms and standards developed, capacity built and technical support provided to Member States for assessing needs and for planning and implementing interventions during the transition and recovery phases of conflicts and disasters.	05.003. EM01	Support provided to countries and assisted in sector-wide early recovery and rehabilitation plans/programmes, that provide effective and efficient transition from relief to recovery	Percentage crisis affected MS where comprehensive health assessments have been completed	100%	100%
				Percentage of crisis-affected countries having comprehensive health sector recovery strategies and plans developed	100%	100%
05.004	Coordinated technical support provided to Member States for communicable disease control in natural disaster and conflict situations.					
05.005	Support provided to Member States for strengthening national preparedness and for establishing alert and response mechanisms for food-safety and environmental health emergencies.					
05.006	Effective communications issued, partnerships formed and coordination developed with other organizations in the United Nations system, governments, local and international nongovernmental organizations, academic institutions and professional associations at the country, regional and global levels.	05.006. EM01	Organizational support to countries improved by having effective communications issued, partnerships formed and coordination developed with other organizations in the United Nations system, governments, local and international nongovernmental organizations, academic institutions and professional associations	Number of partnerships and forums developed at local, national and regional level to address health and disaster management	5	10
				Percentage of emergencies with WHO press releases, communication reports and advocacy material issued	100%	100%

¹ Priority areas, as agreed to and defined by IASC Health Cluster

Strategic Objective 6: Promote health and development, and prevent or reduce risk factors for health conditions associated with use of tobacco, alcohol, drugs and other psychoactive substances, unhealthy diets, physical inactivity and unsafe sex

Regional issues and challenges

The six major risk factors that this strategic objective addresses are responsible for more than 60% of mortality and at least 50% of morbidity worldwide. They have important gender dimensions and particularly affect poor populations in low-income and middle-income countries. Although emphasis has been placed on treating the adverse effects of these risk factors, much less attention has been devoted to prevention and gender-responsive ways of dealing effectively with these health determinants and of reaching vulnerable and socially disadvantaged groups.

Tobacco use is a risk factor for six out of eight of the leading causes of death globally. It is the leading cause of preventable deaths worldwide, with at least 70% of tobacco-attributable deaths occurring in developing countries. Tobacco use and poverty are closely linked and prevalence rates are higher among the poor. Although measures that are both successful and cost effective are available for reducing tobacco use, only 5% of the world's population is completely covered by any one of the core demand-reducing policies. The WHO Framework Convention on Tobacco Control is an evidence-based treaty designed to help to reduce the burden of disease and death caused by tobacco use. The MPOWER package of tobacco control policies reflect and build on the WHO Framework Convention, and aims to turn this global consensus into global reality.

Every year, alcohol consumption is linked to 1.8 million deaths globally and 58.3 million years of life lost. In developing countries with low overall mortality, alcohol use is the leading risk factor, accounting for 6.2% of the total burden of disease. In a growing number of countries, injecting drug use is the driving force behind the rapid spread of HIV infection. Despite evidence of the substantial burden on health and society arising from alcohol and other psychoactive substance use, there are limited resources at WHO and in countries for preventing and treating substance use disorders, even though US\$ 1 invested in treatment produces at least US\$ 7 of savings in health and social costs.

The WHO Global Strategy on Diet, Physical Activity and Health, endorsed by Member States in 2004, provides all stakeholders with recommendations and policy options to improve risk factors related to unhealthy diets and physical inactivity. Globally, 17% of the population is estimated to be physically inactive and an additional 41% to be insufficiently active to benefit their health. It has been estimated that the resultant annual death toll is 1.9 million. Low fruit and vegetable consumption as part of diet is responsible for at least 2.7 million deaths each year; overweight and obesity are responsible for 2.6 million deaths annually¹.

Unsafe sexual behaviour significantly increases the burden of disease through unintended pregnancy, sexually transmitted infections (including HIV), and other social, emotional and physical consequences that have been seriously underestimated. WHO estimates that unsafe sex is the second most important global risk factor to health in countries with high mortality rates. Each year, 80 million women have an unwanted pregnancy, 46 million opt for termination, and 340 million new cases of sexually transmitted infections and 5 million new HIV infections are reported. Risky behaviour does not often occur in isolation but rather in clusters. For example, hazardous use of alcohol and other substances and unsafe sex frequently go together. It is important to understand the underlying social and environmental determinants of risky behaviour and to recognize factors that create vulnerability to individual risks such as social and cultural determinants, including gender, low education, poverty, and other inequities. For that reason, WHO recognizes the need for a comprehensive, integrated approach to health promotion together with effective preventive and protective strategies that build resilience of individuals and strengthen community capacity for health.

In addition, global estimates show that 3 billion people, or half of humanity, now live in urban areas. As a determinant for the major risk factors, urbanization has both positive effects and negative

¹ *The world health report 2002. Reducing risks, promoting healthy life.* Geneva, World Health Organization, 2002.

implications for health. With more and more people living in towns and cities, where the impact of social, economic, environmental and technological changes is greatest, new public health issues and problems are emerging.

The global burden of death, disease and disability due to conditions associated with the major risk factors is substantial. Nevertheless, there is a continuing lack of awareness of the differential impacts of this burden on women and men, girls and boys, together with an absence of political commitment to vigorously promoting health, and preventing or reducing the occurrence of risk factors. In order to reduce the burden, significant additional investment in financial and human resources is urgently needed at all levels of the Secretariat and in Member States in order to build capacity and strengthen interventions at national and global levels.

Strategic approaches

Taking a gender-responsive, integrated approach to health promotion and preventing or reducing major risk factors will enhance synergies, improve the overall efficiency of interventions and broaden the scope and effectiveness of existing vertical approaches.

In countries, it is essential to strengthen institutions and build national capacities for surveillance (including appropriate disaggregation by sex and age and where possible by socioeconomic group) and prevention or reduction with respect to the common risk factors and the health conditions with which they are associated. Furthermore, strong leadership and stewardship by health ministries are necessary to ensure that all sectors of society participate effectively. Action at the multisectoral level is vital because the main determinants of the major risk factors lie outside the health sector. The process of urbanization (in all its aspects: physical, social and economic) needs also to be supported to ensure that it produces positive health outcomes. The urbanization-related determinants need to be effectively addressed in the strategies for risk factor reduction.

In the area of health promotion, significant efforts are required to strengthen leadership and build capacity to take account of increased needs and activities across all relevant health programmes. As well, concerted efforts are needed to implement the recommendations made at the 6th Global Conference on Health Promotion (Bangkok, 7–11 August 2005), to address the determinants of health in the global development agenda, across the whole of government, in communities and civil society as well as making health considerations a requirement for good corporate practice.

In order to ensure lasting success there is a need for comprehensive approaches that use a combination of strategies to resolve policy issues and build capacities at individual, family and community levels.

Assumptions and risks and option analysis

The following assumptions underlie achievement of this strategic objective.

- There is additional investment in financial and human resources to build capacity for health promotion and for preventing risk factors.
- There is recognition in countries of the importance of action at national and local levels as well as the importance of synergistic action by national and local governments to promote health in all policies.
- Effective partnerships and multisectoral and multidisciplinary collaboration are established in relation to establishing equitable policies, mechanisms, networks and actions and with the involvement of all stakeholders at municipal, national, regional and international levels.
- There is commitment to comprehensive and integrated policies, plans and programmes addressing common risk factors, together with recognition that equitable, integrated approaches to preventing major risk factors result in a wide range of health benefits.
- Investment in research, especially to find effective population-based prevention strategies for vulnerable populations, is increased.

The risks that could prevent achievement of the strategic objective are:

- Working or interacting with industry will expose efforts to the competing interests of the private sector, including the tobacco, alcohol, sugar, food and beverage industries. Guidelines for appropriate conduct must be followed in all cases and the primacy of public health safeguarded.
- Health promotion and prevention efforts with regard to the risk factors may be adversely affected by the low priority afforded to this area and the scarcity of resources allocated to it as a result by the Secretariat and countries. Continued advocacy for increased investment is essential in order to minimize this risk.
- Integrated approaches to prevention or reduction of risk factors may compromise the capacity of both the Secretariat and countries to provide expertise in relation to specific diseases and risk factors. In order to avoid that outcome, adequate resources must be maintained for integrated approaches, as well as a critical mass of expertise in major areas.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
06.001	Advice and support provided to Member States to build their capacity for health promotion across all relevant programmes, and to establish effective multisectoral and multidisciplinary collaborations for promoting health and preventing or reducing major risk factors.	06.001. EM01	Capacity strengthened in most countries in planning and implementation of multisectoral health promotion policies/programmes for promoting health and preventing occurrence of major risk factors	Regional training course on health promotion developed and implemented	6	18
		06.001. EM02	Multisectoral networks and partnership promoted and established	Regional mechanism with International Union for Health Promotion and Education established	Mechanism not yet established	Mechanism in place
				Number of multisectoral networks and partnerships at the subnational, national and regional levels that are fully functional	3	12
		06.001. EM03	Health promotion policies and multisectoral strategies developed by Member States, drawing upon the regional health promotion strategy	Number of national plans subjected to mid-term review	8	18
				Number of health promotion foundations networked to the global network	2	4
06.002	Guidance and support provided in order to strengthen national systems for surveillance of major risk factors through development and validation of frameworks, tools and operating procedures and their dissemination to Member States where a high or increasing burden of death and disability is attributable to these risk factors.	06.002. EM01	Risk factor surveillance mechanisms established for decision making	Number of countries supported for developing a database on major health risk factors	13	18
				Number of Global School-Based Student Health Surveys (GSHS) evaluated	12	18
		06.002. EM02	Support provided for community-based initiatives addressing issues related to social and environmental determinants of health including reorienting health systems	Number of community-based initiatives at the country level documenting health promotion actions	3	8
				Number of countries linking health promotion work with the work of social determinants of health	2	8
06.003	Evidence-based and ethical policies, strategies,	06.003. EM01	National level policies and plans for tobacco	Number of countries having ratified the FCTC	15	18

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
	recommendations, standards and guidelines developed, and technical support provided to Member States with a high or increasing burden of disease and death associated with tobacco use, enabling them to strengthen institutions in order to tackle or prevent the public health problems concerned; support also provided to the Conference of the Parties to the WHO Framework Convention on Tobacco Control for implementation of the provisions of the Convention and development and implementation of protocols and guidelines.		control in place as envisaged in the FCTC including activities to increase public knowledge and to monitor/regulate the tobacco industry activities	Number of countries having national level policies and plans taking into account the required legislation as envisaged in the FCTC	5	10
		06.003 EM02	Technical guidance and support provided to countries for completion of their tobacco control related database through the implementation of global tobacco-surveillance system/global surveys	Number of guidelines agreed to by the Conference of the Parties, including number of protocols adopted by the Conference of the Parties	18	22
		06.003. EM03	Technical support provided to countries to ratify and implement the FCTC and at the same time to participate effectively in any FCTC related activities	Number of countries that have conducted the national tobacco survey and produced a national database	16	20
06.004	Evidence-based and ethical policies, strategies, recommendations, standards and guidelines developed, and technical support provided to Member States with a high or increasing burden of disease or death associated with alcohol, drugs and other psychoactive substance use, enabling them to strengthen institutions in order to combat or prevent the public health problems concerned.	06.004. EM01	Evidence-based and ethical policies, strategies, recommendations, standards and guidelines developed on prevention and addressing the public health problems associated with drugs, alcohol and other psychoactive substances	Number of evidence-based and ethical policies, strategies, recommendations, standards and guidelines developed on prevention and addressing the public health problems associated with drugs, alcohol and other psychoactive substances	5	10
		06.004. EM02	Technical support provided to countries for the development of evidence-based and ethical policies, strategies, recommendations, standards and guidelines on prevention and addressing the public health problems associated with drugs, alcohol and other psychoactive substance	Number of countries that received technical support on evidence based and ethical policies ,strategies, recommendations, standards and guidelines developed on prevention and addressing the public health problems associated with drugs, alcohol and other psychoactive substance	19	22
		06.004. EM03	Support provided to most countries to strengthen institutions to address/prevent public health problems associated with drugs, alcohol and other psychoactive substance	Number of countries that received support to strengthen their institutions to address/prevent public health problems associated with drugs, alcohol and other psychoactive substance	19	22
06.005	Evidence-based and ethical policies, strategies, recommendations, standards and guidelines developed and technical support provided to Member States with a high or	06.005. EM01	Most countries supported to adapt and implement Global Strategy on Diet, Physical Activity and Health	Number of countries implementing Global Strategy on Diet, Physical Activity and Health based on regional adaptation	13	18

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
	increasing burden of disease or death associated with unhealthy diets and physical inactivity, enabling them to strengthen institutions in order to combat or prevent the public health problems concerned.	06.005. EM02	Technical support provided to develop policies, strategies, standards and guidelines to promote healthy nutrition and physical activity in different settings, including strengthening physical activity and sports in school curricula and extra-curricular activities	Number of countries with reduction in the attributable disease fraction due to unhealthy diet and physical inactivity	15	18
06.006	Evidence-based and ethical policies, strategies, interventions, recommendations, standards and guidelines developed and technical support provided to Member States to promote safer sex and strengthen institutions in order to tackle and manage the social and individual consequences of unsafe sex.	06.006. EM01	Development and implementation of policies, strategies and plans, and cost effective interventions to promote responsible sexual behaviour based on the full respect of ethical and religious values and social norms	Number of adolescent health programmes that address the issue of irresponsible sexual behaviour evaluated	0	5
		06.006. EM02	Adequate gender sensitive and culturally appropriate educational and social communication programmes and counselling services developed to promote responsible sexual behaviour among individuals, particularly adolescents and young people to prevent risk-taking behaviour	Number of education programmes that have been evaluated and have integrated responsible sexual behaviour and related life-skills education to resist peer pressure into formal and non-formal education programmes	0	6

Strategic Objective 7: Address the underlying social and economic determinants of health through policies and programmes that strengthen health equity and integrate pro-poor, gender-responsive, and human rights-based approaches

Regional issues and challenges

All male and female populations no matter their socioeconomic status, ethnicity, displacement, stigmatization, demographic location, or religious beliefs are entitled to the basic human right of health. This is the founding principle of Health for All. Without recognition of the social determinants of health that influence the health outcomes of men, women, boys, and girls, health for all can never be achieved.

Social determinants of health have been underestimated largely due to the biomedical orientation that is firmly ingrained in health institutions. Although a biomedical orientation is indeed essential, consideration must also be given to the contexts within which diseases and poor health outcomes occur. Health information systems that do not collect disaggregated data and present health data as averages prevent identification of the gaps between the most and least privileged male and female populations of society and establishment of clear causal links between social determinants and health outcomes. Well functioning information systems, regular health surveys and health system research are critical for monitoring health goals. This information is essential in the Region to drive policy change towards addressing the underlying causes of ill health which can then lead to more sustainable positive health outcomes. However, many of the countries of the Region that suffer from a high burden of health problems targeted by the Millennium Development Goals have poor information systems, providing a challenge to monitoring progress towards achieving the targets of the Millennium Development Goals.

Many health programmes, both within WHO and Member States, do not integrate social determinants of health, and therefore community empowerment, gender and health inequities, and absence of human rights are not addressed. The minimal resources assigned to this strategic objective proves a significant barrier to filling health gaps that are exacerbated and perpetuated by differential distribution of social and economic resources and that result in vulnerable communities of men, women, boys, and girls who are denied their basic human rights to health. In addition, lack of widespread government support in terms of policy and funding, has been a major challenge faced by the community-based initiatives programme, particularly in terms of scaling up and sustaining the programme.

The Eastern Mediterranean Region is plagued by both chronic and acute complex emergencies which are of themselves significant determinants of health as well as barriers for implementation of health programmes, especially those deemed less of a priority, such as social determinant based programmes. Yet it is in complex emergencies that the needs of vulnerable male and female populations rise to higher levels of urgency and require the recognition of differential needs that integration of social determinants of health provides. To achieve this objective, social determinants of health need to be made a priority for health and for development at both national and international levels.

Intersectoral collaboration and partnership are instrumental in addressing the underlying social and economic determinants of health, and while their value is widely recognized, they are not widely practised in the Region. Vertical health programme approaches and lack of coordination between governments and development partners result in unnecessary overlap, fragmented efforts and consequently inefficient use of resources. It will only be through broad-based partnership that the adverse effects of poverty and the health of vulnerable male and female populations will be sustainably improved.

Strategic approaches

The Millennium Development Goals, which all Member States are committed to achieving for the betterment of their populations, reflect the intersection of health and economic development and the relevance of addressing poverty and gender inequities to enable men, women, boys, and girls to access their human rights to health. Accordingly governmental and nongovernmental organizations at the

global and regional levels need to align their activities to reduce health gaps and to improve health status by addressing the social and economic determinants of health. Redressing the root causes of health inequities will also need coordinated integration by both WHO and Member States of gender, poverty, ethics, and human rights-based perspectives into health guidelines, policies and programmes. The strategic objective therefore develops and promotes tools that identify and address health inequities, such as the community-based initiatives, which empower impoverished and vulnerable communities to address their health priorities, and the WHO gender mainstreaming for health managers training, which increases capacities of health managers to integrate gender influences, multiple social determinants and human rights into health policies and programmes. Achievement of gender equity in health requires the perceptions, experiences and interests of women and men, boys and girls of all population groups, to bear on policy-making, planning and decision-making in all sectors.

Systematic action on the social determinants of health requires addressing the major burden of disease problems in countries of the Region through linking disease-specific programmes to health systems development. The best way to improve population health in developing regions, and thus to maximize the contribution of health to development, will be to link priority targeted programmes, health systems strengthening and wider action on the upstream social determinants of health. Community-based initiatives, comprising the basic development needs, healthy cities, healthy villages and women in health and development approaches, provide a key mechanism for addressing social determinants of health in the Region. The common goals of these development initiatives are to create supportive political, physical and economic policies and plans, driven by community needs and participation.

Poverty reduction is integral to improving health outcomes and to implementing health reforms. National health and poverty reduction strategies should be able to demonstrate how the benefits of development reach poor men, women, boys and girls, and should put in place monitoring systems and the processes by which these benefits are delivered. Activities will support Member States in the development of regional and national databases on development, poverty and health for evidence-based policy and planning. The Regional Office will also work towards strengthening national health institutions and human resources to integrate and scale up community-based initiatives, gender mainstreaming and human rights. Enhanced evidence-based management will facilitate gender responsive national health policies and plans to integrate social determinants of health.

Partnering with other United Nations agencies, and when appropriate civil society and the private sector, is essential to advance health as a human right and to reduce health inequities. It is also essential to address macroeconomic factors relevant to health including international trade, and to support and facilitate ethical decision-making on health-related policies, programmes and regulations. Coordination and partnership of will be focused on public sector development agencies, public health institutions, civil society and United Nations agencies. In addition, partnership and coordination with initiatives and mechanisms such as National Poverty Reduction Strategies, the Global Fund to Fight AIDS, Tuberculosis and Malaria, Global Alliance for Vaccines and Immunization, Commission on Macroeconomics and Health and the Commission on Social Determinants of Health is an integral strategic approach. Coordinated efforts will also be made to enhance gender mainstreaming both within the Regional Office and in country programmes.

The Regional Office will continue to consult with stakeholders on the adoption of regional strategies for achieving the Millennium Development Goals; monitoring and assessment of Poverty Reduction strategy papers; review and finalization of national investment plans for health; and identification of essential interventions for countries with focus on the health of vulnerable males and females. Human rights offer a unifying conceptual framework for these strategic approaches and collection and documentation of social and economic influences on health in the Region will continue the knowledge built through the Commission on Social Determinants of Health. Principles of human rights and ethics should guide the policy-making process to ensure the fairness, responsiveness, accountability and coherence of health-related policies and programmes, and to overcome social exclusion.

Regional assumptions and risks and option analysis

The following assumptions underlie achievement of this strategic objective.

- WHO senior management and national policy-makers support and mandate the inclusion of cross-cutting social determinants in policies, plans and programmes.
- The tools of human rights, ethics, economic, gender and poverty analysis are widely and effectively deployed when WHO develops normative guidance as well as when Member States make policies and implement programmes.
- Commitment to multisectoral cooperation and partnership between global and national stakeholders moves beyond rhetoric to action.
- National policy-makers recognize the added value of institutionalizing a replicable community-based initiatives model.
- Innovative measurement tools are developed for monitoring and evaluating causal links between social determinants and health outcomes.

The risks that could prevent achievement of this strategic objective are as follows.

- While social and economic determinants concern all government sectors, as well as the public, no one takes responsibility or is accountable to them, also known as the 'responsibility gap'.
- Emphasis of the biomedical approach at the expense of addressing social determinants of health continues to undermine resource allocation to this strategic objective.
- Sufficient human resources with relevant expertise are not in place across programmes and agencies to ensure that the tools of human rights, ethics, economic, gender and poverty-analysis are widely and effectively deployed.
- Misconceptions are perpetuated which associate inclusion of cross-cutting social determinants in policies, plans and programmes as an additional responsibility secondary to the Organization's regular normative and developmental work.
- WHO senior management and Member States do not hold the secretariat accountable to Health Assembly resolution WHA60.25 *Strategy for integrating gender analysis and actions into the work of WHO*.
- Commitment is not operationalized to resolution EM/RC50/R.7 *Investing in health of the poor: regional strategy for sustainable health development and poverty reduction*.

Organization-wide expected result		Regional Expected results		Indicators	Baseline	Target
07.001	Significance of social and economic determinants of health recognized throughout the Organization and incorporated into normative work and technical collaboration with Member States and other partners.	07.001. EM01	Social Determinants of Health addressed through implementation of the community-based initiative approach, focusing on community empowerment on development agenda, partnership, intersectoral collaboration and reducing inequities in health	Report of the Commission on Social Determinants of Health is produced and launched and discussions are organized with countries to develop regional strategy for actions on social determinants of health	0	Commission on Social Determinants of Health Report available in English and Arabic and recommendations are discussed in the Regional Committee and their implementation planned at least in 4 countries
				Number of countries that have adapted the health equity assessment and response tool and incorporated equity assessment and response in their national programmes	1	3
				Number of countries that have institutionalized community-based initiatives and social determinants of health as integral parts of their national development and health plans	6	9
07.002	Initiative taken by WHO in providing opportunities and means for intersectoral collaboration at national and international levels to address social and economic determinants of health, including understanding and acting upon the public health implications of trade and trade agreements, and to encourage poverty-reduction and sustainable development.	07.002. EM01	Technical support provided to improve intersectoral collaboration among government agencies and enhanced partnership with the civil society organizations to tackle social and economic determinants of health, with interventions at the policy as well as community levels	Number of context-specific guidelines and tools developed and disseminated to support countries in selecting and sequencing intersectoral policies on the social and economic determinants of health	4	8
				Number of regional and national forums organized in which policy-makers, programme implementers and civil society share experience and knowledge on community, public and civil society actions addressing the social and economic determinants of health and experiencing community-based initiatives as a tool to overcome inequity in health and development	9	15
				Regional strategy developed and approved on trade in health services from a public health perspective	0	1
		07.002. EM02	Enhanced evidence, awareness and capacity among national policy-makers regarding the implications of trade on public health in countries particularly in relation to international trade agreements	Number of countries that have undertaken general or modal studies on trade in health services	6	12
07.003	Social and economic data relevant to health collected, collated and analysed on a disaggregated basis (by sex, age, ethnicity, income, and health conditions, such	07.003. EM01	Technical support provided to develop and analyse national disaggregated databases related to health and development including the Millennium	Countries having developed and analysed national disaggregated databases related to the health and development including the	9	17

Organization-wide expected result as disease or disability).		Regional Expected results		Indicators	Baseline	Target
			Development Goals and community-based initiatives programme, and use data for planning and scaling up of related interventions	community-based initiatives programme and use it for planning and scaling up of related interventions		
				Number of tools and guidance documents developed and used by Member States to support monitoring and reporting progress towards achievement of the Millennium Development Goals	0	1
				Number of tools developed, disseminated and used for assessing the impact of policies outside the health care sector on health and health equity	1	4
07.004	Ethics- and rights-based approaches to health promoted within WHO and at national and global levels.	07.004. EM01	Advancing the concept of health as a fundamental human right at regional and national levels and supporting the mainstreaming of the right to health in all levels of health sector	Number of activities conducted in support of country efforts to advance the concept of health as a human right	4	+5
				Number of publications produced in support of countries in their activities in the area of health and human rights	4	+4
07.005	Gender analysis and responsive actions incorporated into WHO's normative work and support provided to Member States for formulation of gender-responsive policies and programmes.	07.005. EM01	Integration of gender perspectives in planning, implementation, and monitoring of national health policies and programmes	Number of activities in Member States informed by WHO gender and health tools and guidelines	3	+6
		07.005. EM02	Greater evidence base on gender and health available for Member States to inform gender responsive health planning and programming	Number of evidence-based publications and tools on gender and health produced for Member States	4	+4
		07.005. EM03	Increased productive involvement of women and girls in society for maximizing health outcomes	Number of activities in Member States using tools developed for increasing participation and capacity of women and girls in the Member States	2	+3

Strategic Objective 8: Promote a healthier environment, intensify primary prevention and influence public policies in all sectors so as to address the root causes of environmental threats to health

Regional issues and challenges

This strategic objective relates to healthier physical environments and has the unique advantage of clearly placing WHO action in the area of health and environment under the umbrella of addressing the determinants of health (together with strategic objectives number 6, 7 and 9).

The Eastern Mediterranean Region is a largely arid region which poses significant challenges for human habitation such as water shortage, extreme temperature changes, seasonal dust storms, etc. The recent climatic changes will probably increase these constraints; however the degree to which they will increase is still unknown.

In addition, many emerging environmental health problems that require attention such as urbanization, increased use of chemicals, noise and vibration and urban air pollution require special attention and both human and financial resources.

In a changing environment with modern and emerging environmental health problems, the Region is still struggling with "old problems" such as water contamination and insecurity of drinking-water supplies, solid waste disposal, inadequate environmental policies and legislation, indoor air pollution, liquid waste management, and inadequate public awareness for influencing environmental policy. In 2002, out of the 503 million inhabitants of the Region, about 86 million were without access to an improved water source and 191 million had no access to improved sanitation. In fact, a number of Member States have little chance of achieving the MDGs related to water supply and sanitation unless bold policies and action are implemented to that end.

Existing economic and political systems in countries and the challenges faced in setting national priorities result in a high number of actors in environmental health (such as municipalities and ministries of agriculture, environment and health). The roles and responsibilities of these actors are fragmented and a coherent policy for streamlining environmental health interventions is often absent at country level. There is an urgent need to promote and strengthen a multi-disciplinary collaborative approach that would involve all concerned sectors and ministries.

Several countries of the Region have experienced or are currently experiencing natural and man-made disasters that put affected populations under serious immediate and long-term environmental health risks. Unfortunately, most of these countries are not currently prepared to respond to the demands on water supply, sanitation and other environmental health services that arise during and after disasters. Climate change is also exacerbating the health condition of vulnerable populations in these Member States. Efforts have started to support countries of the Region to develop their national environmental health preparedness plans for emergencies. Lack of staff experienced in handling environmental health services is one of the main bottlenecks faced by countries. A regional network of well trained sanitarians and other environmental health specialists that are aware of emergencies needs to be established, to help in solving this problem.

Lack of access to reliable and relevant environmental health information is still a problem in most countries of the Region. This is affecting the decision-making process in different sectors concerned with environmental health. Both producers and users of environmental health information belong to a wide spectrum of governmental and nongovernmental institutions, highlighting the need for networking. Efforts are continuing towards improving access to information; networking at national and regional levels and human resources development on modern information processing and management procedures.

Some components of surveillance exist in many countries. However, the existing systems lack validity and still need developing and strengthening. In the least developed countries of the Region, there is limited capacity in ministries of health to conduct environmental health surveillance. There are no programmes for environmental health per se in these countries. In a number of countries, the hygiene

conditions of health care facilities is a cause for concern. Similarly, school environmental health facilities are inadequate.

The integration of occupational health as an essential element into the primary health care system in order to provide appropriate care to workers in the community is receiving increased attention by countries. Extensive efforts have been made in countries to develop and further strengthen national programmes on environmental and occupational health.

Strategic approaches

- The preparation by each country of a national strategy and plan of action for health and environment and of such instruments for specific environmental health areas.
- Technical support to Member States to make use of WHO guidelines to establish and apply national standards for drinking-water quality, including water from desalination facilities; to promote best practices in hygiene and sanitation, wastewater management and safe reuse, and health care waste management; and guide risk factor assessment for action in air quality, chemical safety, food safety and empowerment of communities for enhancing social responsibility in matters of health and environment.
- Support to countries for developing policies and actions conducive for the implementation of environmental health measures in healthy settings and for ensuring healthy environments of the vulnerable populations, particularly children, including in case of emergencies.
- Assistance to ministries of health to increase their capacity for promotion, monitoring and surveillance of environmental health components as well as for documenting and developing evidence for health related national environmental policies and measures.
- Maintaining the traditional role of the Centre for Environmental Health Activities (CEHA) in facilitating Member States' access to environmental health information through provision of information services, linking CEHANET with the virtual health library of the Regional Office, and updating CEHA's regional environmental health bibliography and library database and making these available to users through the virtual health library.
- Promotion of coherence in policy and synergy in practice regarding healthy workplace environments among all concerned parties (governments/ministries and agencies, employees' organizations and employers' organizations).
- Encouraging establishment and strengthening of basic occupational health services and integrating occupational health and safety into primary health care systems and into academic curricula for educating and training primary health care workers.
- Guidance of the Regional Advisory Committee on Health and Environment for activities of the environmental health programmes in the Region.

Regional assumptions and risks and option analysis

- The role of the Ministry of Health in maintaining governance towards identifying and addressing health effects of environmental condition is not always recognized or exercised.
- Human, physical and financial resources to address traditional, modern and emerging environmental health problems are limited at country and regional level.
- Some countries of Region are facing major administrative difficulties and challenges in the development of environmental and occupational health services and programmes, such as multisectoral supervision, lack of coordination between concerned authorities and insufficient material and human resources.
- Available data on health risks, morbidity and mortality from diseases and accidents attributed to living, working and recreational conditions at the country and regional levels are not adequate and probably underestimate environment-related health and safety problems.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
08.001	Evidence-based assessments, norms and guidance on priority environmental health risks (e.g. air quality, chemical substances, electromagnetic fields, radon, drinking water, wastewater reuse) developed and updated; technical support to international environmental agreements and for monitoring MDGs.	08.001. EM01	Technical support provided to countries to adapt and implement evidence-based normative guidance and adopt appropriate environmental initiatives and interventions for managing and reducing environmental public health risks, including in case of emergencies and disasters	Number of science-based guidelines and norms on priority environmental public health risks from the environment produced, adapted, translated and disseminated	5	7
				Number of countries with national legislation and standards based on WHO guidance for reducing health risks in the context of implementation of international environmental and occupational health agreements	13	18
08.002	Technical support and guidance provided to Member States for the implementation of primary prevention interventions that reduce environmental hazards to health, enhance safety and promote public health, including in specific settings and among vulnerable population groups.	08.002. EM01	Technical guidance and support provided to countries for developing policies and actions, encouraging and motivating the implementation of environmental health measure in healthy settings, including workplaces, schools, homes and the community	Number of country-based initiatives successfully using healthy settings approaches for preventing environmental health risks	3	5
				Number of evaluations of costs and benefits of primary prevention of environmental risks to health and interventions in specific settings developed and disseminated	0	2
08.003	Technical assistance and support provided to Member States for strengthening occupational and environmental health policymaking, planning of preventive interventions, service delivery and surveillance.	08.003. EM01	Technical support provided to countries for building capacities to develop policies and national plans of action for strengthening environmental and occupational health services, surveillance systems, and research	Number of countries having updated policies and national plans of action for healthy and safe occupational environments	2	11
				Number of countries implementing WHO guidance for tackling priority occupational health risks, such as elimination of silicosis and asbestos-related diseases, occupational health and safety of health care workers, occupational cancer, work-related stress	0	6
08.004	Guidance, tools and initiatives created in order to support the health sector to influence policies in priority sectors, assess health impacts, determine costs and benefits of policy alternatives in those sectors, and select investments in non-health sectors that improve health, the environment and safety.	08.004. EM01	Technical guidance, knowledge and tools made available for health sector to provide strategic direction and leadership to influence other sectors' policies (such as water, energy, transport, agriculture), promote awareness networks and partnerships for prevention of environmental risks to health, and adapt the structure of the ministries of health to this role	Number of countries with initiatives assessing health impacts of development policies to facilitate decision making	4	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
08.005	Health-sector leadership enhanced for creating a healthier environment and changing policies in all sectors so as to tackle the root causes of environmental threats to health, through means such as responding to emerging and reemerging consequences of development on environmental health, climate change, and altered patterns of consumption and production and to the damaging effect of evolving technologies.	08.005. EM01	Technical guidance and support provided to countries for strengthening capacities for monitoring trends and assessing health impacts of recognized emerging and re-emerging environmental and socioeconomic development, especially, on vulnerable populations	Number of countries implementing a system for identifying and responding to key emerging and re-emerging environmental health concerns	0	1
08.006	Evidence based policies, strategies and recommendations developed and technical support provided to Member States to identify and tackle public health problems resulting from climate change.	08.006. EM01	Strengthened national capacities for formulating policies, strategies/plans of action to identify and tackle public health problems resulting from climate change	Number of countries with national capacities for formulating policies, strategies and plans of action addressing public health problems emanating from climate change	0	3

Strategic Objective 9: Improve nutrition, food safety and food security, throughout the life-course and in support of public health and sustainable development

Regional issues and challenges

Food safety and nutrition interventions are vital for both preventing and ameliorating the harmful consequences of the burden of foodborne diseases and burden of malnutrition that often starts as a consequence of inadequate fetal and infant growth.

Some improvement in the nutritional status of the population, particularly children under the age of five years, has been achieved in several Member States, although prolonged humanitarian crises in several Member States have either halted any progress or caused further deterioration in the nutritional status of the population. While acute under-nutrition (wasting) has declined in many Member States, chronic malnutrition (stunting) remains at an average of 20% in the Region. 30% of the children under the age of five years and 40% of the women in the reproductive age group are anaemic, primarily due to iron deficiency. The average prevalence of overweight has been estimated at 50% and the average prevalence of obesity, a major risk factor for noncommunicable diseases, at 30%. Ensuring adequate nutrition for the mother, the fetus, infants and young children will continue to remain a major challenge. Wider adaptation and implementation of the global strategy on infant and young child feeding is needed. For the control and prevention of micronutrient malnutrition, a comprehensive approach consisting of food diversification, supplementation, treatment and food fortification would be advocated.

Member States of the Eastern Mediterranean Region have recognized the need for increased national attention and international cooperation to strengthen food safety systems in the Region. Ensuring safe and healthy food, whether domestically produced and consumed, imported or exported, continues to remain a major challenge. As a whole, the Region relies on imports for over 60% of its food supply. To reduce the occurrence of foodborne disease and protection of consumers, countries opted for sampling and analysing the final food products. However, it is now widely recognized in the Region that it is impossible to provide adequate protection by merely sampling and analysing the final food products. Food safety is multisectoral and must be addressed throughout the farm-to-fork food continuum utilizing a science-based approach. The challenges related to food safety in the Region are enormous. There is poor epidemiological characterization of food hazards and foodborne diseases and their direct and indirect impact on public health. As well, the public health infrastructure, including laboratories, is inadequate in many countries; the public health sector has a weak role in the development of food safety policy and plans; food legislation and organizational services are fragmented; and human resources are limited in food inspection. The multidisciplinary nature of food safety and quality work requires the cooperation of many professionals and resources. The other major issue is the lack of adequate consumer information and consumer protection agencies and legislation in the Region. Food insecurity in the Region is becoming an issue of concern due to fuel prices and decreasing purchasing power of the community for food. If it continues, this trend will reverse the improvements gained in nutrition and will increase the incidence of food-related diseases. Five countries of the Region are suffering from food insecurity and complex emergencies.

Strategic approaches

The work under this strategic objective focuses on: the nutritional quality and safety of foods: the promotion of healthy dietary practices through the life-course, starting with pregnant women, breastfeeding and adequate complementary feeding, and considering diet-related chronic diseases; the prevention and control of nutritional disorders, including micronutrient deficiencies, especially among the biologically and socially vulnerable with emphasis on emergencies and in the context of HIV/AIDS epidemics; the prevention and control of zoonotic and non-zoonotic foodborne diseases; the stimulation of intersectoral actions that promote the production, consumption and access to food adequate quality and safety; the promotion of higher levels of investment in nutrition, food safety and food security, at global, regional and national levels.

Regional assumptions and risks and option analysis

Nutrition, food security and food safety issues exhibit widely divergent features in countries of the Region. Such scenarios require situation-specific strategies to address the emerging challenges. While some countries possess high quality nutrition and food safety delivery infrastructures with strong food security, several others have poor performing or rudimentary food safety and nutrition services with co-existing food insecurity among their population groups.

While the current interest and support of the national authorities to the different food safety and nutrition programmes are expected to remain in place, the emergence of complex emergencies and humanitarian crises may result in an increase in food insecurity at the household and individual levels. In the formulation of the regional expected results in the areas of nutrition/food security and food safety, these factors have been taken into consideration.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
09.001	Partnerships and alliances formed, leadership built and coordination and networking developed with all stakeholders at country, regional and global levels, in order to promote advocacy and communication, stimulate intersectoral actions, increase investment in nutrition, food safety and food security interventions, and develop and support a research agenda.	09.001. EM01	Technical and programmatic collaboration established with partners at the global, regional and national levels for promoting intersectoral food and nutrition actions	Number of selected low income developing countries that have institutionalized and functional coordination mechanisms, for promoting intersectoral approaches and actions in the area of food safety, food security and nutrition	0	5
		09.001. EM02	Enhanced international and national multisectoral food safety collaboration	Number of targeted low income developing countries that have included nutrition, food safety and food security activities in their sector-wide approaches, poverty reduction strategies and/or development policies, plans and budgets including a funding mechanism for supporting nutrition and food safety activities	0	4
09.002	Norms, including references, requirements, research priorities, guidelines, training manuals and standards, produced and disseminated to Member States in order to increase their capacity to assess and respond to all forms of malnutrition, and zoonotic and nonzoonotic foodborne diseases, and to promote healthy dietary practices.	09.002. EM01	Development of regional and national-level standardized guidelines and protocols for the detection, prevention, management and monitoring of all forms of malnutrition	Number of new nutrition and food safety standards, guidelines and training manuals produced and disseminated to countries and the international community	2	4
		09.002. EM02	Development of regional and national-level standardized guidelines/protocols for the detection, prevention, management of foodborne diseases and monitoring food safety and quality	Number of new norms, standards, guidelines, tools and training materials for zoonotic and non-zoonotic foodborne diseases prevention and management	1	4
09.003	Monitoring and surveillance of needs and assessment and evaluation of responses in the area of nutrition and diet-related chronic diseases strengthened, and ability to identify best policy options improved, in stable and emergency situations.	09.003. EM01	New WHO growth standards introduced and implemented in Member States	Number of countries that have adopted and implemented the WHO Child Growth Standards	2	10
		09.003. EM02	National nutrition databases and nutrition surveillance systems on the major forms of malnutrition strengthened	Number of countries that have nationally representative surveillance data on major forms of malnutrition	3	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
09.004	Capacity built and support provided to target Member States for the development, strengthening and implementation of nutrition plans, policies and programmes aimed at improving nutrition throughout the life-course, in stable and emergency situations.	09.004. EM01	Strengthened national capacities for formulating and implementing national food and nutrition strategies/plans of action to improve the nutrition status of the population as well as to address emerging nutrition challenges and transitions	Number of countries receiving WHO support that have developed and implemented at least 3 high-priority actions recommended by the Global Strategy on IYCF	2	6
		09.004. EM02	Micronutrient status of the populations of Member States improved through innovative programmes of food supplementation and food fortification with micronutrients of public health significance	Number of countries receiving WHO support that have developed and implemented strategies to prevent and control micronutrient malnutrition	4	8
		09.004. EM03	National capacities of Member States to respond to nutrition needs of the population living under complex emergency situations strengthened and improved	Number of countries receiving WHO support that have developed and implemented strategies to promote healthy dietary practices to prevent diet-related chronic diseases	1	5
				Number of low-income developing countries receiving WHO support that have included nutrition in their comprehensive responses to HIV/AIDS and other epidemics	0	2
				Number of countries receiving WHO support that have strengthened national preparedness and response to nutritional emergencies	1	4
09.005	Systems for surveillance, prevention and control of zoonotic and non-zoonotic foodborne diseases strengthened; food hazard monitoring and evaluation programmes established and integrated into existing national surveillance systems, and results disseminated to all key players.	09.005. EM01	Zoonotic and non-zoonotic foodborne disease surveillance, and hazard monitoring programmes strengthened and integrated into existing national surveillance system with results being disseminated	Number of countries that have established/ strengthened intersectoral collaboration for the prevention, control and surveillance of foodborne zoonotic diseases	2	4
				Number of countries that have initiated/ strengthened programmes for the surveillance and control of at least one major foodborne zoonotic disease	1	5
09.006	Capacity built and support provided to Member States, including their participation in international standard-setting in order to increase their ability to assess risk in the areas of zoonotic and non-zoonotic foodborne diseases and food safety, and to develop and implement national food control systems, with links to international emergency systems.	09.006. EM01	Strengthened capacity to conduct chemical and microbiological risk assessment, including their participation in international standard-setting to increase their ability to assess risk in the areas of zoonotic and non-zoonotic foodborne diseases and food safety	Number of countries supported to participate in international standard setting activities related to food, such as the Codex Alimentarius	3	6
				Number of countries receiving support from WHO that have built national systems for food safety and foodborne zoonoses emergencies with international links	9	12

Strategic Objective 10: Improve health services through better governance, financing, staffing and management, informed by reliable and accessible evidence and research

Regional issues and challenges

Health care delivery and health management support (SO10.1)

In line with the goals, objectives and standards of health for all (HFA), this strategic objective aims to maximize the potential of the national health systems to provide equitable access, quality of care and better performance at all levels of care.

Partnership between WHO and its Member States should result in the development of credible health care delivery with strong leadership and effective management of national health systems. This partnership will focus on developing adequate organizational and managerial capacities at different levels of care delivery to address: a) population-based and personal health; b) decentralization; c) quality and safety of care and quality monitoring and assurance; d) district health system development; e) integration of vertical programmes and projects at country level; f) public-private mix; g) continuity of care including referral systems among different levels and providers of care; h) management effectiveness programmes; i) models of health care delivery; j) essential packages of care at primary and secondary levels; k) balance of ambulatory and inpatient hospitalized care; and l) family practice and other gate-keeping mechanisms.

New global partnerships are being involved in health system strengthening activities: with the GAVI Alliance for six low income countries in the Region, and with the Global Fund to Fight AIDS, Tuberculosis and Malaria in addressing the systemic impediments for HIV/AIDS, tuberculosis and malaria control programmes in many countries of the Region.

This agenda of this strategic objective will require close collaboration with efforts to achieve other strategic objectives. The organization, management and delivery of health services underpins all strategic objectives concerned with:

- equity and health systems (strategic objectives 12–14);
- achievement of specific health outcomes (primarily strategic objectives 1–4);
- the leadership role of ministries of health and the need for institutional capacity to organize health services, especially in least developed countries;
- primary health care elements and components;
- community roles in health care delivery.

Challenges in the organization of health care delivery include:

- Fragmentation of health services and complexity in management of the different networks, hierarchies and voluntary organizations.
- Rampant inequity in access of health care among and within countries of the Region.
- Limited managerial capacity at all levels of care, particularly at district level, in most countries of the Region, especially in ways to organize the services, to maximize use of resources and to ensure core public health functions are provided.
- Needs for effective decentralization and accountability to national health care delivery goals and policies.
- Lack of clear health care delivery policy objectives and strategies, such as Certificate of Need, that correspond to emerging health care system needs.
- Difficulties in reconciling legitimate community health care demands with capacity for provision of health care, especially in countries with limited resources.
- Lack of quality culture, clinical governance and evidence-based practices.
- Inadequate regulatory and legislative mechanisms to ensure patient rights and the quality of health care delivery by public and private providers.
- The double burden of disease, which has created the need for reorientation of ministries of health to address diseases of lifestyle and relevant risk factors.

Efforts to achieve this strategic objective will focus on ensuring that: local, national and regional institutions own and implement relevant health care delivery initiatives; health care delivery programmes are incorporated into WHO planning, policy and technical work; and there is consensus on health care delivery agenda in countries of greatest need. Collaborative efforts by WHO, countries and other partners will aim to make organization of health services more visible, understood, widely discussed and recognized as important.

Health policy and systems development (SO 10.2 and 10.3)

- Limited capacity of ministries of health to formulate clear policy objectives and strategies that correspond to health system needs, are based on evidence and are compatible with the cultural and social values of societies.
- Inadequate capacity among ministries of health to manage the increasing number of financing and implementation partners and networks such as ministries of finance and planning, national legislature, international agencies, multilateral, bilateral and nongovernmental agencies, and various types of private enterprises and civil society organizations.
- Inadequate legislative and regulatory mechanisms to ensure socially responsible behaviour of stakeholders, fair rules of game for all players, and implementation of strategies leading to the attainment of policy objectives.
- Lack of mechanisms to ensure effective interaction with other sectors that influence social, economic and environmental determinants of health.
- Lack of mechanisms to establish and promote effective accountability mechanisms to protect nationally agreed priorities. Making evidence on health and the health system widely available is a key aspect of accountability.

Health information, systems research and knowledge management (SO 10.4, 10.5, 10.6, 10.7)

- Weak capacity and infrastructure to manage information and knowledge including the necessary policies, processes, technologies and institutions for generation, publishing, managing, dissemination, sharing and utilization of knowledge.
- Limited capacity of ministries of health to establish or operate health information systems that correspond to health system functions that are based on evidence, resulting in weak monitoring of health system performance.
- Critical lack of basic elements of information, research and knowledge management, which need to be looked as a continuum rather than separate programmes at different levels of health systems in the Region.
- Shortcomings in national health information systems, as identified in the WHO framework on health statistics, which have resulted in development of the programme on health statistics and the Health Metrics Network partnership.
- Limited capacity for delivering nationally-relevant research for health including health systems research, for establishing and maintaining sound health information systems and translating research findings into policy and practice.

Health workforce (SO 10.8 and 10.9)

“Health workforce in crisis”, the theme of World Health Day 2006, marked the beginning of a decade devoted to human resources development as a priority, by WHO and Member States, and to the strategies and actions that can be taken to address various issues.

Human resources development has been a priority in the Eastern Mediterranean since 2001. It is an essential part of health systems reform. The process of human resources development extends into three stages: 1) policy formulation and multisectoral and integrated planning for human resources for health; 2) production in balanced quantities and relevant quality; and 3) optimum utilization and management.

It is estimated that there are currently more than 2 million health workers in the Eastern Mediterranean Region and that the immediate need exceeds an additional 2 million health workers.

National policies on human resources for health are lacking in many of the Region's countries. In other countries, updating and modernization of the existing policy is also needed to meet the rapid global, regional and national changes and challenges. Economic, social and political factors are a few of many factors that should be considered when such policies are formulated. Currently, several countries possess national policies which respond to both national needs and global challenges. The erupting conflicts in the Region have created great demands for revising and formulating new policies to meet post-conflict situations. Political drives have resulted in the establishment of a large number of educational institutes for certain health professions, while at the same time ignoring other health professions. This can be seen in the sharp increase in the number of medical colleges from 17 in 1950 to more than 250 in 2005. The number of colleges of nursing, pharmacy and dentistry are generally still low. Most of the new colleges were established as carbon copies of already existing colleges without analysis of the need for, or quality of, the graduates. Educational programmes in the majority of health professions education institutes need to be thoroughly revised and redesigned according to the needs of the evolving health systems and services, rapid global changes, market economy, public health demands and other priorities. Information on health professions education institutes is inadequate to share and encourage collaboration and exchange of experience. Optimal utilization, updating and management of human resources for health is a challenge for many countries of the Region. Issues include inequitable recruitment policies and procedures, low pay systems, maldistribution in location and in skill mix, emigration and so-called brain drain, weak national systems of continuing professional development and vertical postgraduate programmes.

Shortages of nurses and midwives in many countries of the Region, within the global shortage of nursing and midwifery personnel, require a systematic review of the nursing human resource situation in the Region with respect to planning, development and management of this resource to meet the increased demand for nursing and midwifery services, both quantitatively and qualitatively. The general public and managers of health services of the countries in the Region are calling for better prepared nurses at all levels, especially at the advanced specialist level. There is urgent need for implementation of management processes conducive to efficiency and effectiveness to ensure retention and development of the nursing workforce. In order to provide a quality nursing service and continually improve the quality of care provided to patients and clients, nurses in this millennium are expected to base their practice on evidence derived from research. They are also expected to collaborate and develop partnerships with other health care providers and sectors, both public and private, in order to share their expertise and better utilize available resources. In collaboration with others, they are expected to develop policies, regulations, legislation and evaluation to ensure a consistent standard of care for all, regardless of whether the care is obtained in the private or public sector. As nursing is a labour-intensive service, there is a great demand on all countries of the Region to invest in the development of the managerial and leadership capabilities of nurses. Changes in the delivery of health care in general and nursing services in particular call for the institutionalization of nursing regulation as a means to improve the quality of nursing care and protect the health of the people.

Addressing the shortage of nurses in many countries of the Region will require expansion of schools of nursing to accommodate the number of incoming students. However, these schools are currently struggling with a shortage of qualified national nursing and allied health teachers who can contribute to the production of the human resources required to meet the health services needs, and who can act as role models for the younger generation.

Health care financing (SO 10.10, 10.11, 10.12)

- The economies of many countries in Eastern Mediterranean Region have been experiencing a boom or modest growth as a direct or indirect result of increasing oil prices and a weaker dollar.
- The majority of populations in the Region are not covered by adequate social health protection schemes. Many households are exposed to catastrophic health spending or are impoverished because of health spending, and some even forgo seeking health care altogether.
- Public and private health expenditure has been increasing in nominal terms in most countries in the Region. However, the share of out-of-pocket expenditure has not been showing any sign of

decline despite the efforts of some countries to do so by introducing new prepayment schemes or expanding the existing ones.

- The situation in low income countries remains problematic. The available resources in low income countries is not sufficient to enable these countries to provide even the basic public health services for their population and available resources are allocated inefficiently.
- Political instability in some countries in the Region remains a major concern for both economic and health system development.
- National capacity to support development of health care financing remains weak in the Region, but past efforts to build such capacity has started paying off.

Strategic approaches

Health care delivery and health management support

The strategic intent of the organization and management of health services based on primary health care is to maximize the potential of national health systems to achieve well-distributed quality care in line with the HFA goal and standards. The Regional Office in collaboration with countries will embark on the following strategic approaches.

- Assessing and advising on the trend of expanding responsibilities in primary care, resulting from task transfer from secondary care, as well as the stronger involvement of primary care in screening, prevention and health promotion.
- Ensuring institutional development of different levels of delivery of care and clarifying roles, functions and responsibilities through developing flexible structures, organizational charts, functions and lines of accountability through and building capacity for leadership and coordination.
- Strengthening and aligning structures, processes and functions at different levels of care to define their range for strategic, executive and operational mandate.
- Promoting and supporting the approach of general practice or family medicine to ensure continuous comprehensive and coordinated care covering all categories of the population, irrespective of age, socioeconomic level or sex.
- Encouraging risk stratification of catchment populations of health care facilities to ensure proactive and strategic delivery of care. Risk stratification will also provide the basis for integrated approaches to care.
- Supporting Member States to assess decentralization, including hospital autonomy, to come up with a best practices framework for decentralization in health systems to assist countries in implementation of health system decentralization.
- Improving the credibility of health service delivery through implementing a programme of continuous quality improvement based on quality standards for individuals, departments, hospitals and organizations, against which performance will be measured.
- Developing a health facilities coverage plan through mapping health facilities in coordination with the development of a human resource plan.
- Mapping of actors and their decision-making process for organizing different forms of services such as networks, hierarchies, isolated clinics and their combinations and building effective partnerships between various providers and including civic groups.
- Supporting countries to develop cost-effective core packages of care at primary and secondary care levels.
- Strengthening the delivery of sustainable, quality, accessible health services through planning for, and the effective and efficient implementation of, the basic health services package with an emphasis on community and outreach activities.
- Assessing and strengthening entry points to service delivery, including the emergency medical services, to ensure continuity of care.
- Enhancing risk management approaches at all levels of care with particular emphasis on hospital patient safety.

- Developing strategies to position health care delivery so that it interacts effectively with communities and related sectors to maximize the impact of their respective inputs and policies in health promotion and risk reduction.
- Exploring the potential of e-care in increasing access and improving quality of care.

Health policy and systems development

- Providing support to ministries of health for activities aimed at strengthening the health policy and planning function. Efforts will be made to ensure fit between structures and functions, powers and responsibilities, resources and needs.
- Promotion of evidence-based, ethical and consistent health policies and strategies including establishment of a regional health system observatory for analysis, monitoring and policy advice on health system reforms in countries of the Region.
- Building capacity to develop necessary skills in policy analysis and formulation and in strategic planning among Member States as well as WHO country and Regional Office staff.
- Building effective partnerships between various stakeholders including donor agencies, academia, civil society and the private sector, and supporting national governments to coordinate donor assistance.
- Promoting stronger health legislation and regulatory support to ministries of health for effective policy implementation.

Health information, systems research and knowledge management

- Supporting ministries of health in improving performance of health systems through building capacity in the use of health information and evidence to enable the health system to achieve its goals and objectives.
- Bridging knowledge gaps with pertinent health systems research and ensuring wider dissemination of research results.
- Supporting organizational change in the system management based on evidence-based information and applying comparative knowledge gained through experience and health system research.
- Strengthening national information systems that can generate, analyse and use reliable information from population-based sources (surveys, vital registration), as well as clinical and administrative data sources, through collaboration with partners (e.g. United Nations, other agencies and the Health Metrics Network partnership).
- Supporting a relevant research agenda to ensure that the right knowledge gets to the right people (policy-makers, managers, practitioners, development partners and the general public) for effective decision-making and performance monitoring across the health system.
- Facilitating exchange and dissemination of knowledge and experience within and between countries, and enhancing access to information and knowledge.
- Strengthening capacity and infrastructure to manage information and knowledge including developing the necessary policies, processes, technologies and institutions for generating, publishing, managing, disseminating, sharing and utilizing knowledge.

Achievement of the strategic objective will require an available workforce in the right places, in the right numbers, and with the right skills to respond to the health needs of the population within the context of each country's own health system.

To create such a workforce, it will be necessary to strengthen advocacy for health workforce improvement at global, regional and national levels with partnerships created and promoted at all levels. Health workforce information systems are required, as is the development of evidence-based comprehensive national workforce policies and strategic health workforce plans which are systematically implemented, monitored and evaluated. Evidence-based best practices on development, education and management of health workers need to be collated and disseminated. Similarly, adequate funding is needed for the health workforce which will require discussions and negotiations with finance ministries, labour and education ministries and international development counterparts.

In addition, it will be necessary to expand the capacity and improve the quality of education and training institutions and ensure appropriate skill mix and equitable geographical distribution of the health workforce through effective deployment and retention by means of context-specific incentives.

In supporting the efforts of Member States, the Secretariat will develop and share the knowledge (data, information and evidence) needed to change current practices, so that health workforce challenges are addressed and the overall performance of the health workforce continuously improves. Specifically, the Secretariat will focus on the following areas.

- Provision of support and guidance and development of guidelines and evidence to support human resources for health policy formulation and implementation by linking policy frameworks for human resources for health with other aspects of health systems development.
- Development of guidelines to support quality assurance of health professions education and training and to support optimum and integrated utilization and management of human resources for health with special emphasis on nursing and allied health professions.
- Supporting the development of national health workforce leadership at central and peripheral levels to mobilize resources for the health workforce and to formulate, implement, monitor and evaluate health workforce policies and plans responsive to health needs.
- Strengthening national education systems, including schools and universities, to support the production of all types of health workers, with appropriate skills and competencies.
- Strengthening the knowledge base through supporting national capacity to develop a health workforce information system and promote health workforce research.
- Supporting mechanisms for regional networking of stakeholders, such as health workforce observatories and educational development centres, to generate information for evidence-based policy-making, monitoring and evaluation.

Health care financing

Use of health economic principles, analytical tools and knowledge-based approaches to health financing policy formulation and sharing of experience between countries and partnership with national and international related institutions represent the foundation of WHO's approach to improving health care financing in the Region.

The regional strategic approaches to support Member States will be based on resolutions EM/RC51/R.6 and WHA58.33, and will cover all the WHO core functions. This will involve the following action.

- Advocating for more and predictable funds for country health systems and participation in global and regional partnerships that further this aim.
- Supporting ministries of health to put health higher on the domestic agenda and, as appropriate, to advocate for more funds from ministries of finance and external sources, and for health-related activities from other sectors.
- Supporting countries to develop and sustain high levels of accountability and transparency in the use of funds, and to develop their stewardship functions relating to financial management where necessary.
- Developing evidence and options and providing technical support to reduce reliance on out-of-pocket payments where they deter people from obtaining interventions or result in severe financial hardship by developing prepayment institutions and mechanisms.
- Provision of technical support and evidence on ways to improve transparency and accountability in health financing systems, including efforts to identify the extent of informal payments, to ensure that the entitlements and obligations of the population are specified, and that systems deliver on them.
- Provision of technical support and evidence for policy development on ways to improve efficiency, including ensuring adequate financing for key inputs such as medicines and human resources, and for key actions such as prevention, promotion and intersectoral action.

- Provision of technical support and evidence for policy development on ways to improve equity in resource use, including identifying groups suffering financial catastrophe and impoverishment because of health payments, and methods that can be used to protect them.
- Sharing of country experiences with different types of financing, pooling and purchasing and provision arrangements in different settings, and the factors associated with success in sustaining progress on key policy objectives.
- Provision and dissemination of norms, standards and analytical tools relevant to the above.
- Provision and dissemination of information necessary for the development, operation and monitoring of fair, adequate and sustainable health financing systems.
- Building capacity at the country level and in WHO regional and country offices.

Regional assumptions and risks and option analysis

Health care delivery and health management support health policy and systems development, and health information, systems research and knowledge management

- All planned resources, especially other resources for WHO will be available on time.
- Contributions from donors to countries of the Region will increase.
- The role of ministries of health will be strategic and ministries of health will adapt and adopt WHO policies and guidelines on organization of health services.
- No drastic interruptions to national health systems will occur due to natural or man-made emergencies.
- Resources allocated to health care delivery will increase to allow quality core services.

Success in strengthening the way the health system is governed will be based on several assumptions: basic consensus that the state has responsibility for the health of the whole population; increasing commitment by countries to using evidence for resource allocation for health programmes; and adequate capacity and resources in WHO country offices to provide health system and policy support to Member States.

Risks include the lack of international and national investment in this area; inadequate coordination and harmonization between major international partners; and the tendency to invest in short-term solutions that may not be sustainable. Another risk is that of not ensuring a baseline and proper situation analysis based on evidence to be able to advise on best policy options. It must be emphasized that stronger health information systems are a prerequisite for stronger health systems.

Health workforce

Progress towards this strategic objective depends on the overall strengthening of health systems. In particular, it depends on the sustainability of recent international efforts to tackle the crisis in human resources for health, including the plan of action proposed in the *World health report 2006*.

Partnerships in this area are very important, as this is a cross-sectoral issue. But success will depend on key processes such as WHO's active engagement with all stakeholders, including civil society, professional associations and the private sector.

A number of risks should be kept in mind, such as political instability, migration, disasters, wars and economic trends. It is assumed that the trend towards privatization of health education will continue, as well as other trends such as the privatization of service delivery and the migration of health personnel.

This is an environment of constant change: agile and rapid reactions are required from WHO and its partners to respond to these continuous challenges.

Health care financing

Achieving the strategic objective will require:

- Improved political stability throughout the Region

- Developing and maintaining effective partnerships and involvement of stakeholders at national, regional and global levels. Of particular importance are the international and regional financial institutions, bilateral donors and ministries of finance and planning.
- Sustained commitment by countries and development partners to the goal of achieving universal coverage and reducing out-of-pocket expenditure.

Risks to achieving the strategic objective include the following.

- Major events and disruptions, natural or manmade, in Member States.
- Recent increases in funding for health in poor countries are not sustained, or are tied very closely to only a few of the key health problems facing countries.
- Increased funding from external sources bypasses, rather than strengthens, domestic institutions for revenue collection, pooling of funds and purchasing/provision of interventions and services.
- Mechanisms of trying to improve the predictability of external flows for health will not be supported internationally.
- Countries and development partners reduce the emphasis placed on universal coverage.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
10.001	Management and organization of integrated, population-based health-service delivery through public and nonpublic providers and networks improved, reflecting the principles of integrated primary health care, scaling up coverage, equity and quality of health outcomes	10.001. EM01	Strengthened national capacities for organizing health services based on primary health care to be able to chart the future of the health services, acquire resources, coordinate, communicate and effectively manage the environment and operate technical health services and programmes	Number of countries that have adopted the WHO revitalized primary health care strategy	0	6
				Number of countries that have adopted the family practice model	0	4
		10.001. EM02	Improved continuity of care, accountability and communication between levels of care and within health facilities and their staff members to ensure more efficient operations by introducing multidisciplinary teams to manage integrated quality services through decentralized structures	Number of countries that have developed policies and guidelines on essential packages of services for the first level of care	3	6
				Guidelines on developing national standards and norms for first and secondary care levels including staffing pattern and technology	0	Manuals
				Number of targeted countries that have reviewed and redesigned standard operating procedures, patient pathway to improve integration of services	3	6
		10.001. EM03	Operationalized district health systems schemes as the viable decentralized entity of the health care delivery system ensuring universality, quality equity and efficiency of health services	Number of countries where districts have developed and used the empowering tool of district team problem appraisal and solving techniques	4	7
				Number of targeted countries that have assessed decentralization using WHO/EMRO tools	2	10
				Number of GAVI/HSS eligible countries that have developed a successful proposal, implemented and evaluated at least one yearly plan	0	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
		10.001. EM04	Regional strategy developed to find the best balance of public–private mix and a partnership role for the private and civil society sectors, with a view to cost containment, improving quality, equity and coverage	Number of targeted countries that have developed a national public–private partnership strategy	0	4
		10.001. EM05	Comprehensive programmes for continuous quality improvement and accreditation implemented based on quality standards for departments, facilities and organizations against which performance will be measured and which enhance accountability of the national health system	Number of countries implementing continuous quality improvement	2	5
				Number of countries launching accreditation	2	4
		10.001. EM06	Strengthened national, subnational and facility management capacities—including secondary and tertiary care—concerning adequate numbers, competencies, functional support systems and conducive and enabling work environment	Number of countries using the health services management framework to research, analyse and apply to strengthen the management system	0	10
				Number of countries that have reviewed medical records according to WHO standards	0	6
				Number of countries with updated leadership and capacity development programmes in hospital management using WHO manuals and guidelines	4	8
		10.001. EM07	Enhanced safety of patients at policy, managerial and clinical levels, with safety systems in health care services redesigned so that adverse events are less likely to occur	Number of targeted countries that have adapted and conducted WHO/EMRO's guidelines on patient safety	6	8
				Number of hospitals that have undertaken the patient safety friendly hospital initiative	0	10
				Number of countries that have launched the patients for patient safety programme	0	6
10.002	National capacities for governance and leadership improved through evidence based policy dialogue, effective governance and leadership, institutional capacity-building for policy analysis, greater transparency and accountability for performance, and more effective intersectoral collaboration.	10.002. EM01	Strengthened governance function of the health system and improved capacity of ministries of health to formulate evidence-based, ethical and consistent health policies and strategies that contribute to improved performance of the health system	Number of countries with updated national health policies and sector development strategies	2	6
				Number of countries with institutionalized capacity for policy analysis and strategic planning	2	5
		10.002. EM02	Enhanced capacity of the ministries of health of member states in health legislation and regulation, standard setting and their enforcement	Number of countries with institutional capacity for health sector regulation and well established enforcement mechanisms	2	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
10.003	Coordination of the various mechanisms (including donor assistance) that provide support to Member States in their efforts to achieve national targets for health system development and global health goals improved.	10.003. EM01	Ministries of health in low income and countries in complex emergencies develop effective donor coordination mechanisms and utilize external funds	Number of countries with established donor coordination mechanisms	0	5
10.004	Country health information systems that provide and use high-quality and timely information for health planning and for monitoring progress towards national and major international goals strengthened.	10.004. EM01	Health information systems strengthened based on strategic plans developed following assessment and identification of priorities with the support of the Health Metrics Network	Number of countries that have well functioning health management information systems	5	8
		10.004. EM02	Health sectors review and situation analysis developed based on national burden of disease	Number of countries that plan/conduct national burden of disease studies	2	4
		10.004. EM03	WHO programme on health statistics regionalized and progress on MDGs monitored using core indicators	Number of countries that use evidence-based information to monitor performance of health system and health-related MDGs	5	8
		10.004. EM04	Norms and standards established and technical guidelines developed to support implementation of WHO international classification, notably International Classification of Diseases (ICD) and International Classification of Functioning, Disability and Health (ICF)	Number of countries using ICD	8	12
		10.004. EM05	Sample or sentinel vital registration systems established and/or strengthened in low and middle income countries	Number of countries with functioning vital registration system	4	7
10.005	Better knowledge and evidence for health decision-making assured through consolidation and publication of existing evidence, facilitation of knowledge generation in priority areas, and global leadership in health research policy and coordination, including with regard to ethical conduct.	10.005. EM01	Improved capacity of ministries of health to formulate evidence-based, ethical and consistent health policies and strategies that contribute to improved performance of the health system	Number of countries that develop policies and strategies based on evidence	2	6
		10.005. EM02	Improved leadership in health system research and capacity for health system research strengthened in countries	Number of countries that document the knowledge process (all levels: sub-national, national, regional, global)	2	6
				Number of countries that identify health system governance needs in terms of information and evidence	2	6
		10.005. EM03	Transparent, evidence-informed conduct of health research ensured	Number of countries that conduct evidence-informed health research	2	10
				Number of countries with standard mechanisms for evidence-informed health research	2	6
10.006	National health research for development of health systems strengthened in the context of regional and international research and	10.006. EM01	Health system research institutionalized	Number of countries in which health systems research has been recognized and institutionalized	2	6

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
	engagement of civil society.	10.006. EM02	Capacity for health systems research strengthened in countries	Number of health system research projects (US\$ 25 000 or above) funded and implemented and number of health researchers trained	NA	20
		10.006. EM03	National priorities for health research set out and conduct of health research organized according to the national priorities	Number of countries that have set out national priorities for health research using standard methods for priority setting	3	10
		10.006. EM04	Strengthened national capacity for harnessing genomics and biotechnology in health research	Number of health genomics and biotechnology projects (US\$ 25 000 or above) funded and implemented and number of health researchers trained	2	6
		10.006. EM05	Strengthened national capacity in ethical review of research	Number of countries in which health research ethics committees exist	2	6
		10.006. EM06	Community participation and conduct of health research targeting community problems ensured	Proportion of health research conducted with community participation	NA	0.33
10.007	Knowledge management and eHealth policies and strategies developed and implemented in order to strengthen health systems.	10.007. EM01	Strengthened capacity and infrastructure to manage information and knowledge including the necessary policies, processes, technologies and institutions for publishing, managing, disseminating, sharing and utilization of knowledge in multilingual environment	Number of countries that have a national framework for knowledge management and e-health	NA	10
				Number of countries that have established national projects for health information management and dissemination	NA	22
				Number of countries that have established programmes for training of health information specialists	NA	6
				Number of countries that have initiated e-health projects and services	NA	15
				Number of countries that have established focal points and programmes for language services	0	10
				Number of countries participating in knowledge networks	8	22
10.008	Health workforce information and knowledge base strengthened, and country capacities for policy analysis, planning, implementation, information-sharing and research built up.	10.008. EM01	Strengthened country capacity to establish, develop and sustain systems for human resources evidence-based policy and planning with a focus on imbalanced coverage and categories like nursing and midwifery	Number of countries performing human resources for health analyses and research and/or using human resources evidence and guidelines	3	8
		10.008. EM02	Strengthened national and regional capacities in generating, developing, researching and maintaining quality data on human resources	Number of countries using information for policy	3	10
				Number of countries operating national human resources for health observatories	3	10
		10.008. EM03	Regional database and website on health professions education programmes including public health institutes developed and updated	Number of schools registered in database	505	605 including updates

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
		10.008. EM04	Networking of centres of educational, human resources development and nursing established to become increasingly capable of producing guidance and policies addressing the stages of workforce development – entry, working life and exit	Number of centres connected	20	30
10.009	Technical support provided to Member States, with a focus on those facing severe health-workforce difficulties in order to improve the production, distribution, skill mix and retention of the health workforce.	10.009. EM01	Countries assisted in their efforts to develop human resources systems of optimum utilization and development including career development, retention and regulatory systems pertaining to different health professions categories with a focus on nursing, midwifery and allied health	Proportion of countries with effective utilization and regulatory systems for all categories	10	14
		10.009. EM02	Effective policy and management of WHO fellowships programme developed to focus on priority areas	Number of fellows/time in priority areas	70%	90%
		10.009. EM03	Support and guidance to countries provided to plan for, implement and monitor national accreditation systems to assure quality programmes and graduates of all health professions education institutes	Number of countries in which health professions education schools are subjected to an accreditation system	6	16
		10.009. EM04	Support and guidance to countries provided to improve learning process in health professions education institutes towards more relevance, community oriented medical education and life-long learning and to establish an educational development centre in each institution	Number of health professions education schools adopting new programmes	40	70
				Number of health professions education schools with active educational development centre	80	105
		10.009. EM05	Countries supported to initiate programmes to prepare qualified and competent health leadership, management, training and teaching with a focus on public health for all health professionals	Established regional and national training programmes in national languages to prepare leaders, managers, teachers and trainers in different categories with a focus on nursing, midwifery, and allied health sciences in countries with shortage of human resources for health	Estimated 12 countries are in crisis shortage 5 countries have programmes in management	At least 8 countries to be ready to produce human resources for health At least 3 more countries have programmes in management
10.010	Evidence-based policy and technical support provided to Member States in order to improve health system financing in terms of the availability of funds, social and financial-risk protection, equity, access to services and efficiency of resource use.	10.010. EM01	Ethical, equitable, gender responsive, efficient, sustainable, and evidence based policy options developed and technical support provided to Member States to improve the performance of their financing systems	Number of countries that were provided with technical support to improve their health financing system in line with regional expected result	5	7
				Number of key information to guide policy formulation and implementation developed and disseminated.	3 policy brief produced and disseminated	5 policy briefs produced and disseminated
				Number of new networks and websites to support policies are established and supported	2 networks	3 networks

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
10.011	Norms, standards and measurement tools developed for tracking resources, estimating the economic consequences of illness, and the costs and effects of interventions, financial catastrophe, impoverishment, and social exclusion, and their use supported.	10.011. EM01	Norms, standards, and knowledge developed by WHO are regionalized and their development and use in Member States promoted and supported	Number of countries that develop and use the analytical tools in order to improve health financing policy	12	15
10.012	Steps taken to advocate additional funds for health where necessary; to build capacity in framing of health financing policy and interpretation and use of financial information; and to stimulate the generation and translation of knowledge to support policy.	10.012. EM01	Steps taken to advocate additional funding for health in under funded health systems and promote the efficient use of available resources	Number of selected countries with increased level of funding for under-funded health systems	4	5
				Number of countries with increased productivity and return on investment in the health system	0	2

Strategic Objective 11: Ensure improved access, quality and use of medical products and technologies

Regional issues and challenges

Medical products and technologies save lives, reduce suffering and improve health, but only if they are of good quality, safe, effective, available, affordable, acceptable and properly used by prescribers and patients. In many countries in the Region, not all these conditions are met. This is often due to a lack of awareness of the potential benefits in medical outcomes and economic savings; lack of political will and public investment; commercial and political pressures; and fragmented financing and supply strategies.

The Eastern Mediterranean Region is very diverse with regard to health situations and systems. Like elsewhere in the world, primary health care, the health-related Millennium Development Goals and new global funding mechanisms fully depend on reliable and sustainable supply of medicines, vaccines and health technologies of assured quality. Within Member States, about half of overall health expenditure is on medical products. Despite this large share of health expenditure, thousands die unnecessarily in the Region every day due to lack of access to basic essential medicines and technologies. Countries in the Region vary greatly in their ability to serve their populations with essential medical products and technologies as a part of the basic health services. The regulatory institutional development also varies from country to country. Poor countries have weakest coverage and regulation. The people most in need in these countries are the people most underserved. High out-of-pocket expenditures, burgeoning prices and lack of confidence in the quality of medicines is commonplace. Weak implementation of relevant policies and laws are also a big challenge. As a result of this situation, there are huge health and economic losses. International and bilateral trade agreements price future essential medicines out of reach for most people who need them, and globalization allows for an unprecedented growth in counterfeit medical products which is an issue in at least a few countries in the Region. Safety monitoring of new medicines for HIV/AIDS, tuberculosis and malaria is missing in exactly those areas where these medicines will be used most.

A balance needs to be struck between short-term gain through special vertical systems, and long-term development of comprehensive national policies and supply systems for medical products and technologies. Without high-level political support and additional investment, both in WHO and in national health budgets, the large potential of essential medical products and technologies will continue to remain untapped, leading to unnecessary disease, disability, death and economic waste. There is great potential for quality improvements and economic savings; for example, rational use programmes can yield a three-fold economic return.

Strategic approaches

Expanding access to essential medicines, vaccines and technologies of assured quality, and improving their use by health workers and consumers can best be achieved through the establishment and implementation of comprehensive national medicine policies, ongoing investment in and development of national regulatory institutions and setting rational priorities in selection and use of medicines and technologies. In supporting the efforts of Member States, while combining WHO's recognized technical leadership role and unique global normative functions with advocacy, policy guidance and targeted country support, WHO will apply evidence-based international norms and standards and assist Member States in establishing and implementing programmes to promote good supply management (including emergency supply management) and rational use of products and technologies as essential components of its approach. Special focus will be placed on establishing reliable procurement, combating counterfeit and substandard products, promoting cost-effective clinical interventions, ensuring long-term adherence to treatment, and containing antimicrobial resistance.

Technical support at country level will be undertaken in close collaboration with health authorities, professional networks, consumer organizations and other stakeholders. Similarly, a close and coordinated approach will be employed towards disease-oriented vertical programmes. Countries having industrial potential will be supported to produce, use and export products of assured quality,

safety and efficacy through strengthening of national regulatory authorities and through assisting in prequalification of products. Countries will also be supported in establishing and strengthening systems for post-marketing surveillance, pharmaco-vigilance and prescription monitoring, and in communicating the outcomes to citizens and other stakeholders to promote patient safety. National efforts to train and retain professional workforce will be actively supported.

Regional assumptions and risks and option analysis

It is assumed that expanding access to essential products and technologies of assured quality, and improving their use by health workers and consumers, will remain a priority area for Member States and therefore for the Regional Office. It is also assumed that Member States will provide the required political will and financial resources to develop this crucial part of national health system and service.

Within national systems and within WHO there is a risk that medical product and technology-related work is split between different vertical programmes. Insufficient recognition by the new global funding programmes of the need for national capacity-building in quality assurance, procurement and supply management, rational use and pharmacovigilance might result in wasting a large proportion of the new supply funds. There is also a looming risk of further deterioration in chronic emergency situations in a few countries in the Region and emergence of new emergency situations in these and other countries.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
11.001	Formulation and monitoring of comprehensive national policies on access, quality and use of essential medical products and technologies advocated and supported	11.001. EM01	Development and monitoring of comprehensive national medicine policies based upon essential medicines concept, vaccine quality and regulation strategies and strategies for essential health technologies advocated and supported	Number of countries supported to develop and implement official national policies on access, quality and use of essential medical products and technologies	12	15
		11.001. EM02	Development and monitoring of national approach on traditional medicine and complementary and alternative medicine (TM/CAM) advocated and supported	Number of countries supported to develop a national approach / system on TM/CAM	5	10
		11.001. EM03	Development and monitoring of comprehensive national medical products and technologies supply systems advocated and supported	Number of countries supported to develop or strengthen comprehensive national procurement and supplies systems	10	15
		11.001. EM04	Development of national capacities for production of priority vaccines in pursuance of Regional vaccine self-sufficiency strategy	Number of WHO prequalified vaccines from the Region	0	1
11.002	International norms, standards and guidelines for the quality, safety, efficacy and cost effective use of medical products and technologies developed and their national and/or regional implementation advocated and supported	11.002. EM01	Establishment and strengthening of comprehensive national regulatory authorities advocated and supported	Number of countries with their national regulatory authority assessed and/or supported	11	15
				Number of countries fulfilling WHO requirements for effective regulation of vaccines	0	4
		11.002. EM02	Appropriate tools and technical support provided to all countries to improve immunization safety	Number of countries using exclusively auto-destruct syringes in their routine immunization programme	13	18
				Number of countries with established adverse events following immunization (AEFI) surveillance system	9	15

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
		11.002. EM03	Appropriate tools and technical support provided to all countries for rational selection and management of essential health technology.	Number of countries using WHO requirements for rational selection and management of essential health technology	0	2
		11.002. EM04	Appropriate tools and technical support provided to laboratory network to improve quality	Number of countries using the quality assurance system	0	2
		11.002. EM05	Technical support provided to upgrade and improve blood transfusion services	Number of countries with safe blood transfusion services	6	10
11.003	Evidence-based policy guidance on promoting scientifically sound and cost-effective use of medical products and technologies by health workers and consumers developed and supported within the Secretariat and regional and national programmes.	11.003. EM01	Development and monitoring of comprehensive national programmes on rational use of medical products and technologies advocated and supported	Number of national programmes/initiatives to promote sound and cost-effective use of medical products and technologies	5	10

Strategic Objective 12: Provide leadership, strengthen governance and foster partnership and collaboration with countries, the United Nations system, and other stakeholders in order to fulfil the mandate of WHO in advancing the global health agenda as set out in the 11th General Programme of Work

Regional issues and challenges

Issues and challenges with regard to the leadership and governance of the Organization in the Region concern the relationship between the Regional Committee and the Secretariat, through the Regional Director, which requires the Secretariat to meet the needs of the Regional Committee effectively and to ensure responsive and transparent implementation of its decisions. Within the Secretariat, more robust mechanisms are needed to ensure clear lines of authority, responsibility and accountability, especially in a context where resources and decisions on the use of these resources are increasingly being devolved to the level at which programmes are implemented.

At all levels, the Organization's capabilities must be strengthened to cope with the ever growing demand for information on health. The Organization must be equipped to communicate internally and externally in a timely and consistent way at regional and country levels, both proactively and in times of crisis, to articulate its leadership in health, to provide essential health information and to ensure visibility.

There is a need for strong political will, good governance and leadership at the country level. Indeed, the state has a key role in shaping, regulating and managing health systems and designating the respective health responsibilities of government, society and the individual. This means dealing not only with health sector issues but with broader ones, for example civil service reform and macroeconomic policy, which can have a major impact on the delivery of health services. The Secretariat, for its part, must do more to ensure it focuses its support around clearly articulated country strategies, that these are reflected and coherent with WHO's medium-term plans and programme budgets, and on matching the Organization's presence to the needs and level of development of the country concerned in order to provide optimal support.

At the regional level, mechanisms such as the Regional Committee could be further strengthened to allow stakeholders to tackle regional health issues in a transparent and effective way. WHO must ensure that national health policy-makers and advisers are fully involved in all international forums where issues affecting health status are being discussed. This is particularly important in a time of social and economic interdependence, where decisions on issues such as trade, conflict and human rights can have major consequences for health. The numerous players in public health, outside government and intergovernmental bodies, whether they be activists, academic or private sector lobbyists, need to have forums to contribute in a transparent way to global and national debates on health-related policies. They are also central to ensuring good governance and accountability.

The increasing number of sectors, actors and partners involved in health work has led to gaps in accountability and lack of synergy in the coordination of actions to improve health. The Region will make use of the global health partnerships which offer the potential to combine the different strengths of public and private organizations, along with civil society groups, in tackling health problems. As the number of partnerships grows, this may paradoxically give rise to duplication of effort between initiatives, high transaction costs to government and donors, unclear accountability, and lack of alignment with country priorities and systems. WHO will need to work to mitigate this.

The expectations for the United Nations system as a whole are increasing, as is the need to clarify the ways in which it adds value. This is a challenge for WHO as well as partner United Nations agencies. Of particular importance are relations at country level, where many changes are taking place as international agencies align their work with national health policies and programmes, and harmonize their efforts in order to reduce the overall management burden. Within this context, WHO needs to continue to play a proactive role with the United Nations system, as well as develop innovative mechanisms for managing or participating in global partnerships. The aim is to make the overall international health architecture more efficient and responsive to the needs of Member States.

Strategic approaches

Achieving the strategic objective will require Member States and the Secretariat to work closely together. More specifically, the strategic approaches are as follows.

- Leading, directing and coordinating, at the regional level, the work of WHO in relation to the global health agenda. This will include providing leadership and direction to the work of WHO; strengthening the governance mechanism of the Organization through stronger engagement of Member States and effective secretariat support; and effectively communicating the work and knowledge of WHO to Member States, other partners, stakeholders and the general public.
- Engaging with countries to advance the global health agenda, contribute to the national strategies and priorities, and bring country realities and perspectives into global policies and priorities. Actions will include orienting and coordinating WHO regional and country levels on the basis of an effective country presence that reflects national needs and priorities; promoting multisectoral approaches for advancing the global health agenda; developing institutional capacities at the national level for leadership and governance; building national capacity for health development planning; and facilitating technical cooperation among developing and developed countries.
- Promoting development of functional partnerships in the Region that ensure equitable health outcomes at all levels. Actions will include encouraging harmonized approaches to health development and health security with other international agencies at regional and country levels, including United Nations organizations; actively engaging in the United Nations reform dialogue; promoting the development of effective partnerships for health; reaching out to other stakeholders in health; and acting as a convener of relevant stakeholders on health issues of global and regional importance.

Regional assumptions and risks and option analysis

Good governance and strong leadership require continued commitment from all stakeholders. Good governance requires resolutions and decisions of the governing bodies to be upheld and respected by Member States and the Secretariat. It also requires a relationship bound by trust between Member States and the Secretariat, which today is strong and recognized. This must be maintained. Mechanisms to ensure greater accountability and adherence to decisions are being strengthened in the context of the results-based management framework covered in Strategic Objective 13.

It is also assumed that while changes in the external and internal environment are likely to occur over the six-year period of the Medium-term strategic plan, these would not fundamentally alter the role and functions of WHO. Should this happen, for example in the context of United Nations reforms, WHO must have the ability to respond and adapt itself accordingly. Adverse consequences of the United Nations reform process could be mitigated and opportunities increased if WHO takes initiatives and plays a proactive role in the reform process.

In an environment with an increasing number of sectors, actors and partners, WHO's role and comparative advantage needs to be well understood and indeed recognized. This is the case in most countries today, and across the majority of programmatic areas. It will be critical to maintain this advantage in order to successfully implement the ambitious strategic objectives set out in this plan, and hence contribute to achieving the health-related Millennium Development Goals

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
12.001	Effective leadership and direction of the Organization exercised through enhancement of governance, and the coherence, accountability and synergy of WHO's work.	12.001. EM01	Effective leadership and direction of the Organization in the region through the enhancement of governance, coherence, accountability and synergy of the work of WHO	Proportion of resolutions adopted by Regional Committee that focus on policy and can be implemented at regional and national levels	100%	100%
				Proportion of documents submitted to Regional Committee within constitutional deadlines, in the three official languages in the Region	100%	100%
12.002	Effective WHO country presence established to implement WHO country cooperation strategies that are aligned with Member States' health and development agendas, and harmonized with the United Nations country team and other development partners.	12.002. EM01	Effective WHO country presence to implement WHO Country Cooperation Strategies that are aligned with, Member States' national health and development agenda, and harmonized with the UN country team of other development partners	Number of countries actively using the CCS development process as a basis for planning WHO's country work and for harmonizing WHO's cooperation with the United Nations country team members and other development partners	22	22
				Proportion of countries where WHO country presence, including regional and global support, reflects the respective Country Cooperation Strategies	22	22
				Number of countries with updated Country Cooperation Strategy (second generation)	15	22
12.003	Global health and development mechanisms established to provide more sustained and predictable technical and financial resources for health on the basis of a common health agenda which responds to the health needs and priorities of Member States.	12.003. EM01	More sustained and predictable technical and financial resources for health based on a common health agenda which responds to the health needs and priorities of countries in the Region	Proportion of external aid flows to health supplied through flexible and long-term instrument	10%	20%
				Number of the countries implementing joint health priority programmes with other United Nations agencies and partners according to best practice principles for global health partnerships	10	14
				Number of countries where WHO is leading or actively engaged in the context of United Nations reforms	1	1
12.004	Essential multilingual health knowledge and advocacy material made accessible to Member States, health partners and other stakeholders through the effective exchange and sharing of knowledge.	12.004. EM01	Essential health knowledge and advocacy material made accessible to Member States, health partners and other stakeholders in the Region through the effective exchange and sharing of knowledge	Number of countries that have access to relevant health information and advocacy material for the effective delivery of health programmes as reflected in the Country Cooperation Strategies	18	22
				Average number of daily visits to EMRO internet website	700	2500

Strategic Objective 13: Develop and sustain WHO as a flexible, learning Organization, enabling it to carry out its mandate more efficiently and effectively

Regional issues and challenges

As highlighted in the 11th General Programme of Work, continuous change is today the norm. War, internal conflict and sanctions in many countries of the Region continue to impede the work of the Organization, which must continue to evolve in a flexible and responsive manner to respond successfully to evolving global and regional health, social and political challenges that affect the Region while preparing for a future that may be very different from today.

The global public health architecture, within which WHO plays a key role, is increasingly complex. New players and partnerships are continuously emerging. Moreover, harmonization efforts in the development community and broader reforms within the United Nations system also influence the way global and local actors operate. WHO must not only participate actively in these developments, it must also ensure that its ways of working reflect this changing environment proactively.

Investments in health have increased substantially over the past 10 years. This has led to an increasing demand from countries for technical support from WHO. It has also affected WHO's relations with major partners and contributors, who are expecting increasing transparency and accountability both in terms of measurable results and in the use of financial resources.

Advances in information technology, increasing dependence on global economic cycles, innovation in managerial techniques and an increasingly competitive job market impact the way WHO can and should be managed. Within this context, and despite progress in a number of areas, there remain a number of challenges for improving managerial and administrative support throughout the Organization.

WHO's results-based management framework has been strengthened through the critical work done on the 11th General Programme of Work and the development of a Medium-term strategic plan. More can be done, however, to ensure that the results-based management framework effectively builds on lessons learnt, better reflects country needs and encourages greater collaboration throughout the Organization.

Management of financial resources is a challenge in an environment where more than 70% of the Organization's resources are voluntary contributions. Regular monitoring and reporting of the resource situation across the Organization has improved. However, more engagement is required internally with all technical programmes and externally with partners to ensure better alignment of resources with the programme budget and to lower transaction costs. A substantial part of the budget is for response to emergencies in the Region and requires prompt implementation, increasingly in a United Nations Development Group framework agreement with other United Nations agencies.

Progress has been achieved in implementing far-reaching human resources reforms, including a far-reaching contractual reform, the adoption of a global competency model for all staff, the establishment of a staff development fund and the roll-out of a leadership programme for all senior managers. Building on this, further efforts are needed to develop better human resource planning in WHO as well as a culture that promotes learning and manages performance. More must be done to facilitate the rotation and mobility of staff within the Organization. In emergency situations, budgets are often short-term and do not provide sustainable funding, resulting in high staff turnover.

A system is being implemented that allows the Organization to leverage its knowledge base better and to access timely information to support management decision-making. It will be critical to ensure that the system is continuously aligned and responsive to the changing needs of the Organization. Efforts must be pursued to improve the quality of managerial and administrative service delivery throughout the Organization.

Alignment between responsibility and authority is a prerequisite for robust accountability. Critical thinking is required to ensure that decision-making and implementation take place at the right levels in the Organization to maximize efficiency and effectiveness and ensure robust accountability, in line

with the needs and demands of the Organization. Particular emphasis should be placed on strengthening the managerial capacity of WHO country offices.

Strategic approaches

This strategic objective will require a major change in the operation of WHO programmes and roles of the country offices and Regional Office. Although in recent years major shifts were made in the delegation of authority to the field offices by the Regional Director, the system still relied substantially on prior approvals and bureaucratic controls. Technical, administrative and financial controls will remain essential for proper accountability but will be exercised post facto, with greater responsibility transferred to field offices and technical staff.

A strengthened results-based management culture and systems will be the essential basis and enabling factor for the increased delegation to the field and technical units.

Information and communication technology infrastructure must be developed and maintained at the Regional Office and country offices to support secure, reliable and cost-effective data and voice communication and to run applications and systems necessary for evolving of the learning organization. Information and management systems, notably the Global Management System (GSM) will need to be maintained and strengthened and staff will need to continue to be trained and be capable to utilize the system.

The Regional Office will shift its support towards more guidance, advice upon request and training. Regional staff will increase their visits to field offices to review constraints faced, find local solutions consistent with general policies and provide training and advice as required.

Regional assumptions and risks and option analysis

A key assumption is that there is support both among Member States and within the Secretariat to continue and further accelerate the reforms that are being undertaken. Indeed, successfully improving managerial ways of working in a sustainable fashion requires strong leadership from senior management and strong commitment from all staff throughout the Organization to ensure that strategies and policies are effectively translated into day-to-day practices and behaviour. Reaching out and communicating internally and externally will be critical to ensure that this objective remains relevant to the changing needs of the Organization.

It is also assumed that while changes in the external and internal environment are likely to occur over the six-year period of the Medium-Term Strategic Plan, these will not fundamentally alter the role and functions of WHO. Nonetheless, should this alteration occur, managerial reforms that are part of this strategic objective will shape WHO into a more flexible Organization able to adapt itself accordingly.

The Region is characterized by low administrative costs. Pressures to contain administrative costs are likely to remain. The Secretariat will continue to minimize costs and ensure that all options are considered in this regard, including outsourcing or relocation opportunities. However, this must not be done to the detriment of maintaining institutional knowledge, quality, appropriate controls and accountability. It must also be recognized that this objective is inherently linked to the work of the rest of the Organization; increasing workloads in other strategic objectives will require increased resources to support that work, even if the relationship is not a linear one due to economies of scale.

Lack of security, as well as sanctions against Member States, hamper the work of WHO and may impede achievement of the strategic objective.

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
13.001	Work of the Organization guided by strategic and operational plans that build on lessons learnt, reflect country needs, are elaborated across the Organization, and are used to monitor performance and evaluate results.	13.001. EM01	The work of the Organization is guided by strategic and operational plans that build on lessons learnt; reflect country needs; are developed jointly across the Organization; and are effectively used to monitor performance and evaluate results	Plans are based on up-to-date CCS, and JPRM developed jointly and bringing together all resources to be allocated from all WHO offices	JPRMs do not include HQ contributions and only to a limited extent Regional Office funding	JPRMs are based on CCS and include all planned resources (accessed contributions and voluntary contributions)
				Monitoring and evaluation integrated for any given country or regional expected result	Monitoring and evaluation are not integrated for any given country or regional expected result	Monitoring and evaluation are integrated for any given country or regional expected result
				Proportion of SO reports for mid term review and Programme Budget Assessment that have been peer reviewed and submitted on time	90%	100%
13.002	Sound financial practices and efficient management of financial resources achieved through continuous monitoring and mobilization of resources to ensure the alignment of resources with the programme budgets.	13.002. EM01	Sound financial practices and efficient management of financial resources through continuous monitoring and mobilization of resources to ensure the alignment of resources with the programme budgets (as may be revised by the Director-General and Regional Director within their delegated authority)	International Public Sector Accounting Standards implemented	100%	100%
				Proportion of managers trained and certified on WHO accountability	0%	100%
13.003	Human resource policies and practices in place to attract and retain top talent, promote learning and professional development, manage performance, and foster ethical behaviour.	13.003. EM01	Human resource policies and practices in place to attract and retain top talent, promote learning and professional development, manage performance and foster ethical behaviour	Staff trained in human resources planning to effectively determine workforce requirements, using competency model	Limited training provided	All line managers (Directors, WRs, Coordinators and head of administrative units) trained
				Proportion of offices having completed their human resources plan	100%	100% maintained
				Staff are trained on human resources policies and administration with special emphasis on competency based recruitment and on contractual agreements	Training not yet provided to target population	All managers are trained in human resources based on competency for planning resources and recruitment process
				Proportion of e-PMDS users in compliance with PMDS cycle whose staff development objectives have been met	80% GSM	100% GSM
13.004	Management strategies, policies and practices in place for information systems, that ensure reliable, secure and cost-effective solutions while meeting the	13.004. EM01	Information systems management strategies, policies and practices that ensure reliable,	Reliable, secure and operational 24x7 networks infrastructure	Partially secured and integrated networks at the Regional	Integrated and fully secured networks

Organization-wide expected result		Regional expected result		Indicator	Baseline	Target
	changing needs of the Organization.		secure and cost-effective solutions while meeting the changing needs of the Organization		Office and country offices	
				Integrated information systems to support administrative and technical programmes	Data warehouse and integrated systems in preparation for the GSM	GSM operational in harmony with remaining regional systems
				Trained staff on systems and productivity tools in support of learning organization	Systems are under-utilized due to lack of training; Systems do not support decision-making and planning	Training of 70% of staff on applications and systems
13.005	Managerial and administrative support services necessary for the efficient functioning of the Organization provided in accordance with service-level agreements that emphasize quality and responsiveness.	13.005. EM01	Infrastructure support services operated in a resource-effective and efficient manner	Proportion of services delivered in accordance with Service Level Agreements	75%	100%
		13.005. EM02	Logistic support functions operated in a resource effective and efficient manner	Proportion of services delivered in accordance with Service Level Agreements	75%	100%
		13.005. EM03	Health supplies of the highest quality at the best practice procured for Member States and technical programmes	Proportion of services delivered in accordance with Service Level Agreements	75%	100%
				Proportion of procedures delivered according to criteria in emergency standard operating procedures	75%	100%
13.006	Physical working environment conducive to the well-being and safety of staff in all locations.	13.006. EM01	Security and safety of grounds and premises improved	Proportion of offices MOSS compliant	80%	100%
		13.006. EM02	Real estate facilities improved	Proportion of components of the Capital Master Plan implemented in a timely way, within the approved budget	0	50%
				Proportion of offices that have implemented policies and plans to improve staff health and safety in the workplace	50%	100%